

BECHIR BENSAID

TRANSFORMING AMERICAN
HEALTHCARE

A Six-Pillar Framework for System Transformation

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TRANSFORMING AMERICAN HEALTHCARE

A Six-Pillar Framework for System Transformation

Health Transformation Review

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PREFACE

Why This Book Exists

American healthcare is the most expensive, most administratively complex, and most inequitably distributed healthcare system in the developed world. This is not a new observation. It has been documented, analyzed, debated, and lamented for decades. What is new — or more precisely, what is newly urgent — is that the system is now visibly breaking under pressures that

cannot be managed through incremental adjustment. The financial logic of fee-for-service payment, the demographic reality of an aging population, the exhaustion of a clinical workforce buried in administrative burden, and the structural inadequacy of a hospital network designed for a different century are converging into a crisis that requires systematic response, not marginal reform.

This book is a response to that crisis. It is designed for the healthcare leaders, policymakers, analysts, and students who must understand not just what is wrong with American healthcare but what a transformed system would look like, how to get there, and what the organizations executing that transformation need to know to succeed. It is simultaneously an academic text — grounded in evidence, rigorous in its analytical framework, honest about what is uncertain — and a practitioner’s guide — organized around decisions, investments, contracts, and programs that real organizations must make in real time.

The book is organized around the Health Transformation Review six-pillar framework: Policy, Economics, Technology, Clinical, Equity, and Operations. This framework emerged from the conviction that healthcare transformation fails when it addresses any single pillar in isolation. Payment reform without clinical redesign produces financial savings that evaporate when care models do not change. Clinical quality improvement without payment reform produces better care that the financial system immediately discourages. Technology investment without operational readiness produces platforms that no one uses. Equity goals without structural SDOH investment produce aspirations without outcomes. The six pillars must move together — which means the people responsible for each pillar must understand all the others.

Vermont runs through this book as its primary teaching case. This requires explanation, because Vermont is a small state — 647,000 people, 14 hospitals, roughly the population of a mid-sized American city — and its direct policy relevance might seem limited. Vermont is in this book not because Vermont is large or economically powerful, but because Vermont is early. Early in its demographic aging. Early in its hospital financial fragility.

Early in its recognition that voluntary payment reform cannot produce the structural change a system in crisis requires. And early in its willingness to legislate the structural responses — Acts 167 and 68, the AHEAD Model participation, the Rural Health Transformation Program — that other states will reach for when their own systems arrive at Vermont's current condition.

Vermont is also unusually transparent. The combination of a small state's intimacy, a strong public-sector analytical tradition, and an active regulatory body in the Green Mountain Care Board has produced a public record of healthcare system analysis — the Oliver Wyman Act 167 report, the GMCB's monthly hospital data, the AHS transformation reports, the AHEAD State Agreement — that gives this book a documentary foundation most states cannot match. Vermont's healthcare transformation is happening in public, with detailed data, and the lessons are directly transferable.

A word about what this book is not. It is not a policy brief for or against any particular political agenda. Healthcare transformation is contested terrain, and this book presents the evidence and the analytical frameworks that inform decisions without pretending those decisions are purely technical. The five goals of Vermont's Act 167 — reduce inefficiencies, lower costs, improve health outcomes, reduce health inequities, and increase access to essential services — are goals that virtually every person across the political spectrum would endorse. The means for achieving them are legitimately contested, and this book engages that contestation honestly.

It is also not a prediction. The transformations described in this book are underway but unfinished. The outcome of Vermont's experiment — whether the hospital system stabilizes or contracts, whether global budgets reduce costs or constrain access, whether the Agency of Human Services can execute the restructuring its mandate requires — is genuinely uncertain. This book provides the analytical framework for understanding what is happening and why, and for contributing to better decisions as the story unfolds. The decisions, and their consequences, belong to the people who make them.

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INTRODUCTION

The Transformation Imperative

A System at the Breaking Point

On September 18, 2024, the Green Mountain Care Board of Vermont convened a public meeting at which Oliver Wyman Healthcare and Life Sciences Group presented the findings of a year-long, \$1 million statewide hospital systems analysis commissioned under Act 167 of 2022. The room was full. The findings were not surprising to anyone who had been paying attention — but they were stated with a precision and a comprehensiveness that made them impossible to dismiss.

Nine of Vermont's fourteen hospitals were reporting operating losses in 2023, with the worst reaching negative 8.9%. If current trends continued, thirteen of the fourteen would be reporting losses by 2028. The cumulative five-year deficit facing the system ranged from \$700 million against break-even under conservative assumptions to \$2.4 billion against break-even under realistic ones. Average premiums for silver exchange plans had increased 108% in six years, while median household income had grown 22%. Out-of-pocket maximums had more than doubled in five years. The

working-age population was declining; the over-65 population was growing toward 30% of the state's total by 2040.

The Oliver Wyman consultants offered a three-imperative summary of what Vermont needed to do: build housing and fix transportation; pay hospitals with reference-based pricing and move to global budgets; and move all care possible out of hospitals. These were not incremental recommendations. They were structural prescriptions for a system that, absent structural change, was heading toward collapse.

Vermont is not an outlier. It is a preview.

The structural forces producing Vermont's crisis — demographic aging, hospital financial fragility, the unsustainability of commercial cross-subsidy, the failure of voluntary payment reform, the exhaustion of a workforce buried in administrative complexity — are present to varying degrees in every American state. Vermont faces them earlier and more acutely because of its demographics and its economic structure. But the states of the Northeast, the rural Midwest, and eventually the national system will arrive at Vermont's current condition on a lag of ten to twenty years. What Vermont is forced to solve now, the rest of the country will be forced to solve later.

This book is written in that context. It is not a retrospective analysis of a completed transformation. It is a real-time account of a transformation in progress, with all the uncertainty, difficulty, and instructiveness that implies.

What Transformation Actually Means

The word 'transformation' is used so frequently in healthcare that it has nearly lost its meaning. Press releases describe routine process improvements as transformational. Strategic plans invoke transformation as aspiration rather than commitment. This book uses the word in its precise sense: transformation is structural change that alters the fundamental incentives, relationships, and capabilities of a system, producing different

behaviors and different outcomes rather than incremental improvements within the existing structure.

By this definition, most of what has been called healthcare transformation over the past fifteen years has not been transformational. Voluntary accountable care organization programs that high-cost providers can decline to join are not transformation. Shared savings models that reward hospitals for reductions in utilization they would have achieved anyway are not transformation. Electronic health record adoption that digitizes paper processes without changing care delivery is not transformation. Quality measurement initiatives that produce reports without affecting payment are not transformation.

Transformation, in the sense this book uses the term, looks like what Vermont is attempting: mandatory reference-based pricing that eliminates the ability of dominant hospitals to charge commercial payers three to four times Medicare rates; statutory global hospital budgets that change every hospital's financial incentive from maximizing volume to managing population health; a Statewide Health Care Delivery Strategic Plan that commits Vermont's Agency of Human Services to specific, measurable outcomes by specific dates; and capital investment through the Rural Health Transformation Program that builds the community infrastructure that the payment reform requires to produce population health improvement rather than just cost reduction.

Whether this constitutes successful transformation will be determined by what happens over the next five to ten years. What is already clear is that Vermont has moved further toward genuine structural change than any comparably sized system in recent American health policy history.

The Six-Pillar Framework: Why All Six Must Move Together

The central analytical contribution of this book is the six-pillar framework for healthcare transformation: Policy, Economics, Technology, Clinical, Equity, and Operations. The framework is not a checklist of things that

should be done. It is an integrated analytical architecture for understanding why transformation attempts fail when they address any pillar in isolation.

The most common failure mode in healthcare transformation is single-pillar thinking. A payer that focuses exclusively on the Economics pillar — redesigning payment models — while leaving the Technology, Clinical, and Operations pillars unchanged produces payment reform that providers cannot operationalize. Hospitals without population health analytics infrastructure cannot manage global budgets. Clinicians without team-based care models cannot deliver the primary care access that makes reduced hospitalization possible. Administrators without administrative simplification cannot absorb the management complexity of value-based contracts on top of fee-for-service billing requirements.

Conversely, a technology vendor that focuses exclusively on the Technology pillar — building sophisticated health information exchange and population health platforms — while leaving the Economics and Operations pillars unchanged builds tools that no one has the financial incentive or organizational capacity to use. Vermont's Oliver Wyman analysis found this pattern explicitly: VITL (Vermont's health information exchange) exists but is not widely used, OneCare (Vermont's ACO) exists but participation is voluntary and incomplete, and the 'all-payer' system does not actually include all payers. The infrastructure exists; the incentives and operations to use it effectively do not.

The six pillars interact through a logic that this book develops in detail in Chapter 1. The short version is this: payment reform (Economics pillar) changes what behaviors the financial system rewards; clinical redesign (Clinical pillar) changes how care is actually delivered; technology investment (Technology pillar) provides the data infrastructure that makes new care models manageable at scale; policy and regulatory change (Policy pillar) creates the mandatory architecture that ensures voluntary actors cannot opt out of structural reform; equity investment (Equity pillar) ensures that transformation benefits reach the populations that need them most and does not concentrate gains among those already well-served; and operational execution (Operations pillar) converts strategy into

implemented programs, trained staff, and measured outcomes.

Remove any one pillar and the transformation fails. Vermont's reform agenda is instructive precisely because Acts 167 and 68, the AHEAD Model, and the Rural Health Transformation Program together address all six pillars simultaneously — imperfectly and incompletely, as any real-world policy effort does, but with a comprehensiveness that distinguishes Vermont's approach from single-pillar interventions.

How to Use This Book

This book is organized in three parts. Part I (Chapters 1-3) establishes the foundational framework: the six-pillar architecture, the Vermont system portrait, and the analytical tools for reading transformation efforts in any context. Part II (Chapters 4-15) develops each pillar in depth, with a dedicated chapter for each, integrating academic framework, evidence base, Vermont case study, and practical application for healthcare leaders and policymakers. Part III (Chapters 16-17) addresses advisory practice and the future — the institutional infrastructure for translating analysis into action, and the forces shaping American healthcare through 2035.

Throughout Part II, Vermont functions as a continuous teaching case rather than an isolated example. Each chapter begins with the national analytical framework for its pillar, develops the evidence base from national and international research, and then grounds the framework in Vermont's specific implementation — using Vermont's acts, Oliver Wyman's findings, GMCB's data, and AHS's transformation reports as the primary case material. This structure allows a reader who is not specifically interested in Vermont to use the national framework; a reader focused on Vermont to use the Vermont sections as a field guide; and a reader designing transformation efforts elsewhere to use Vermont as a template and a cautionary inventory of what is hard.

The book concludes with a chapter specifically addressed to the Vermont Agency of Human Services and the Health Care Delivery Advisory Commit-

tee: a six-pillar roadmap for the Statewide Health Care Delivery Strategic Plan that AHS must deliver to the Vermont legislature by December 2028. This chapter is simultaneously the most specific and the most generalizable section of the book — because the question it addresses (how does a state agency restructure itself to serve as a population health system operator?) is the question every state will face as their healthcare systems reach the condition Vermont is managing today.

Chapter 1: The Six-Pillar Framework

Why Transformation Fails When Any Pillar Is Neglected, and How All Six Must Move Together

Chapter 1 establishes the six-pillar framework that organizes this entire book. It answers three questions: what each pillar is, how the pillars interact with each other, and why transformation fails when any pillar is neglected. It also introduces the Vermont thread that runs through every subsequent chapter — establishing Vermont’s system in broad strokes before Chapters 2 and 3 develop the detail.

The Problem with Single-Pillar Thinking

In 2010, the Affordable Care Act created the Center for Medicare and Medicaid Innovation with a mandate to test new payment and delivery models and scale what worked. Over the following fifteen years, CMMI launched more than fifty models, investing billions of dollars and the attention of thousands of healthcare organizations. The results were modest. A 2023 evaluation found that the vast majority of CMMI models had not achieved statistically significant reductions in spending or improvements in quality. Most were discontinued. A handful showed promise and were

expanded.

The pattern of failure across CMMI models is instructive. Models that addressed only the Economics pillar — changing what Medicare paid without addressing how care was delivered or whether providers had the infrastructure to respond — generated financial pressure without behavioral change. Models that addressed only the Clinical pillar — redesigning care coordination or care management programs — produced clinical improvements that the payment system then penalized by reducing volume-based revenue. Models that required Technology pillar investment — electronic health records, population health platforms, data exchange — found that providers lacked the operational capacity to use the tools effectively.

The lesson is not that payment reform, clinical redesign, or technology investment is wrong. It is that each intervention, pursued in isolation, runs into the constraints created by the pillars it ignores. This is the core analytical argument of the six-pillar framework: healthcare transformation is a system problem, and system problems require system solutions.

Every major healthcare transformation framework in the past thirty years has identified the same list of necessary conditions. Pay differently. Redesign care delivery. Build data infrastructure. Address social determinants. Ensure equity. Execute operationally. The list is right. The reasoning behind it is right. And yet the transformation that the list is supposed to produce has not arrived — not in most states, not at national scale, not with the speed and completeness the evidence has long indicated is possible.

The problem is not the list. The problem is treating it as a list.

A checklist is a set of independent items. You can check them in any order. Checking one does not affect the others. Failing to check one does not prevent you from checking the rest. A checklist of healthcare

transformation requirements implies that you can make progress on any one of these independently, and that partial completion is partial progress. This is wrong. The six pillars of the HTR framework are not independent items on a checklist. They are a system of interdependent components in which each pillar's effectiveness depends on the others, in which the failure of any single pillar cascades through the system, and in which the order and timing of interventions matters as much as the content of those interventions. Treating the six pillars as a checklist is precisely how decades of technically correct healthcare reform recommendations have produced inadequate results.

“A policy intervention that passes question one but fails questions three, four, five, or six is not a solution — it is a well-intentioned mistake waiting to be made.”

— **HTR Mission Statement**

The Six Pillars Defined

The six pillars are not equal in their causal priority — payment reform (Economics) and enabling legislation (Policy) are preconditions that the other pillars depend on — but they are equal in their necessity. A transformed healthcare system requires all six to be functional.

The table below characterizes each pillar's diagnostic function and structural role in the system — not as a repetition of the definitions above, but as a specification of what each pillar produces when functioning and what question it answers in the transformation logic.

Figure 1.1 — The six pillars: diagnostic questions and structural roles in the HTR

framework. Source: HTR framework documentation; Acts 167 and 68 of 2025.

The Architecture of Interdependency: The Fifteen Dependency Relationships

The six pillars do not operate independently. They interact through a set of dependencies and feedback loops that determine whether transformation produces the outcomes it promises. Understanding these interactions is the core analytical skill this book develops.

The foundational dependency is between Policy and Economics. Payment reform (Economics) requires policy authority — either statutory mandate or regulatory power — to be effective at scale. Vermont learned this lesson through a decade of voluntary payment reform efforts: the Vermont All-Payer ACO Model, OneCare’s voluntary hospital participation, VITL’s voluntary HIE connectivity. Each produced partial results because voluntary participation allowed the highest-cost actors to opt out. Act 68’s mandatory RBP and global budget requirements are a Policy pillar response to the failure of Economics pillar interventions without Policy pillar backing.

The second critical dependency is between Economics and Clinical. Payment reform changes the financial incentive structure; clinical redesign changes the care delivery model to respond to the new incentives. A hospital operating under a global budget has a financial incentive to prevent hospitalizations — but it can only act on that incentive if it has the primary care network, care coordination infrastructure, and behavioral health integration to actually keep patients out of the hospital. Without the Clinical pillar investment, the Economics pillar incentive produces financial pressure without clinical improvement.

The third dependency is between Technology and all other pillars. You

cannot attribute patients to hospitals for global budget purposes without accurate patient matching across providers. You cannot identify care gaps for quality improvement without population-level claims and clinical data. You cannot model the financial impact of service line closures without analytics infrastructure. You cannot monitor equity metrics without data stratified by race, income, and geography. Technology is the information substrate on which every other pillar's management functions depend.

The Equity pillar has a different kind of dependency: it is simultaneously a constraint on every other pillar and a beneficiary of all of them. Every payment reform intervention, clinical redesign, and technology investment must be evaluated for its equity implications — does it reduce disparities or exacerbate them? Does it reach the populations that need it most or concentrate benefits among those already well-served? Vermont's Act 168 goals explicitly include reducing health inequities alongside reducing costs and improving outcomes, recognizing that transformation that achieves the first two without the third is incomplete.

The Operations pillar is the translation layer — the organizational capacity that converts policy intent, payment incentives, technology tools, clinical models, and equity goals into implemented programs with trained staff and measured outcomes. A transformation strategy that is analytically sound, financially compelling, technically feasible, and clinically validated will still fail if the organization responsible for executing it lacks the project management infrastructure, the change management capability, and the sustained leadership attention that execution requires.

These five relationships are the framework's structural core. What follows develops the complete architecture: all fifteen directed dependency relationships between and among the six pillars, the specific mechanisms by which each dependency operates, the failure modes that emerge when dependencies are ignored, and the Vermont evidence that confirms the theoretical structure with empirical specificity. The HTR framework map

— available as an interactive visualization on the HTR platform — renders these fifteen relationships as a navigable diagram.

Policy Dependencies: Three Relationships That Establish the Mandatory Architecture

Policy is the pillar from which mandatory authority flows. Its three outbound dependencies — to Economics, Operations, and Equity — are among the most critical in the framework, because they are the relationships that convert every other pillar from optional to required.

Policy → Economics: Mandatory authority enables structural payment reform

Dependency type: ENABLES (critical path)

The most important dependency in the entire framework. Payment reform that is voluntary produces voluntary results. Vermont spent a decade under OneCare Vermont — a voluntary ACO model — and produced modest results among participating hospitals while non-participating hospitals continued operating under fee-for-service without consequence. Act 68 of 2025 changed the architecture: reference-based pricing is mandatory for all Vermont hospitals beginning FY2027, and global budgets are mandatory beginning FY2028. This is not an incremental improvement on the voluntary model. It is a structural change that eliminates the opt-out that made the voluntary model ineffective.

Vermont evidence: Oliver Wyman's Act 167 diagnosis explicitly found that voluntary reform was insufficient given the financial trajectory. The \$700 million to \$2.4 billion five-year deficit projection assumes the continuation of the commercial pricing structure that mandatory RBP is designed to dismantle. Without the Policy pillar's mandatory authority, the Economics pillar remains aspirational.

Policy → Operations: Statutory deadlines force execution capacity

Dependency type: REQUIRES (critical path)

One of the most underappreciated dependencies in healthcare transformation. Organizations develop the capacity to execute complex initiatives when they have non-negotiable deadlines, not when they have good intentions. Act 68's December 2028 Statewide Strategic Plan deadline, the FY2027 RBP mandate, and the FY2028 global budget mandate are not just policy milestones — they are organizational forcing functions that require AHS to build the project management infrastructure, analytics capability, and staffing depth that the transformation requires.

Without the Policy pillar imposing external accountability, the Operations pillar faces the chronic problem of healthcare administration: transformation planning that is perpetually in progress but never complete, capacity building that is always on the agenda but never funded, and organizational development that yields to operational demands every time there is a conflict. Vermont's statutory deadlines are the mechanism that prevents this. They are deliberately irreversible.

Policy → Equity: Statutory mandates embed equity accountability

Dependency type: ENABLES

Acts 167 and 68 both contain explicit equity requirements — they are not aspirational statements about desired outcomes but operational mandates: equity metrics must be tracked, disparities must be reported, and the Statewide Strategic Plan must address geographic and demographic equity gaps. This is the Policy pillar enabling the Equity pillar's accountability function. Without statutory embedding, equity monitoring is a discretionary activity that competes with operational priorities. With it, equity measurement is a compliance requirement.

Vermont evidence: the Health Care Delivery Advisory Committee established under Act 68 is required to consult with the Health Equity Advisory Commission and to incorporate equity considerations into the Statewide Strategic Plan. This institutional design is the Policy pillar

building the Equity pillar's accountability structure into the statutory framework.

Economics Dependencies: Three Relationships That Transmit Incentive Change

Economics is the pillar through which policy mandates are converted into behavioral change at the organizational and clinical level. Payment models do not change behavior directly — they change the financial logic that shapes decision-making. The Economics pillar's three outbound dependencies — to Clinical, Technology, and Equity — describe the mechanisms through which this conversion happens.

Economics → Clinical: Payment incentives drive care delivery re-design

Dependency type: DRIVES (critical path)

The most important economics dependency. Under fee-for-service, preventing a hospitalization costs the hospital revenue. A primary care practice that successfully manages a diabetic patient's condition and prevents a \$15,000 hospitalization has generated no billable encounter for the prevented admission. The clinical logic and the financial logic point in opposite directions. Under global budgets, the calculus reverses: every avoidable hospitalization is a cost against a fixed revenue envelope, and every prevented admission is a margin improvement. The payment model makes population health management financially rational for the first time.

Vermont evidence: Vermont's Blueprint for Health demonstrates this dependency in reverse. The Blueprint's 5.8:1 return on investment — \$5.8 million in reduced medical expenditure per \$1 million invested in primary care — is achievable because the Blueprint's all-payer PMPM payments to PCMHs create a financial structure that rewards the clinical investment. Remove the Economics pillar and the Blueprint's clinical model produces

its results without capturing its financial returns. Sustain the Economics pillar through AHEAD's global budgets and the Clinical pillar's population health logic becomes fully incentive-aligned.

Economics → Technology: VBC contracts require data infrastructure

Dependency type: REQUIRES

Value-based care contracts are agreements to share financial accountability for a population's total cost of care. They cannot be executed without the data infrastructure to measure that total cost. Attribution lists, TCOC benchmarks, shared savings calculations, risk adjustment factors, quality measure reporting — all of these require the all-payer claims data, the population health analytics platform, and the FHIR-based data exchange that the Technology pillar provides. An organization that signs an APM contract without adequate data infrastructure is making a financial commitment it cannot monitor, manage, or dispute.

Vermont evidence: Vermont's AHEAD global budgets for hospitals, beginning January 2027, require GMCB to set prospective annual budgets based on historical Medicare spending adjusted for population risk. This calculation requires accurate VHCURES data, validated risk adjustment methodology, and an analytics platform capable of producing hospital-specific TCOC projections. The AHS-GMCB analytics vendor procurement underway in 2025-2026 is the Technology pillar investment that makes the Economics pillar's AHEAD implementation analytically executable.

Economics → Equity: VBC design must include social risk adjustment

Dependency type: REQUIRES

Value-based care contracts that do not adjust for social risk systematically penalize providers serving high-SDOH populations. A hospital in Essex County, Vermont — serving a population with higher poverty rates, housing instability, food insecurity, and transportation barriers — faces higher care management costs and lower quality measure performance potential than a

hospital in Burlington serving a more economically stable population. If their global budgets are set at the same percentage of Medicare rates without social risk adjustment, Essex County's hospital is being held to a standard that does not reflect its population's actual cost burden.

Vermont evidence: Vermont's AHEAD global budget design must address this explicitly. The methodology for social risk adjustment in Vermont's hospital global budgets is an active design question as of early 2026. Getting it wrong — designing an Economics pillar instrument that ignores the Equity pillar's requirement — would produce a payment system that penalizes the hospitals serving the most vulnerable Vermonters.

Technology Dependencies: Three Relationships That Enable Analytical Management

Technology is the data substrate on which every other pillar's management functions depend. It has the broadest set of outbound dependencies in the framework — three enabling relationships to Economics, Equity, and Clinical that describe what becomes possible when data infrastructure is adequate, and what remains impossible when it is not.

Technology → Economics: Analytics makes VBC financial management possible

Dependency type: ENABLES

This dependency runs in both directions between Technology and Economics: the Economics pillar requires data infrastructure (documented above), and the Technology pillar enables the financial management capability that VBC contracts require. The distinction is between existence and effectiveness. An organization can enter an APM contract without adequate analytics — the contract still exists, the financial obligations are still real. But the organization cannot manage its financial performance under that contract, cannot identify where its costs are above benchmark, cannot target

care management investment toward the highest-return opportunities, and cannot dispute errors in the payer's shared savings calculations.

Vermont evidence: VHCURES population health analytics make the APM financial modeling described in Chapter 7 possible. The HTR APM Shared Savings Calculator, the Hospital Financial Stress Test Model, and the VBC Readiness Assessment — all tools in the HTR Research Lab — are built on VHCURES data logic. Vermont hospitals that invest in VHCURES analytics capability before AHEAD launches in January 2027 will be able to manage their global budgets proactively rather than reactively.

Technology → Equity: Disaggregated data reveals disparities hidden in averages

Dependency type: ENABLES (critical)

The Equity pillar's entire analytical function depends on the Technology pillar's ability to produce demographic stratification. You cannot measure a disparity you cannot see. Vermont's 91% primary care access rate — four points above the national benchmark — is a genuine achievement. It also hides an 11-point gap between white Vermont adults and BIPOC Vermont adults. That gap is invisible in aggregate data. It becomes visible only when VHCURES data is stratified by race/ethnicity, and that stratification is only possible when the data contains adequate demographic information.

Vermont evidence: Vermont's race/ethnicity data in VHCURES is incomplete — commercial claims frequently lack demographic fields. This is not a minor data quality issue; it is a Technology pillar gap that directly limits the Equity pillar's analytical capability. The Vermont Department of Health Health Equity Data Report (January 2025) documents this limitation explicitly. Closing the Technology gap is prerequisite to the Equity pillar's disparity measurement function.

Technology → Clinical: Data infrastructure enables population health management

Dependency type: ENABLES

The clinical programs described in Chapter 10 — Blueprint PCMH, CoCM, CCBHC, CHT deployment — can all be designed and implemented without data infrastructure. What they cannot do without data infrastructure is function as population health programs rather than individual encounter programs. The difference is analytically fundamental. Risk stratification identifies which patients are most likely to deteriorate; without it, care management deploys reactively, after hospitalization rather than before. Care gap identification triggers outreach for overdue preventive services; without it, gaps close only when patients happen to schedule appointments. SDOH screening tools flag patients for CHT navigation; without the data platform to connect the flag to the navigation workflow, the flag is generated and ignored.

Vermont evidence: Blueprint's clinical registry is the Technology pillar product that enables Blueprint's Clinical pillar outcomes. The 5.8:1 ROI documented for Blueprint is achievable precisely because Blueprint combines the Clinical pillar's care team model with the Technology pillar's population health analytics. The AI scribe grants in the RHT Program are the Technology pillar's contribution to the Clinical pillar's workforce capacity problem.

Clinical Dependencies: Two Relationships That Connect Care Models to System Requirements

Clinical is the pillar that converts payment incentives into actual health outcomes. Its two outbound dependencies — to Operations and Equity — describe the operational requirements of clinical programs and their equity implications.

Clinical → Operations: Care models require execution infrastructure

Dependency type: REQUIRES

The most consistently underestimated dependency in healthcare transformation. A care model is a diagram until it has workforce, credentialing, administrative infrastructure, and operational management. The Collaborative Care Model requires a behavioral health care manager employed by a primary care practice. The CCBHC model requires a certified entity with the organizational capacity to serve all patients regardless of insurance status. Blueprint PCMHs require care coordinators, health information exchange connectivity, and NCQA recognition processes. None of these are available without Operations pillar investment.

Vermont evidence: Oliver Wyman's Act 167 analysis found that Vermont's administrative cost per adjusted discharge is 91% above the national benchmark. This is not merely an inefficiency problem — it is a signal that Vermont's Operations pillar infrastructure is absorbing cost without producing the execution capability that clinical transformation requires. The RHRC engagement with all 14 Vermont hospitals is specifically designed to build the Operations pillar capacity that Clinical pillar programs need.

Clinical → Equity: Care delivery expansion is the primary access gap mechanism

Dependency type: ENABLES

Geographic and demographic disparities in healthcare access are primarily clinical access problems — not enough primary care providers, not enough behavioral health capacity, not enough community health workers in underserved communities. The Equity pillar's access gap goals cannot be achieved without the Clinical pillar's care model expansion. Blueprint's PCMH expansion into Northeast Kingdom communities, the CCBHC network's geographic reach, and the CHT deployment in high-SDOH communities are Clinical pillar investments that directly serve Equity pillar objectives.

Vermont evidence: Essex County's 8% uninsurance rate and limited primary care access are equity problems with a clinical solution — ex-

panding Blueprint-participating practices in underserved HSAs, deploying telehealth through the RHT Program to reduce geographic access barriers, and certifying additional CCBHCs in Northeast Kingdom communities. Remove the Clinical pillar investment and the Equity pillar's access gap remains unmeasured but unaddressed.

Operations Dependencies: Two Feedback Relationships That Close the Loop

Operations is the pillar that executes all other pillars' intentions. Its two outbound dependencies run back to Policy and Technology — feedback relationships that make the system self-correcting rather than purely top-down.

Operations → Policy: Implementation data feeds back into policy design

Dependency type: ENABLES

This is the feedback loop that allows the policy architecture to be self-correcting. AHS's monthly transformation reports to the Vermont Legislature — required under Act 68 — document whether transformation is proceeding on the statutory timeline, where gaps have emerged, and what adjustments are required. The GMCB's RBP methodology for FY2027 is being refined based on hospital financial data from the operations layer. The December 2028 Statewide Strategic Plan will be drafted based on what the Operations pillar has learned from implementing the preceding three years of transformation. This is not passive reporting — it is active policy learning.

Vermont evidence: AHS's November 2025 transformation report changed the policy understanding of Vermont's administrative cost problem by documenting the UVMHC \$3,826 per adjusted discharge figure against the \$1,427 benchmark. That Operations pillar data point is now embedded

in the policy case for mandatory transformation. Without the operations feedback loop, policy would operate on assumptions rather than evidence.

Operations → Technology: Workforce runs the data infrastructure

Dependency type: REQUIRES

The Technology pillar's infrastructure — VHCURES analytics, the CIN data platform, FHIR compliance, AI governance programs — does not operate itself. It requires IT staff to maintain systems, data analysts to generate the reports that clinical and financial management teams use, privacy officers to manage HIPAA compliance, and cybersecurity staff to protect the infrastructure from the ransomware threats that have disabled rural health systems nationally. The Operations pillar's workforce is the human substrate on which the Technology pillar's data infrastructure runs.

Vermont evidence: Vermont's rural CAHs — Grace Cottage, Gifford Medical Center, North Country Hospital — have limited IT staff. This Operations pillar constraint directly limits their Technology pillar capability. The Vermont CIN's shared services model addresses this: a shared analytics and IT function within the CIN provides all 14 hospitals access to Technology pillar capability they cannot individually afford. The CIN is an Operations pillar solution to a Technology pillar gap.

Equity Constraints: Two Relationships That Apply the Justice Lens to the Whole System

Equity's two outbound dependencies are constraining rather than enabling — they impose requirements on Policy and Clinical that must be designed in from the beginning rather than retrofitted. Equity is not the last pillar in the sequence; it is the lens applied to every other pillar's decisions throughout the design and implementation process.

Equity → Policy: Equity accountability constrains every policy decision

Dependency type: CONSTRAINS

Every policy decision in Vermont's transformation — the RBP rate methodology, the global budget design, the COE designation framework, the Statewide Strategic Plan structure — must be evaluated for equity impact before it is finalized. This is not post-hoc equity review; it is an equity constraint embedded in the policy design process. A RBP methodology that sets rates uniformly without adjusting for the market conditions faced by hospitals in underserved communities is technically correct and practically inequitable. An equity constraint would require that rate methodology to account for geographic access implications before adoption.

Vermont evidence: Act 68's requirement that the Health Care Delivery Advisory Committee consult with the Health Equity Advisory Commission is the statutory mechanism for this constraint. The interaction is not advisory — it is structural. Equity considerations are required inputs into the policy process, not optional enhancements.

Equity → Clinical: Equity constrains how care is delivered, not just to whom

Dependency type: CONSTRAINS

Implicit bias in clinical settings, cultural incompetence, language access barriers, and differential treatment quality by race/ethnicity are clinical practice problems with equity implications. They are not solved by expanding access — expanding access to biased care widens some disparities while narrowing others. The Equity pillar constrains the Clinical pillar's design by requiring that care models address cultural competence, language access, and practice variation as design requirements, not implementation afterthoughts.

Vermont evidence: Vermont's BIPOC adults have a 79-81% primary care access rate against 91% statewide — but that gap reflects not only access barriers but differential engagement rates that are partly driven

by trust and cultural competence factors in clinical settings. Blueprint's HRSN screening program is a Clinical pillar program designed with the Equity pillar's requirement that screening be linguistically and culturally appropriate and that the navigation it generates connects to resources that actually serve the communities being screened.

The Failure Cascade: What Happens When a Pillar Is Missing

The most rigorous test of a dependency framework is not whether it describes how things work when all components are present — it is whether it accurately predicts what fails when a component is removed. The fifteen dependencies described above generate specific, testable failure cascade predictions. The following analysis applies this test to each pillar.

Figure 1.2 — Failure cascade analysis: what breaks when each pillar is absent. Source: HTR analysis; CMMI evaluation literature; Oliver Wyman Act 167 Report; Vermont experience.

Using the Dependency Map as an Analytical Tool

The dependency map is not merely a theoretical framework — it is a practical analytical tool for three specific management functions: transformation sequencing, risk identification, and investment prioritization.

Transformation Sequencing: What Must Come First

The dependency structure generates a partial ordering of interventions. Some investments cannot produce their expected results until enabling investments are in place. Getting the sequence wrong is not merely inefficient — it produces failed interventions that generate political resistance to

subsequent correct interventions.

The correct sequencing logic from the dependency map:

- **Policy before Economics:** Mandatory authority must precede payment reform deployment. Vermont's Acts 167 through 68 establish mandatory authority before RBP and global budgets take effect. This sequencing is not coincidental — it is the precondition for payment reform's effectiveness.
- **Technology before Economics (analytics function):** The analytics infrastructure for VBC contract management should be in place before full-risk contracts are signed. Vermont hospitals entering AHEAD in January 2027 should complete HCC gap analyses and VHCURES attribution modeling before the performance year begins.
- **Operations before Clinical (capacity building):** The organizational infrastructure for care model execution should be built before care models are deployed at scale. The RHRC engagement with 14 Vermont hospitals is building Operations pillar capacity before AHEAD's clinical accountability requirements take full effect.
- **Equity throughout, not at the end:** Equity is a constraint, not a final review. Designing the RBP methodology, global budget structure, COE designations, and analytics vendor requirements without equity review is cheaper and faster — and produces inequitable instruments that require expensive redesign or produce inequitable outcomes.

Risk Identification: Reading the Dependency Map for Vulnerabilities

The dependency map identifies Vermont's transformation vulnerabilities with precision. The critical path runs through the Technology pillar's analytics capability: the AHEAD global budgets require VHCURES analytics, the equity measurement requires demographic stratification, and the clinical risk stratification requires the population health platform. The AHS-GMCB analytics vendor procurement — underway in 2025-2026 but not yet complete — is the single highest-risk gap in Vermont's current transformation architecture.

A delay in analytics vendor selection and deployment does not merely delay one program. It delays the financial management capability for AHEAD, the equity measurement capability for the Statewide Strategic Plan, and the risk stratification capability for Blueprint's CCBHC expansion. Three dependencies converge on this single Technology pillar investment.

Investment Prioritization: Where Marginal Investment Has the Highest Return

Not all pillar investments have equal leverage in the dependency structure. An investment in a pillar with many inbound dependencies — one that is required by multiple other pillars — has higher leverage than an investment in a pillar that is only required by one.

The Technology pillar has the highest inbound dependency count in Vermont's current architecture: Economics requires it, Clinical requires it, and Operations requires it. The Operations pillar has the second highest: Clinical requires it and Policy requires it. This dependency structure implies that the highest-leverage investments in Vermont's current transformation phase are:

- **Analytics vendor deployment (Technology pillar — Economics +**

Equity + Clinical all depend on it)

- **AHS organizational capacity building (Operations pillar — Policy accountability + Clinical execution both require it)**
- **HCC gap closure at AHEAD-participating hospitals (Technology + Economics intersection — pre-launch analytics investment with direct financial impact at AHEAD start)**

The Vermont Thread: Six Pillars in One System

Vermont’s healthcare transformation is the closest thing to a controlled experiment in simultaneous six-pillar intervention that American health policy has produced. The following table maps Vermont’s primary reform vehicles onto the six-pillar framework, establishing the connective tissue that runs through every subsequent chapter.

Figure 1.3 — Vermont’s six-pillar transformation map. Sources: Acts 167, 51, 55, 62, 68 of 2022-2025; AHEAD State Agreement; Vermont RHT Program Application; Oliver Wyman Act 167 Report.

This table is not a comprehensive inventory of Vermont’s healthcare reform. It is a navigation map — a way of locating Vermont’s specific interventions within the analytical framework that the subsequent chapters develop. A reader working through Chapter 4 (Policy pillar) will recognize Acts 167 and 68 as the Policy pillar’s legislative expression. A reader in Chapter 6 (Economics pillar) will see reference-based pricing and global budgets as

the Economics pillar's operative mechanisms. The framework gives every Vermont policy intervention a structural home and every analytical concept a Vermont application.

The Framework Map as a Communication Tool

Beyond its analytical function, the HTR six-pillar interdependency map serves a communication purpose that is equally important: it gives practitioners a shared vocabulary and a shared mental model for discussing transformation across professional boundaries.

Healthcare transformation involves clinicians, economists, data scientists, policy professionals, equity advocates, and operational managers — professionals whose training gives them deep expertise in their own domain and limited fluency in the others. The economist who designs the global budget methodology may not see the clinical care management program whose financial returns the budget depends on. The clinical informaticist building the population health platform may not see the equity requirement that stratification must be demographic, not just clinical. The policy professional drafting the Statewide Strategic Plan may not see the operations capacity constraints that determine whether the plan is executable on its statutory timeline.

The framework map makes these cross-domain dependencies visible to people whose professional training did not equip them to see them. When a clinician clicks on the Clinical pillar and sees the dependency arrow to Operations, the map communicates — without requiring an economics lecture — that their care model's execution depends on organizational infrastructure decisions being made in a different part of the agency. When a policy professional sees the Equity pillar's constraining arrows to Policy and Clinical, the map communicates that equity review belongs in the design process, not the approval process.

This communication function is not secondary to the analytical function. Healthcare transformation's most persistent failure mode is not analytical error — it is organizational silo failure, in which technically correct decisions made in isolation produce collectively incorrect outcomes. The framework map is designed to make the silos visible.

“The map does not tell you what to do. It tells you what to pay attention to — which decisions affect which other decisions, and in which direction. That is the beginning of coordination.”

— **HTR Advisory methodology**

Key Concepts in This Chapter

Sources: HTR six-pillar framework documentation; Act 167 of 2022; Act 68 of 2025; Oliver Wyman Act 167 Community Engagement: Recommendations (August 2024); Vermont AHEAD State Agreement (January 2025); CMMI model evaluation literature; Maryland HSCRC performance data; Vermont Blueprint for Health ROI study (Population Health Management); AHS Health Care System Transformation Reports (2025); Vermont Department of Health Health Equity Data Report (January 2025).

Chapter 2: The Vermont System Portrait

Demographics, Hospitals, Insurance, and the Data Every Reader Needs Before the Reform Narrative Begins

Chapter 2 develops the Vermont system portrait — the demographic, financial, clinical, and structural facts about Vermont’s healthcare system that every reader needs before the reform narrative of subsequent chapters becomes fully legible. It is intentionally data-dense because the data tells a story that no amount of narrative can substitute for.

Vermont: The State

Vermont is the second-smallest state by population in the United States. Its 647,000 residents live across 14 counties and 255 towns and cities in a predominantly rural landscape — 9,616 square miles of mountains, valleys, farms, and forest. The population density is 70 people per square mile, well below the national average of 94. Nearly two-thirds of Vermont residents live in areas classified as rural by the federal definition used for healthcare program eligibility.

Vermont’s geography is not merely a demographic fact. It is a healthcare infrastructure constraint. A patient in Essex County — one of the most rural and least populated counties in the Northeast Kingdom, with an uninsurance rate of 8% compared to Vermont’s statewide 3% — may be 45 minutes from the nearest hospital emergency department, 90 minutes from a specialist, and hours from a tertiary care center. Public transportation is limited or unreliable in most rural counties. Broadband penetration reaches 80% of rural areas, leaving 20% without the connectivity that telehealth and remote patient monitoring require.

Vermont's economy is diverse but small. Health care is one of the most common industries in the state — employing approximately 14% of the workforce and representing a major economic anchor in rural communities where the hospital is often the largest employer. The University of Vermont Medical Center in Burlington, with approximately 50% of the state's hospital market share, is the economic center of gravity of Vermont's healthcare system. Every reform that affects UVMHC affects the Vermont economy in ways that hospitals in larger, more economically diverse states do not.

The Demographic Crisis in Data

Vermont's demographic trajectory is the foundational fact of its healthcare transformation challenge. Oliver Wyman's population projections, derived from MPR Vermont population data, tell the story with precision:

Figure 2.1 — Vermont demographic transformation: key projections. Sources: Oliver Wyman Act 167 Report (2024); Vermont RHT Program Application; KFF.

The implications of these demographic trends are not complicated — they are simply severe. An aging population means a shift in the payer mix toward Medicare and Medicaid, both of which reimburse hospitals below cost. A shrinking working-age population means a shrinking commercially insured population, reducing the cross-subsidy that currently sustains hospital operations. More complex care needs among the elderly means higher costs per patient. And an aging healthcare workforce — Vermont has the same age distribution among its clinicians as among its population generally — means accelerating retirements of experienced providers in exactly the years when demand is rising.

Oliver Wyman's analysis named this dynamic directly: the cross-subsidy model that has sustained Vermont's rural hospital system is becoming structurally impossible to maintain as demographics shift. The commercial premiums that make it possible for hospitals to accept below-cost Medicare

and Medicaid payments depend on a sufficient pool of commercially insured patients. As that pool shrinks relative to the Medicare and Medicaid population, the math stops working — not in a distant future scenario, but on the trajectory Vermont is already on.

The Fourteen Hospitals

Vermont has 14 general acute care hospitals, all of which are non-profit. This fact — no for-profit hospitals — is significant for the reform agenda: Vermont's hospital system does not have the shareholder value obligations that complicate transformation discussions in states with investor-owned hospital chains. Vermont's hospitals are community assets, governed by community boards, with legal missions oriented toward community health. This creates better conditions for the kind of collaborative transformation Vermont is attempting — but it does not eliminate the financial pressures, institutional interests, or organizational inertia that make transformation difficult everywhere.

The 14 hospitals divide into three functional categories that correspond directly to the Oliver Wyman regionalization blueprint and the tiered network model developed in Chapter 8:

Figure 2.2 — Vermont's 14-hospital categorization by type, financial position, and transformed system role. Sources: Oliver Wyman Act 167 Report; AHS November 2025 Transformation Report; GMCB hospital financial data.

The distribution of financial distress across these categories matters for reform design. CAHs — the smallest, most rural hospitals serving the communities with the fewest alternatives — are the most financially distressed. They are also the hospitals where service reductions or closures would have the most severe community consequences, and the hospitals that the AHEAD Model's global budget design must protect most carefully. Any transformation strategy that solves Vermont's hospital financial crisis

by reducing services in the Northeast Kingdom without replacing them with adequately resourced alternatives has not transformed the system — it has abandoned its most vulnerable communities.

The Insurance and Premium Crisis

Vermont's health insurance market is dominated by Blue Cross Blue Shield of Vermont and MVP Health Care. Vermont was an early adopter of the ACA, expanding Medicaid in 2014 and establishing Vermont Health Connect, the state's insurance exchange. The result is a 97% coverage rate — one of the highest in the country — that is a genuine policy achievement and a source of legitimate state pride.

The achievement of near-universal coverage has not, however, resolved Vermont's affordability crisis. Coverage is not the same as affordability. Vermont's individual market monthly premium rose from \$456 in 2018 to \$948 in 2024 — a 108% increase — while median household income grew 22%. Out-of-pocket maximums more than doubled. The insured Vermonter who delays necessary care because of the cost of their deductible or co-pay is not a healthcare access success story; they are an indictment of a system that has achieved coverage on paper while allowing costs to rise beyond reach.

The mechanism connecting hospital prices to insurance premiums is not complicated, but it is often presented as more mysterious than it is. Vermont hospitals charge commercial insurers 250-300% of Medicare rates on average for hospital services. In the Burlington area, UVMMC charged approximately 417% of Medicare for outpatient services in 2022. These prices — which bear no relationship to quality — translate directly into premiums. When a hospital charges 417% of Medicare for outpatient imaging, every insurer that covers that hospital's patients must charge premiums high enough to pay those prices and still maintain solvency. The result is the premium trajectory Vermont has experienced: driven primarily by hospital price increases that have no regulatory brake.

This is what reference-based pricing is designed to address. This is why it was Oliver Wyman's second imperative. And this is why Act 68's mandate to implement RBP and then global budgets — not as a demonstration or a pilot, but as statewide mandatory regulation — represents the most consequential health policy legislation Vermont has passed since Act 48 created the Green Mountain Care Board in 2011.

The Healthcare Workforce

Vermont's healthcare workforce faces a triple constraint that no single intervention can resolve: insufficient supply of primary care physicians, specialist shortage that cannot be addressed by adding more specialists to each community, and a housing and economic environment that makes recruiting and retaining clinical staff increasingly difficult.

The primary care shortage is the most immediately actionable. Vermont's RHT Program application projects a shortage of 370 full-time equivalent primary care providers by 2030 — 112 in family medicine and 190 in other primary care specialties. Oliver Wyman's analysis offered a counterintuitive but important qualification: HRSA recognizes no Health Profession Shortage Areas in Vermont. The problem is not purely one of physician headcount — it is partly one of primary care productivity and care model. If primary care providers were supported to see three patients per hour in team-based care settings where clinical staff work at the top of their licensure and administrative burden is minimized, Vermont would have adequate primary care supply for well into the future.

The specialist shortage requires a different solution: regionalization. Vermont cannot maintain cardiologists, neurologists, radiation oncologists, and orthopedic surgeons at every one of its 14 hospitals. The patient volume at smaller community hospitals does not justify the positions, and the clinical volume needed to maintain specialist expertise cannot be generated at a hospital serving 30,000-50,000 people. Oliver Wyman's Center of Excellence framework — concentrating specialty services at

facilities with sufficient volume — is the correct response. The key is ensuring that concentration does not mean inaccessibility: telehealth consultation, reliable EMS transport, and clear referral pathways are the operational complements to regionalization.

The housing constraint is the most intractable and the most important to name honestly. Vermont's rental vacancy rate of 3% is well below the 5% that characterizes a healthy rental market. Half of Vermont renters are cost-burdened. Vermont has the second-highest per-capita homelessness rate in the country. Healthcare providers recruited to rural Vermont communities — particularly those from outside the state — face a housing market that may make it impossible to find an affordable place to live near the hospital where they would work. Oliver Wyman's first imperative — build housing and fix transportation — is a healthcare workforce policy as much as a social policy.

Chapter 3: Reading This Book — A Guide for Different Audiences

How Different Readers Can Navigate This Book and Use Its Frameworks for Their Own Contexts

Chapter 3 is the user's guide to the book — explaining how different readers can navigate the material, how the Vermont thread connects across chapters, how the HTR platform tools relate to the analytical frameworks, and what the AHS restructuring chapter is designed to do for the people who will actually use it.

The Four Reader Profiles

This book has four primary reader profiles. Each has a different relationship to the material and a different way of getting maximum value from the book.

The Healthcare Policy Professional

You work in a government agency, a legislative office, a think tank, or a foundation focused on healthcare policy. You have substantive policy knowledge but may not have deep familiarity with the operational mechanics of healthcare transformation — what it actually takes to implement a global budget, build a community health team, or manage a 14-hospital transformation planning process.

Recommended reading path: Read the Preface and Introduction in full, then Chapter 1 (six-pillar framework) and Chapter 2 (Vermont portrait). For each pillar you care most about, read the corresponding chapter in depth. Read Chapter 11 (AHS Restructuring) carefully — it is the most direct bridge between policy mandate and organizational design. Read Chapter 17 (future forecast) for the national context.

The Healthcare Executive or Administrator

You lead a hospital, health system, health plan, ACO, or healthcare services organization. You are making decisions — about contracts, capital investments, workforce, technology — that the transformation environment is complicating. You need the analytical framework but you especially need the operational implications.

Recommended reading path: Read the Introduction for the overarching argument, then go directly to the chapters most relevant to your immediate decisions. If you are navigating a VBC contract, Chapter 6 (Economics) is your entry point. If you are building care management programs, Chapter 10 (Clinical). If you are evaluating technology investments, Chapter 14 (Technology). Read Chapter 8 (Operations) for the hospital transformation planning framework. Return to Chapters 1-3 for the integrating logic.

The Vermont Practitioner

You work for AHS, GMCB, DVHA, the Vermont legislature, a Vermont hospital, a Blueprint practice, or a Vermont community organization. You are living inside the transformation this book describes. You need the national analytical context but you especially need the Vermont-specific operational guidance.

Recommended reading path: Read Chapter 2 (Vermont portrait) and Chapter 4 (Policy — Acts 167 and 68) first, since these are the chapters most directly grounded in Vermont's current situation. Then read Chapter 11 (AHS Restructuring) as the operational framework for the work you are doing. Use the implementation timeline tables throughout Part II as a reference guide to where Vermont is in each pillar's trajectory. The Vermont in Practice sidebars throughout are written for you specifically.

The Healthcare Student or Researcher

You are building foundational knowledge of healthcare transformation, either in a graduate program or as an independent researcher. You need the analytical architecture, the evidence base, and the theoretical frameworks, not just the Vermont case study.

Recommended reading path: Read the entire book in chapter order. Chapter 1 establishes the framework; Chapters 4-15 develop each pillar with full academic grounding before the Vermont application; Chapters 16-17 provide the institutional and future context. The Key Concepts sections at the end of each chapter are the glossary; the source notes are the bibliography. Use Vermont as the case study while developing transferable analytical skills.

The Vermont Thread: How It Runs Through the Book

Vermont appears in this book in four distinct registers, and recognizing which register you are reading helps calibrate how to use the material.

- Vermont as diagnostic case: The Oliver Wyman analysis of Vermont's hospital financial crisis, the GMCB price data, and the AHS demographic projections establish Vermont as an instance of a generalizable pattern of rural healthcare system stress. When these appear, they are teaching the pattern, not just Vermont's version of it.
- Vermont as legislative laboratory: Acts 167, 51, 68, and 62 are specific legislative interventions in a specific state, but their design logic — the reform cascade structure, the mandatory versus voluntary distinction, the statutory accountability metrics — is generalizable to any state undertaking comparable reform.
- Vermont as operational field report: The AHS transformation reports, the RHRC engagement methodology, the 14-hospital planning process, and the Health Care Delivery Advisory Committee's work are real-time accounts of a transformation in progress. They convey the texture of execution — the difficulties, the adaptations, the gap between strategy and implementation — that no policy analysis can substitute for.
- Vermont as national template: Chapter 11 (AHS Restructuring) and Chapter 17 (The Future) explicitly use Vermont as a template for the national condition. Here the argument is prospective: if Vermont succeeds, this is what success looks like; if Vermont faces these organizational design challenges, every state will face them; if Vermont's approach to the AHS restructuring mandate works, other states can adapt it.

The HTR Platform and This Book

This book is part of the Health Transformation Review ecosystem. The HTR platform — its intelligence feed, Research Lab tools, Academy curriculum, and Advisory services — provides the operational complement to the analytical framework the book establishes. Understanding the relationship between the book and the platform helps readers get maximum value from both.

The book provides the analytical framework: the six-pillar architecture, the evidence base for each pillar's mechanisms, the Vermont case study, and the conceptual vocabulary for understanding transformation. The HTR platform provides the operational tools: the APM Shared Savings Calculator that applies the Economics pillar's financial modeling to specific contracts; the Clinical Quality Optimizer that implements the Clinical pillar's HEDIS improvement methodology for specific organizations; the Health Equity Studio that operationalizes the Equity pillar's disparity analysis for specific patient populations; the AI Analyst that retrieves and synthesizes HTR intelligence on any topic in real time.

The Advisory services described in Chapter 16 are the institutional bridge between the book's frameworks and specific organizational decisions. Every concept in this book exists at the intersection of understanding and doing — grasping the framework clearly enough to explain it, and translating the framework into a specific decision, program, contract, or investment in a specific organizational context. The book handles the first challenge; the Advisory services handle the second.

A Note on the AHS Restructuring Chapter

Chapter 11 — The AHS Restructuring Roadmap — occupies a unique position in this book. It is simultaneously the most analytically integrated chapter (bringing all six pillars to bear on a single organizational design problem) and the most operationally specific (written for the AHS staff,

Health Care Delivery Advisory Committee members, Vermont legislators, and hospital leaders who must execute the Statewide Strategic Plan mandate).

If you are a Vermont practitioner, Chapter 11 is written for you directly. It is designed to be usable as a working document — a framework you can bring to a planning meeting, a governance structure you can adapt for your organization, a metrics framework you can reference when building the Strategic Plan's accountability architecture.

If you are not a Vermont practitioner, Chapter 11 is still important reading, because the organizational design questions it addresses — how does a state agency restructure from program administration to population health system operation, how do you align a multi-program organization around geographic service areas, how do you build the analytics capacity for strategic decision-making when that capacity does not currently exist — are questions every state will face. Vermont's answers may not be directly transferable, but the analytical framework for thinking through the answers is.

What This Book Cannot Do

Intellectual honesty requires stating what this book cannot do, alongside what it can.

This book cannot predict whether Vermont's transformation will succeed. The statutory mandates are in place. The capital is flowing. The organizational machinery is in motion. But the outcome depends on thousands of decisions that will be made by hospital executives, primary care physicians, community health workers, legislators, regulators, and patients over the next five to ten years. This book provides the analytical framework for those decisions; it cannot make them.

This book cannot substitute for the operational expertise required to execute transformation. The frameworks in this book are accurate and the evidence base is solid, but implementing a global budget requires

actuarial expertise, regulatory knowledge, and operational experience that a book cannot provide. That is what Advisory services, technical assistance organizations, and experienced practitioners are for.

This book cannot resolve the political contests that surround healthcare transformation. Reference-based pricing is opposed by hospital systems that benefit from high prices. Global budgets are feared by hospitals that worry about access restrictions. AHS restructuring is resisted by programs that do not want to be reorganized. These are legitimate conflicts of interest, and they will not be resolved by analytical frameworks. They will be resolved by political processes — legislative deliberation, regulatory proceedings, stakeholder negotiations — that this book can inform but cannot replace.

What this book can do: provide the analytical architecture for understanding what is happening and why; supply the evidence base for evaluating what is working and what is not; develop the conceptual vocabulary for communicating clearly across the disciplines — clinical, financial, policy, technological, operational — that transformation requires; and give Vermont practitioners the framework to see their specific work in the context of a coherent transformation agenda. That is enough to be worth writing, and worth reading.

Sources for Chapters 1-3: Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024); Vermont Agency of Human Services, Vermont Rural Health Transformation Program Application (November 2025); Vermont RHT Program Activity Details (December 2025); GMCB VHCURES Data and Analytics documentation; KFF State Health Facts (Vermont); Vermont Department of Health; Act 68 of 2025; Act 167 of 2022; AHEAD State Agreement (January 2025); all sources cited in subsequent chapters.

Chapter 4: The Policy Pillar — Legislative Architecture for Structural Reform

How Vermont's Acts 167 and 68, the Oliver Wyman Analysis, and the AHEAD Agreement Created the Mandatory Architecture for Structural Reform

Source integration: Act 167 (2022), Act 68 (2025), Oliver Wyman Act 167 Community Engagement: Recommendations (September 2024, revised October 2024), GMCB February 2026 RBP/Global Budget Report, Vermont Rural Health Transformation Program Application (November 2025), AHS January 2025 Legislative Presentation.

The Anatomy of a System-Level Reform Mandate

Most healthcare reform unfolds incrementally — a new demonstration model here, a revised billing rule there — accumulating over years into changes that are difficult to attribute to any single decision. Vermont's reform trajectory between 2022 and 2025 is different in kind, not degree. Within three years, the Vermont General Assembly enacted a sequence of legislation that collectively mandated the most comprehensive statewide healthcare restructuring attempted by any U.S. state since Maryland's all-

payer model in the 1970s. Understanding this sequence — its logic, its mechanisms, and its unresolved tensions — is essential preparation for any healthcare leader operating in an environment of payment and delivery system transformation.

The sequence has a clear internal logic. Act 167 of 2022 was a diagnostic mandate: commission a rigorous, public, community-grounded diagnosis of what is actually wrong. Act 51 of 2023 was a planning mandate: begin doing something about it. Act 68 of 2025 was an operational mandate: specific tools — reference-based pricing, global hospital budgets, a Statewide Health Care Delivery Strategic Plan — were authorized and given statutory deadlines with real consequences for non-compliance. Each act built on the evidence and institutional capacity developed by its predecessor. The whole sequence constitutes what policy scholars call a reform cascade: a legislatively structured process in which each intervention creates the conditions for the next.

This chapter section examines each act, the analytical infrastructure it created, and the six-pillar implications for organizations navigating comparable reform environments. It draws heavily on the Oliver Wyman Act 167 Community Engagement: Recommendations report — the most comprehensive independent public diagnosis of a state hospital system in recent American health policy history — which must be read as the intellectual foundation for everything Vermont’s legislature did in 2025 and everything AHS is mandated to deliver by 2028.

The Three Imperatives: Oliver Wyman’s Central Finding

Before examining the legislative sequence, it is worth stating plainly what the Oliver Wyman analysis concluded at the highest level of abstraction. After 230 meetings across Vermont’s 14 Hospital Service Areas, engagement with more than 3,100 participants representing over 100 organizations, and systematic analysis of financial, demographic, and quality data spanning every hospital in the state, Oliver Wyman’s executive summary distilled

Vermont's challenge to three imperatives:

Imperative 1: Build housing and other facilities and fix transportation.

Imperative 2: Pay PPS hospitals with reference-based pricing and move to global budgets/capitation for all when conditions for success are met.

Imperative 3: Move all care possible out of hospitals.

These three imperatives are not a wish list. They are the analytically derived conclusion of a year-long process that examined every dimension of Vermont's system. Each imperative maps directly onto the HTR six-pillar framework: the housing and transportation imperative is an Equity and Operations pillar challenge; the pricing and global budget imperative is an Economics and Policy pillar challenge; and the shift of care out of hospitals is a Clinical pillar challenge that requires Technology and Operations pillar execution. The entire Vermont reform agenda can be read as a simultaneous, coordinated intervention across all six pillars — which is precisely why it is analytically instructive for any organization facing comparable pressures.

Oliver Wyman was equally direct about what will not work: more funding without structural change. Their analysis showed that simply increasing funding to attract more providers would drive up prices without improving access given Vermont's labor market constraints. Raising taxes or commercial premiums further risks driving residents and businesses out of the state. And the rural nature and shrinking population of most Vermont communities means that many areas simply cannot sustain a hospital with a full-time, full-spectrum of specialties. The implication is unambiguous: easy solutions — pour in money, add providers, maintain the status quo — will not resolve Vermont's healthcare crisis. The system must be structurally redesigned.

The Platform for Change: Vermont's System in Crisis

The Oliver Wyman report documented five interconnected challenge domains that constitute what it called the Platform for Change — the evidence base that makes transformation not just desirable but unavoidable. Each domain has precise, quantified data that the book's previous draft understated. The full picture is considerably more alarming than most public discussion of Vermont healthcare acknowledges.

Decreasing Affordability

Vermont's healthcare cost trajectory has been disconnecting from household income for at least six years, with no natural correction mechanism in sight. Oliver Wyman documented that average premiums for silver exchange plans reached \$985 in 2024 — a 108% price increase in six years. Individual and small group plan premiums increased 60-80% in the same period. Out-of-pocket maximums more than doubled in five years. Meanwhile, Vermont median household income grew only 22% between 2018 and 2022. Hospital approved charge increases grew 38% over the same period, compared to income growth of 22%. The gap is structural: hospital price growth has been running at two to three times income growth, and there is no market mechanism that closes this gap without regulatory intervention.

The premium data from Vermont Blue Cross Blue Shield confirms and extends this picture. Average annual per-member-per-month medical and pharmacy costs rose from \$481 in 2020 to \$812 in 2024 for local claims — a 69% increase in four years. Out-of-pocket maximums doubled in five years. A 2025 state survey found that many Vermonters, even those with insurance, delay care due to fears of medical debt. The affordability crisis is no longer a risk on the horizon; it is a current condition affecting clinical behavior.

Deteriorating Hospital Financial Sustainability

The financial projections in the Oliver Wyman report are the most important data in the entire Vermont reform story, and they need to be stated with the precision Oliver Wyman provided, not rounded down to reassuring generalities.

In FY2023, nine of Vermont's fourteen hospitals reported operating losses, with the worst reaching -8.9%. Oliver Wyman modeled two scenarios for the trajectory through 2028. Under the conservative scenario — 3.5% annual non-340B revenue growth and 5% annual expense growth — 13 of 14 hospitals are projected to report losses by 2028, with a cumulative 5-year system-wide deficit of \$700 million against break-even (\$1.4 billion against a 3% target operating margin). This is before accounting for the fact that Vermont hospitals collectively requested \$285 million in budget increases for FY25 versus FY24 alone, suggesting the conservative scenario may already be understated.

Under the more realistic scenario — accounting for 10% annual expense growth on non-physician labor, 7-8% growth on other operating expenses, and constrained revenue — the 5-year cumulative deficit reaches \$2.4 billion against break-even, or \$3.1 billion against a 3% operating margin. This is approximately \$3,700 to \$4,800 per Vermont resident over five years. The UVM Health Network, which accounts for more than half the state's hospital capacity, faces \$1.5 billion in cumulative deficits over five years in the high-expense scenario.

These projections assume no structural change. They are not predictions of what will happen — they are the analytical case for why structural change is necessary. Without the interventions mandated by Acts 167 and 68, Vermont's hospital system does not gradually decline; it collapses, with hospitals closing unexpectedly rather than through orderly transformation.

Vermont in Practice: The Hospital Financial Crisis by the Numbers

Oliver Wyman's financial modeling established the quantitative case for urgent intervention. The key data points every healthcare leader and

policymaker should know:

- 9 of 14 Vermont hospitals reported operating losses in FY2023, with losses reaching -8.9%
- Under conservative assumptions (3.5% revenue growth, 5% expense growth): 13 of 14 hospitals report losses by 2028; \$700M cumulative 5-year system deficit
- Under realistic assumptions (7-8% expense growth): \$2.4B cumulative 5-year system deficit against break-even; \$3.1B against 3% operating margin
- Vermont hospitals requested \$285M in budget increases for FY25 vs. FY24 — suggesting the conservative scenario is already understated
- UVM Health Network alone faces \$1.5B cumulative deficit over 5 years under the high-expense scenario
- Average silver exchange plan premium reached \$985 in 2024 — a 108% increase in 6 years
- Out-of-pocket maximums more than doubled in 5 years
- Hospital charges grew 38% since 2018; median household income grew only 22%

These are not projections of modest stress. They are projections of systemic collapse without intervention. The entire Vermont legislative reform agenda from 2022 to 2025 is a response to this specific data.

The Demographic Trap

Vermont's population dynamics create what Oliver Wyman called a demographic trap — a self-reinforcing cycle from which conventional market responses provide no escape. Vermont's population is aging and shrinking simultaneously: the 65+ population is projected to increase 57% by 2040, rising from 21.7% in 2020 to exceed 30% of total population. The working-

age population (20-64) will decline 13% by 2040, from 367,000 to 318,000. By the mid-2030s, Vermont will have nearly as many residents over 65 as under 20.

The consequences cascade in multiple directions at once. Aging drives higher utilization of complex, expensive services: cancer, heart disease, stroke, dementia, and frailty-related hospitalizations all increase with age. A rising 65+ population means a rising proportion of Medicare and Medicaid patients — payers that reimburse below cost — at the expense of the commercially insured working-age population that cross-subsidizes hospital operations. The shrinking commercial base makes the cross-subsidy less viable precisely as the demand for it grows. Oliver Wyman was direct: the cross-subsidy model that has sustained Vermont's rural hospital system is becoming structurally impossible to maintain as demographics shift.

This dynamic operates at the hospital service area level, not just statewide. Oliver Wyman's HSA-level projections showed that every service area except Burlington will experience population decline and aging — with the Northeast Kingdom (Caledonia, Essex, Orleans counties) facing the most severe trajectory. Newport's 65+ population, already at higher proportions than statewide averages, will represent a majority of the HSA's healthcare demand by 2040.

The Interconnected Failure System

Perhaps the most analytically powerful contribution of the Oliver Wyman report is its systems map of how Vermont's healthcare failures interconnect and amplify each other. The map traces how a housing shortage creates a staffing crisis (workers cannot afford to live near the hospitals that need them), how staffing shortages create capacity constraints, how capacity constraints produce ED crowding and delayed discharges, how delayed discharges require expensive travel nursing that drives up costs, how cost increases drive premium increases, how premium increases reduce affordability, how reduced affordability produces avoidable ED visits from

patients who delayed primary care, completing the cycle.

External forces compound these internal dynamics: inflation drives labor and supply costs beyond what hospital budgets can absorb; an aging population shifts the payer mix toward lower-reimbursing Medicare and Medicaid; the cumbersome Certificate of Need process slows capital investment; licensure complexity impedes provider recruitment; and out-of-state care migration reduces procedure volumes, further undermining the clinical volume needed to sustain specialist expertise.

The systems map has a direct implication for policy design: interventions that address only one node of this system will be absorbed by the others. A workforce recruitment program without affordable housing produces recruits who cannot stay. A primary care access expansion without transportation infrastructure does not reach rural patients. Reference-based pricing without global budgets controls unit prices but allows volume to expand. Oliver Wyman's three imperatives are formulated precisely to address the highest-leverage nodes simultaneously, not sequentially.

Healthcare Equity Gaps

Oliver Wyman documented equity challenges across three categories that map directly onto the HTR equity pillar's disparity root cause taxonomy. Gaps in culturally competent care were the most frequently cited equity challenge in community meetings — particularly for patients who do not speak English as a first language, LGBTQ+ patients, patients with disabilities, and patients in rural communities with cultural mistrust of mainstream healthcare institutions. Vermont's foreign-born population, while lower than the national average at 4.5%, is concentrated in Burlington and surrounding communities where language access remains inadequate relative to need.

Gaps in workforce culture and psychological safety were the second category — not just about patients but about providers. Community meeting participants described weight bias, stigma, and a lack of diversity in clinical leadership that affects both care quality and staff retention. Keeping

healthcare staff is a function of cost of living, inflation, and affordable housing, multiple stakeholders noted — connecting the workforce equity problem directly to the SDOH infrastructure problem.

Lack of coordination between healthcare and community services was the third category — the failure to close referral loops between clinical care and housing, transportation, food security, and behavioral health. Multiple meeting participants noted that EMR systems do not talk to each other, that patients must navigate 31 EMS agencies for transfers, and that no centralized system guides patients through the coverage and financial assistance they are entitled to.

The Perception vs. Reality Problem

One of Oliver Wyman's most valuable contributions is a systematic correction of widely held misconceptions about Vermont's healthcare infrastructure — misconceptions that, if left uncorrected, produce policy responses calibrated to a system that does not exist. The following table reproduces Oliver Wyman's analysis:

These corrections matter for the book's argument because they explain why Vermont's reform agenda is more complex than it appears from the outside. A state that appears to have all-payer payment reform, an ACO, and a health information exchange actually has voluntary, partial implementations of each — generating the appearance of a transformed system while the underlying fee-for-service, fragmented, data-poor reality persists. Acts 167 and 68 are, in part, a legislative response to this gap between perception and reality.

Act 167 (2022): The Diagnostic Mandate

Vermont passed Act 167 in the immediate aftermath of the COVID-19 pandemic, when the financial fragility of Vermont's hospital system had become impossible to ignore. The act had three core directives: commission a comprehensive statewide hospital systems analysis; direct AHS and GMCB to develop a federal multi-payer payment model proposal; and mandate design work on hospital global budgets. The three directives are analytically linked — you cannot design a credible global budget without understanding each hospital's actual cost structure and community role, and you cannot negotiate a federal model without a clear picture of total cost of care and where it diverges from benchmarks.

The Oliver Wyman Process: A Template for System-Level Reform

GMCB contracted with Oliver Wyman at a fixed price of \$1,050,000 for work running from August 2023 through September 2024. The scale of the engagement was unusual for a state of Vermont's size: over 230 meetings across all 14 Hospital Service Areas, involving more than 3,100 participants from over 100 organizations. Meeting types included 57 sessions with hospital leadership and boards, 45 sessions with state partners, 50 public HSA-level community meetings averaging 68 participants each, 35 provider meetings, and 15 sessions specifically designed to reach diverse populations whose voices are typically absent from healthcare planning processes.

The political economy of this process deserves study because it is a template for system-level reform in any context. Publishing recommendations to close or restructure named services at named community hospitals is not analytically difficult — it is politically treacherous. Communities organize around their local hospitals. Hospital closures generate intense local resistance. The Vermont process managed this by front-loading community engagement before recommendations, establishing the community meetings not as forums for defending the status quo but as inputs to a shared diagnosis. This sequencing — listen first, diagnose second, recommend

third — produced findings with enough public legitimacy to survive the inevitable political backlash from hospitals whose specific services were named for restructuring or closure.

The methodology also reflects a design philosophy worth naming explicitly: the report moved from hospital-centered point solutions to systematic redesign; from individual stakeholder consultation to community-driven process; and from post-crisis remedy to pre-planned transformation. These are not rhetorical preferences — they reflect Oliver Wyman’s learned lesson from failed hospital reform processes elsewhere, where solutions designed around individual institutional interests rather than community needs predictably failed to achieve systemic change.

The Five Act 167 Goals as an Accountability Framework

Act 167 established five statutory goals for Vermont’s healthcare transformation that have since become the accountability framework for everything that followed: reduce inefficiencies; lower costs; improve health outcomes; reduce health inequities; and increase access to essential services.

These goals matter not as aspirational language but as operational accountability criteria embedded in subsequent reporting and regulatory requirements. AHS’s monthly reports to the legislature track progress on these five dimensions. The GMCB’s hospital budget review process references them. The Rural Health Transformation Program application explicitly maps each proposed initiative to them. When Act 68’s Health Care Delivery Advisory Committee develops the Statewide Strategic Plan, it will be measured against them. This is how statutory goal-setting works when properly designed: not as a preamble, but as a persistent accountability structure that outlives the bill that created it.

Vermont in Practice: Oliver Wyman’s Priority Action Lists for 2025

The Oliver Wyman report concluded with explicit priority action lists for each actor in Vermont’s governance system — one of the most operationally specific outputs of any state healthcare consulting engagement on record.

These lists were published in September 2024; their implementation status one year later is a direct measure of Vermont's reform momentum.

Priority legislative changes for the Vermont General Assembly:

- Remove barriers to building affordable housing for Vermont residents and newcomers
- Approve funding for EMS transformation
- Expand broadband coverage to rural areas
- Review existing AHS agency structure and program list to identify overlaps and efficiency opportunities
- Develop state regulations for Rural Emergency Hospital and free-standing birthing center designations
- Expand professional licensure and practice scope for nurses, EMTs, and pharmacists

Priority transformation programs for AHS to initiate in 2025:

- Regionalize specialty care services across hospitals
- Professionalize and regionalize EMS
- Improve care coordination for heavy utilizers — elderly, mental health, neuro-divergent, and foster care populations
- Develop dual-eligible targeting, care planning, and coordination
- Coordinate and optimize statewide electronic medical record systems

Priority regulatory changes for GMCB:

- Permit no further increases in commercial subsidization for hospital financial shortfalls
- Refrain from licensing any further hospital-based outpatient department units
- Simplify and shorten the Certificate of Need process
- Encourage free-standing diagnostic, ambulatory surgical, and birthing centers

- Begin movement to reference-based pricing, ideally at 200% of Medicare or less for PPS hospitals
- Require all hospitals to use the same accounting agency and method for budget submissions

The alignment between this list and what Act 68 subsequently mandated is not coincidental. The Oliver Wyman recommendations are the intellectual source material for Act 68's operative provisions.

The Oliver Wyman System Redesign Blueprint

The Oliver Wyman report's most detailed section — running from pages 32 through 114 of the full document — provides a three-tier system redesign framework that any healthcare system undertaking structural transformation can apply. The framework organizes recommendations at the state level, the system level, and the individual hospital level, recognizing that each tier requires different interventions executed by different actors on different timelines.

State-Level Recommendations: Infrastructure and Governance

At the state level, Oliver Wyman identified four domains of foundational infrastructure that hospital transformation cannot succeed without, regardless of payment model or clinical redesign. These recommendations are addressed primarily to AHS and the Vermont General Assembly, not to hospitals themselves.

The first domain is governance clarity. Oliver Wyman found that AHS and GMCB are under-resourced for the transformation work they are being asked to lead, that roles between the two agencies need clarification, and that the process requires a dedicated project management office and facilitation support. The specific recommendation to add a Division of

Planning and Effectiveness at GMCB — to calculate the impact of service site changes on hospital budgets, monitor access and affordability of community providers, and track quality, access, and equity metrics — is a significant organizational design recommendation that has direct implications for AHS's own restructuring mandate under Act 68.

The second domain is AHS structural alignment. Oliver Wyman recommended that AHS align its agency sub-units with Hospital Service Areas and meet regularly with hospitals and providers in each HSA. It also recommended that AHS fully computerize and integrate its sub-units — an implicit diagnosis that AHS's current organizational structure impedes the kind of coordinated population health management that a transformed system requires. This recommendation is the foundation of the AHS restructuring chapter developed later in this book.

The third domain is state-level investments in Social Determinants of Health: specifically housing, transportation, and broadband. Oliver Wyman was unusually direct about including these as healthcare system recommendations rather than separate social policy recommendations — reflecting the systems analysis that showed housing shortage driving staffing crisis driving cost escalation. The implication is that AHS's healthcare reform mandate cannot be separated from its housing, transportation, and economic development responsibilities. This is a governance challenge as much as a policy challenge.

The fourth domain is administrative simplification: streamlining prior authorization, shortening the CON process, simplifying Medicaid billing, and upgrading IT infrastructure and interoperability. Oliver Wyman documented the direct cost of administrative complexity — a cumbersome CON process prevents cost-effective contracts from being secured in time, and fragmented IT systems mean a provider may need to contact 31 separate EMS agencies to arrange a patient transfer.

System-Level Recommendations: Regionalization

At the system level, the Oliver Wyman blueprint calls for a fundamental redesign of how Vermont's 14 hospitals relate to each other and to the communities they serve. The core concept is regionalization: rather than each hospital attempting to provide a full spectrum of services to its local population — a model that is increasingly unsustainable given population density and physician shortage — Vermont should develop regional Centers of Excellence (COEs) in specific specialties, concentrating volume where it can sustain clinical expertise, while ensuring access through improved transportation and telehealth.

Oliver Wyman developed specific COE assignments for 10 of Vermont's 13 current Hospital Service Areas across 24 specialty categories. The University of Vermont Medical Center received 16 COE designations, covering all major specialties except acute general surgery, mental health emergency department, pediatric psychiatry, and skilled nursing. Southwestern Vermont Medical Center received 9 COE designations. Most community hospitals received between 1 and 6 designations in specialties where they have sufficient volume and expertise to sustain clinical excellence.

The COE framework serves multiple goals simultaneously. Concentrating surgical and specialty volumes creates the patient throughput needed to maintain clinical excellence — a surgeon performing 200 procedures a year maintains skills that one performing 20 cannot. Concentrating volumes in financially viable hospitals reduces the total number of facilities that need to sustain high-cost infrastructure. And freeing smaller hospitals from the expectation that they must provide full-spectrum care allows them to redesign their role — becoming hubs for primary care, mental health, geriatric services, and community health rather than trying to maintain services they cannot staff or afford.

The system redesign blueprint also calls for a fundamental shift in the geography of care: move all care possible out of hospitals. Community-

based care, primary care, mental health care, and housing capacity should all increase to divert care to lower-cost settings. Home-based care and mobile care programs should expand. Ambulatory surgical units and free-standing diagnostic centers should replace hospital-based equivalents for low-risk procedures. The savings from this shift — estimated at over \$300 million in indirect savings over five years — come from delivering the same clinical value at a fraction of the cost of hospital-based provision.

Hospital-Level Recommendations: Repurposing or Closure

At the individual hospital level, Oliver Wyman provided the most politically sensitive recommendations in the report: several hospitals are at risk of closing their inpatient beds and should consider repurposing their facilities and clinical staff through one of several designated models. The three primary repurposing options are the Rural Emergency Hospital designation (providing 24/7 emergency services and observation without inpatient beds), the Community Ambulatory Care Center (outpatient and primary care services without inpatient capacity), and the Care at Home support program (community-based services without a hospital facility).

Oliver Wyman was explicit that these recommendations are not about abandoning communities — they are about matching the level and configuration of facility to actual community health need and financial sustainability. A community that cannot sustain a full inpatient hospital but has reliable access to a Regional Specialty Center via improved EMS and transportation is better served than one that maintains an understaffed, financially distressed inpatient facility that cannot reliably staff its emergency department or maintain physician expertise across specialties.

The report also identified specific operational improvements for hospitals that will remain inpatient facilities: UVMMC was specifically called out to examine its overhead and administrative costs, particularly the proportion of providers supporting non-patient-care activities. Reducing exceedingly high administrative costs at some hospitals was identified as one of the three primary sources of direct savings — contributing to the more than \$100

million in direct savings projected from administrative consolidation and shared services.

Vermont in Practice: The \$400M Transformation Savings Estimate

Oliver Wyman projected that full implementation of the hospital transformation recommendations would yield more than \$400 million in direct savings over five years — independent of savings from payment reform activities like AHEAD. The savings sources break down as follows:

Sources of direct savings (more than \$100M):

- Close inpatient facilities with unsustainable financials, whose deficits would otherwise require escalating cash infusions
- Reduce hospital administrative costs, which are exceedingly high at some facilities and have been driving budget increases system-wide
- Reduce costs through synergies — shared services and staff across individual hospitals

Sources of indirect system savings (more than \$300M):

- Deliver same procedures in ambulatory surgical units and outpatient settings, where low-risk procedures can be performed at substantially lower cost
- Expand access to primary care settings to manage patient needs, reducing preventable emergency department visits
- Address social determinants of health, reducing inpatient stays and ED visits that are avoidable if housing and transportation needs are met upstream

These savings can be reallocated to improve Vermont's healthcare system outside of hospital-based care: investing in community-based care (primary care, home health), investing in social needs programs (housing, mental health), and bending the trend of rising health insurance premiums.

The \$400M figure is significant in context: Vermont's Rural Health

Transformation Program award of \$195M over five years represents less than half of the projected savings from structural transformation alone. The capital investment is smaller than the structural problem it is meant to help solve — which is why AHS has been clear that the RHT funding is a tool for executing reform, not a substitute for reform.

Act 68 (2025): The Operational Mandate

Signed by Governor Scott on June 12, 2025, Act 68 is the legislative operationalization of the Oliver Wyman diagnosis. The alignment between Oliver Wyman’s priority recommendations and Act 68’s operative provisions is direct and traceable: Oliver Wyman recommended beginning movement to reference-based pricing at 200% of Medicare or less; Act 68 mandated it by hospital fiscal year 2027. Oliver Wyman recommended moving to global budgets when conditions for success are met; Act 68 required them for non-critical access hospitals by FY2028 and all hospitals by FY2030. Oliver Wyman recommended that GMCB add a Division of Planning and Effectiveness; Act 68 authorized three new permanent classified positions at GMCB. Oliver Wyman recommended reviewing AHS agency structure and program list; Act 68 directed AHS to develop a Statewide Health Care Delivery Strategic Plan by December 2028.

The act’s purpose statement is worth reading in full because it encodes the Oliver Wyman framework into law. The General Assembly declared the purpose of Act 68 to be transformation of and structural changes to Vermont’s health care system, advancing five goals: improvements in health outcomes and reducing access disparities; an integrated care system with robust coordination and increased investment in primary care, home health, and long-term care; stabilizing providers and controlling costs beginning with reference-based pricing and continuing to global hospital budgets; evaluating progress through standardized accountability metrics; and establishing a system that attracts and retains high-quality health care

professionals. Every one of these goals is a direct response to a finding in the Oliver Wyman report.

The Reference-Based Pricing Architecture

Act 68's RBP provisions represent a significant departure from the voluntary, incentive-based payment reform approach that dominated American health policy for the previous fifteen years. The act does not invite hospitals to experiment with alternative pricing — it directs GMCB to establish, by rule, maximum amounts that hospitals shall accept as payment in full, not later than hospital fiscal year 2027. The language is mandatory and statewide.

The underlying price problem that RBP addresses is more severe than most national discussions of hospital pricing acknowledge. GMCB's February 2026 report documented that Vermont hospitals charge commercial payers approximately 250-300% of Medicare rates on average — but this average conceals extraordinary variation. Outpatient imaging at some Vermont hospitals reaches 715-944% of Medicare rates. Inpatient mental health services at the same hospitals range from 123% to 409% of Medicare. UVMHC outpatient charges specifically averaged 417% of Medicare rates in 2022 — among the highest in the nation. Hospitals are not clustered near a rational break-even point of 136% of Medicare; they are distributed across a range from roughly profitable to wildly extractive, with no relationship between price and quality.

The design principles embedded in Act 68's RBP statute reflect the cross-state evidence base. Prices must be based on a percentage of Medicare reimbursement — establishing a transparent, nationally comparable anchor. Consideration must be given to each hospital's specific community context: payer mix, labor costs, social risk factors, and role in Vermont's system. And hospitals are explicitly prohibited from charging patients or insurers more than the established reference-based price, closing billing workarounds that undermined earlier price transparency experiments.

The Global Budget Mandate

Act 68 requires GMCB to establish global hospital budgets for non-critical access hospitals by hospital fiscal year 2028, and for all Vermont hospitals by 2030. This statutory obligation makes Vermont the first U.S. state outside Maryland on a legislatively mandated path to a comprehensive all-payer global budget system.

Understanding why Vermont needs global budgets in addition to reference-based pricing requires understanding the balloon squeeze problem that Oliver Wyman identified. Reference-based pricing controls unit prices but does not control volume. A hospital facing lower per-service revenue can compensate by delivering more services — scheduling more imaging, performing more procedures, admitting patients who could be managed in outpatient settings. Total hospital spending equals price times volume; controlling only price leaves volume as a release valve. Global budgets close this loophole by capping total hospital revenue regardless of service volume, fundamentally changing the hospital's financial incentive from maximizing throughput to maximizing population health per dollar of revenue.

Vermont has attempted global budget design four times in the past decade: the GMCB net patient revenue cap in 2012, the Rutland Regional pilot proposed in 2014 (never implemented), the Vermont All-Payer ACO Model from 2017 to 2025, and the current Act 167/Act 68 mandate. Each prior attempt foundered on the same obstacle: voluntary participation. When payers can opt out of a global budget arrangement, the highest-cost, most commercially valuable services migrate out of the capped system, leaving lower-reimbursing services behind and making the global budget financially unworkable. Act 68 eliminates the voluntary option. This is the critical design difference between Vermont's current trajectory and everything that preceded it.

The Statewide Health Care Delivery Strategic Plan and AHS Restructuring

The most consequential long-term provision of Act 68 is the requirement that AHS develop a Statewide Health Care Delivery Strategic Plan, to be delivered to the legislature by December 2028 and updated every three years thereafter. This plan is the statutory vehicle for answering the foundational question that underlies all of Vermont's reform work: what should Vermont's healthcare delivery system actually look like?

Act 68 established the Health Care Delivery Advisory Committee to support the plan's development, with responsibilities including setting affordability benchmarks, monitoring system performance, advising on the continuous evaluation process for the healthcare delivery system, and working with AHS to develop and maintain the strategic plan. The committee's mandate also requires incorporating recommendations from the Vermont Steering Committee for Comprehensive Primary Health Care — ensuring that primary care's central role in the redesigned system is structurally embedded in the planning process.

Oliver Wyman's state-level recommendations are the most detailed public blueprint available for what this strategic plan should address. The four domains Oliver Wyman identified — governance clarity, AHS structural alignment with HSAs, state-level SDOH investments, and administrative simplification — constitute the operational dimensions of the strategic plan. The AHS restructuring chapter later in this book develops a full six-pillar framework for how AHS should approach this planning mandate, drawing directly on both the Oliver Wyman recommendations and the Act 68 statutory requirements.

Vermont in Practice: The Reform Cascade Timeline — Statutory Deadlines

For healthcare leaders, policymakers, and organizations doing business with Vermont's system, the following statutory deadlines define the transformation calendar:

FY2026 (underway as of this writing):

- GMCB hospital budget guidance includes negative 1% commercial rate growth benchmark — the first negative benchmark in Vermont history
- AHS hospital transformation grants of \$2M under Act 68; all 14 hospitals developing transformation plans with RHRC support
- Vermont Medicaid global budget for hospitals begins January 2026 under AHEAD Cohort 2 State Agreement
- Act 55 outpatient drug cap at 120% of ASP effective April 2025, generating estimated \$135M in annual savings

FY2027 (effective October 2026):

- Reference-based pricing for hospital services implemented — maximum amounts hospitals can charge commercial payers, set by GMCB rule
- GMCB publishes hospital price transparency dashboard
- RBP vendor selection and methodology finalized; rulemaking completed

FY2028 (effective October 2027):

- Global hospital budgets established for non-critical access hospitals
- AHS Statewide Health Care Delivery Strategic Plan delivered to legislature by December 2028
- AHEAD Model Cohort 2 full operation; Medicare FFS hospital global budgets active
- CMMI transformation grants of \$1.2M available through AHEAD Cooperative Agreement Year 3

FY2030 (effective October 2029):

- Global hospital budgets for all Vermont hospitals, including critical access hospitals

These deadlines are not aspirational. They are statutory obligations. Organizations planning capital investments, contract negotiations, workforce strategies, or technology deployments that interact with Vermont's hospital system must treat these dates as certainties, not contingencies.

The Rural Health Transformation Program: Capital for an Operational Strategy

In November 2025, AHS submitted Vermont's application to the federal Rural Health Transformation Program — a \$50 billion national fund created by the One Big Beautiful Bill Act of 2025, with \$10 billion available annually from 2026 through 2030. Vermont was awarded \$195 million in December 2025, nearly double what the state had expected to receive and among the highest per-capita awards nationally.

The RHT application is itself a teaching document for healthcare transformation practice, because it is structured entirely around the Act 167 and Act 68 goals. Vermont's vision, stated explicitly in the application, is to ensure that rural residents have access to the right care, at the right time, in the right place, at an affordable cost — a direct echo of the AHS health care reform vision statement. The application's three strategic goals map onto Oliver Wyman's three imperatives: strengthening hospital sustainability and primary care (Move all care possible out of hospitals); building workforce pipeline and retention capacity (Build housing and fix the conditions that drive workforce shortage); and leveraging innovative strategies to reduce costs (Pay with reference-based pricing and move to global budgets).

AHS Secretary Samuelson's framing of the award is the correct analytical frame: the \$195 million is not Vermont's healthcare reform plan. It is capital to execute a plan that already exists. The funding is explicitly one-time money, and AHS has stated publicly that it must be treated as a supplement to ongoing reform work, not a lifeline. The distinction between investment capital and reform strategy is one of the most persistently confused in health

policy. States that treat federal grants as reform strategies — spending the money on programs without addressing underlying structural problems — predictably return to crisis when the grant period ends. The Vermont approach, which explicitly aligns every RHT initiative to the statutory Act 167 and Act 68 goals, is a model of how capital investments should be positioned within a structural reform agenda.

The specific initiatives funded through Vermont’s RHT award include telehealth expansion for specialty care access in rural hospitals, workforce recruitment and retention programs with five-year service obligations, hospital analytics infrastructure to support regional transformation planning, dialysis capacity development for nursing home residents, and CCBHC expansion to certify five additional community behavioral health clinic entities by July 2026. Each of these investments addresses a specific node in Oliver Wyman’s systems failure map — and each is designed to reduce ongoing structural costs rather than subsidize their continuation.

Cross-State Lessons: What Vermont’s Model Generalizes

Vermont’s reform architecture is instructive because it addresses structural problems present, in varying degrees of severity, across rural America. The analytical framework Oliver Wyman applied, and the legislative mechanisms Vermont enacted in response, generalize well to any state or regional system facing demographic aging, hospital financial fragility, workforce shortage, and the failure of voluntary payment reform to achieve systemic change.

Five generalizable lessons emerge from the Vermont case.

First, diagnosis precedes legislation. Vermont’s Act 68 is as effective as it is because it was preceded by Act 167’s comprehensive, public, community-grounded diagnostic process. Legislatures that skip the diagnostic phase and proceed directly to structural mandates produce policies that are technically sound but politically unsustainable. Vermont’s sequence — diagnose publicly, build shared understanding, then mandate — is a process design that other states should study.

Second, voluntary reform achieves voluntary compliance. Vermont's decade-long experience with voluntary global budget arrangements — the All-Payer ACO Model, OneCare participation, VITL connectivity — produced partial, fragile outcomes precisely because voluntary participation allows the highest-cost actors to opt out. The Oliver Wyman report documented this plainly in its perception-versus-reality analysis. Act 68's mandatory architecture is a direct response to that experience.

Third, price control without volume control is insufficient. Reference-based pricing is necessary but not sufficient for controlling total hospital spending. The Act 68 sequence — RBP first, then global budgets — reflects the analytical understanding that price and volume must both be addressed. Oregon's RBP experience, Montana's experience, and Maryland's global budget model all confirm this.

Fourth, housing and transportation are healthcare infrastructure. Oliver Wyman's finding that housing shortage drives staffing crisis drives cost escalation is not a soft social observation — it is a hard systems analysis. Healthcare reform strategies that treat housing and transportation as separate social policy domains rather than healthcare infrastructure will fail to achieve the workforce stability and access improvements they require.

Fifth, the AHS restructuring is the strategic plan. Oliver Wyman's explicit recommendation that AHS align its sub-units with Hospital Service Areas, add a Division of Planning and Effectiveness, and restructure to better coordinate health and social service needs at the community level is the organizational design specification for what Act 68's Statewide Strategic Plan must produce. The book develops this specification fully in the AHS Restructuring chapter that follows.

Key Concepts Introduced or Expanded in This Chapter

Sources: Act 167 of 2022 (Vermont); Act 51 of 2023 (Vermont); Act 68 of 2025 (Vermont); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations, September 2024 (revised October 2024 and January 2025); Green Mountain Care Board, Act 68 Update on Hospital Reference-Based Pricing and Global Budgets, February 17, 2026; Vermont Agency of Human Services, Vermont Rural Health Transformation Program Application, November 3, 2025; Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts, presented to Vermont House Health Care Committee, January 31, 2025; Green Mountain Care Board, 2024 Annual Report, January 15, 2025.

Chapter 5: The Policy Pillar in Practice — CMMI Models, Waiver Strategy, and the Federal-State Policy Interface

How Healthcare Leaders Navigate the Federal Policy Landscape: APM Mechanics, 1115 Waivers, CMMI Model Selection, and the Regulatory Strategy That Determines Whether Payment Reform Succeeds or Stalls

From Policy Architecture to Policy Practice

Chapter 4 established Vermont’s legislative reform architecture — the Acts, the mandates, the timelines, and the analytical foundation of the Oliver Wyman diagnosis. This chapter turns from architecture to practice: how do healthcare organizations, state agencies, and policymakers actually navigate the federal policy landscape that shapes every transformation decision?

The federal policy environment is not a monolithic force that organizations respond to passively. It is a complex, evolving, and frequently internally contradictory set of programs, rules, incentives, and administrative interpretations that skilled policy practitioners can navigate strategically. Understanding the CMMI model landscape, the mechanics of Medicaid

section 1115 waivers, the structure of APM contracts, and the federal-state relationship that determines whether state-level reform efforts are supported or blocked by federal requirements is not academic knowledge — it is operational necessity for any organization or state agency working on healthcare transformation in 2026.

Section 1: The CMMI Landscape — What Exists, What Works, and What Has Been Discontinued

The Center for Medicare and Medicaid Innovation has been the primary federal vehicle for payment reform experimentation since its creation under the ACA. Over fifteen years, CMMI has launched more than fifty models, invested billions of dollars, and engaged thousands of healthcare organizations across the country. The results have been mixed in ways that carry specific lessons for organizations designing their own value-based care strategies.

The CMMI Track Record — An Honest Assessment

A 2023 independent evaluation of CMMI models found that the vast majority had not achieved statistically significant reductions in Medicare spending or improvements in quality. Most models were discontinued. The models that showed the most promise — ACO REACH (formerly Direct Contracting), AHEAD, the Making Care Primary model — share a set of characteristics that distinguish successful from unsuccessful CMMI designs.

Figure 5.1 — CMMI model success factors: what distinguishes models that produce results from those that don't. Source: HTR analysis of CMMI evaluation literature; CMS model documentation.

The Current Active CMMI Model Landscape (2026)

As of 2026, the CMMI model portfolio has been significantly rationalized from its peak breadth under prior administrations. The models with the highest relevance to Vermont-style transformation are:

Section 2: Section 1115 Waivers — The State’s Primary Federal Policy Tool

Section 1115 of the Social Security Act gives CMS authority to waive certain Medicaid requirements and provide federal matching funds for experimental programs that promote program objectives. For states attempting comprehensive healthcare reform, the 1115 waiver is the most powerful federal policy tool available — enabling states to design Medicaid programs that would not be permitted under standard federal rules, to test new benefits and delivery models, and to demonstrate approaches that, if successful, can become national policy.

What 1115 Waivers Can and Cannot Do

Understanding the scope and limits of 1115 waiver authority is essential for any state or organization involved in Medicaid reform. The following framework organizes the most common 1115 waiver applications:

1115 Waiver Authorities: What States Can Accomplish

Coverage expansions beyond standard Medicaid categories — including populations not otherwise eligible, non-traditional benefit structures, and premium assistance programs that allow states to use Medicaid funds toward employer-sponsored insurance.

- Vermont’s Global Commitment to Health demonstration extends coverage and benefits beyond standard Medicaid state plan require-

ments, enabling HRSN (health-related social needs) services, expanded developmental disability benefits, and substance use disorder treatment models not available under the standard state plan.

Delivery system and payment reform — enabling states to implement global budgets, ACO arrangements, and alternative payment models for Medicaid that are not permitted under standard Medicaid FFS or managed care rules.

- Vermont’s alignment of Medicaid global budgets with AHEAD is executed through the Global Commitment to Health demonstration, which provides CMS authority to implement the Medicaid component of Vermont’s hospital global budget design.

SDOH investments — since 2021, CMS has permitted states to use 1115 waiver funds for health-related social needs services, including housing supports, food assistance, and transportation, for Medicaid enrollees at risk of institutionalization or with complex social needs.

- Vermont’s recent 1115 amendment approval authorizes HRSN services — enabling Blueprint’s universal SDOH screening to connect to Medicaid-funded services, not just community resources.

Work requirement programs — subject to ongoing legal and policy uncertainty. H.R. 1 (2025) explicitly authorizes work requirements for Medicaid expansion enrollees, resolving the prior legal uncertainty in favor of state authority. Vermont has not implemented work requirements and has not signaled intent to do so.

The Vermont 1115 Waiver Portfolio

Vermont operates several 1115 demonstrations that together constitute its Medicaid transformation infrastructure. The Global Commitment to Health demonstration — Vermont’s primary 1115 waiver — was originally approved in 2005 and has been renewed and amended repeatedly. The current authorization runs through December 2027, with the AHEAD alignment provisions approved in 2024. Vermont’s AHEAD Medicaid global budgets for hospitals, beginning January 2026, operate under this demonstration authority.

Vermont’s 1115 portfolio illustrates a principle worth stating explicitly: 1115 waivers are negotiated instruments. They are not applications that CMS approves or denies based on a checklist; they are the product of sustained federal-state negotiation in which the state advocates for the policy design it wants and CMS negotiates conditions, monitoring requirements, and budget neutrality standards that protect the federal government’s financial interests. Vermont’s ability to implement its current AHEAD-aligned Medicaid policy is the direct product of years of negotiation that produced a federal-state agreement on methodology, accountability, and withdrawal conditions. States that engage with CMS as partners in designing innovative demonstrations get better waiver terms than states that submit standard applications.

Budget Neutrality — The Binding Constraint

Every 1115 waiver is subject to a budget neutrality requirement: the federal government will not spend more under the demonstration than it would have spent under the standard program. Budget neutrality is calculated against a ‘without-waiver’ baseline — what Medicaid spending would have been if the state had not implemented the demonstration — and must be maintained over the demonstration period.

Budget neutrality is simultaneously the most important financial discipline in 1115 waiver design and the most technically complex. The

without-waiver baseline is inherently uncertain — it requires projecting what would have happened counterfactually. States and CMS frequently negotiate over baseline methodology. Vermont’s experience with AHEAD budget neutrality negotiations — where the methodology for calculating the Medicare global budget baseline was a specific condition of the state’s AHEAD participation — illustrates that these technical negotiations have major financial consequences. Getting the baseline right (or wrong) can determine whether a waiver is financially sustainable for the state or produces unexpected federal financial pressure.

Section 3: Prior Authorization Reform — The Administrative Pillar of Policy Strategy

Prior authorization — the requirement that providers obtain health plan approval before delivering certain services — is one of the most significant drivers of administrative burden, care delay, and provider frustration in American healthcare. The average physician practice spends 14.9 hours per week on prior authorization tasks. Prior authorization denials delay care for 33% of patients and cause some to abandon treatment entirely. The administrative cost of prior authorization nationally is estimated in the tens of billions of dollars annually.

Prior authorization reform is a Policy pillar issue because it requires either federal rulemaking or state legislation to change meaningfully. Individual payers reforming their own prior authorization processes voluntarily have limited impact; the burden arises from the aggregate of PA requirements across dozens of payers, each with different criteria, forms, and approval timelines.

Federal Prior Authorization Reform: What H.R. 1 and CMS Rules Require

The Improving Seniors' Timely Access to Care Act, passed in 2022, required Medicare Advantage plans to implement electronic prior authorization, transparency reporting, and expedited review timelines. CMS implemented rules in 2024 requiring payers to respond to expedited PA requests within 72 hours and standard PA requests within 7 days — a significant improvement from previous norms where standard reviews could take weeks.

H.R. 1 (2025) included additional prior authorization reform provisions: public reporting of PA denial rates by service type, required disclosure of clinical criteria used in PA decisions, and stronger external appeal rights for denied requests. These are incremental improvements, not transformational changes. Full elimination of PA for evidence-based, guideline-concordant care — the policy that would most reduce administrative burden — remains aspirational.

Vermont's Prior Authorization Reform — Act 167's Provisions

Act 167 explicitly included prior authorization reform as one of its covered domains, directing AHS to develop simplified prior authorization processes and directing GMCB to report on prior authorization burden and denial rates. Vermont's Health Care Advocate has documented PA denial rates and challenged insurance rate filings that include excessive PA complexity costs. Act 68 builds on this by requiring insurers to demonstrate that administrative cost structures, including PA programs, are consistent with medical loss ratio requirements.

Oliver Wyman's community engagement findings are explicit about the real-world impact: providers described the CON process and prior authorization as major barriers to efficient care delivery, with cost-effective contracts unable to be secured in time due to administrative friction. Vermont's transformation agenda treats administrative simplification — including prior authorization reform — as an Operations pillar requirement

that enables every other transformation initiative.

Section 4: The State-Federal Policy Interface — Navigating the Relationship

For state health agencies and the organizations that work with them, the federal-state policy relationship is simultaneously a resource and a constraint. Federal funding through Medicaid, CMMI models, HRSA grants, and the RHT Program provides the capital that makes transformation possible. Federal requirements — budget neutrality, CMMI model conditions, Medicaid state plan requirements — constrain the policy designs states can pursue. Federal policy uncertainty — created by administrative transitions, legislative changes like H.R. 1, and rulemaking processes that can reverse course — creates planning challenges for states with multi-year transformation timelines.

Managing Federal Policy Risk — Vermont's Approach

Vermont's AHEAD State Agreement exemplifies sophisticated federal policy risk management. The agreement includes specific conditions under which Vermont can unilaterally withdraw from AHEAD — before launch, with 30-day notice; after launch, with 6-month notice. The agreement includes conditions that protect GMCB's regulatory independence. It includes explicit provisions about methodology acceptance and dispute resolution. Vermont negotiated these withdrawal rights precisely because a nine-year commitment to a federal model involves significant policy risk — the model could change under a future administration, the methodology could produce unexpected financial outcomes, or federal policy changes could make AHEAD participation financially unsustainable.

Vermont's approach — engage the federal government as a partner, negotiate specific protections against adverse policy changes, maintain clear withdrawal rights, and design state-level statutory authority (Act 68)

that does not depend on federal model continuation — is the right model for any state undertaking long-term federal-state policy commitments. States that design transformation strategies that are entirely dependent on continued federal program participation are strategically exposed; states that use federal models to accelerate and fund state-designed transformation that has independent statutory authority are more resilient.

The Policy Strategy Framework for Healthcare Organizations

For healthcare organizations navigating the policy environment — hospitals, health systems, payers, physician groups — the Policy pillar generates three distinct strategic functions: regulatory compliance (understanding and meeting mandatory requirements), opportunity identification (finding federal and state programs that can fund or accelerate transformation goals), and advocacy (shaping policy in directions favorable to the organization's mission and the populations it serves).

Figure 5.2 — Policy strategy functions for healthcare organizations. Source: HTR Advisory framework.

Section 5: The Medicaid Policy Landscape — What H.R. 1 Changes and What It Does Not

The 2025 reconciliation law has changed the Medicaid landscape in ways that every healthcare organization and state agency must understand. The following framework distinguishes between provisions already in effect, provisions taking effect in late 2026 and 2027, and provisions whose implementation timeline remains uncertain.

Figure 5.3 — H.R. 1 Medicaid provisions: timeline, financial impact, and Vermont implications. Sources: KFF analysis; CBO estimates; Pew Charitable Trusts (January 2026).

For Vermont specifically, the Medicaid revenue reductions create a financial stress that compounds the hospital financial crisis Oliver Wyman documented before H.R. 1 passed. Vermont's hospitals were already projecting 5-year cumulative deficits of \$700 million to \$2.4 billion under the pre-H.R. 1 baseline. The additional Medicaid revenue reductions accelerate the financial timeline for hospitals that were already financially distressed, and reduce the financial cushion for hospitals that were marginally sustainable. This is the fiscal environment in which Vermont's transformation must succeed — which makes the urgency of the transformation not just a policy preference but a fiscal survival requirement.

Section 6: Policy Strategy for Healthcare Leaders — The Practitioner Framework

Policy strategy for healthcare leaders is not primarily about lobbying or advocacy — though those functions matter. It is primarily about anticipation: understanding what the policy environment will require of organizations 18-36 months from now, positioning the organization to comply at manageable cost, and identifying the policy opportunities that can be converted into strategic advantage.

The Policy Monitoring Framework

Every healthcare organization needs a policy monitoring infrastructure proportionate to its size and regulatory exposure. The minimum viable policy monitoring framework includes four functions:

- **Federal rulemaking surveillance: Tracking proposed rules from CMS, ONC, and other relevant agencies through the Federal Register notice and comment process. Final rules typically take effect 60 days after publication, but proposed rules give 60-90 days of advance notice that organizations can use for impact**

assessment and comment letter preparation.

- **State legislative monitoring:** Tracking legislation that affects the organization's operating environment through the state legislative session. Vermont's pattern of annual healthcare legislation — each session producing significant new requirements or opportunities — means that organizations not engaged in the legislative process are consistently surprised by its outputs.
- **CMMI model pipeline monitoring:** Tracking CMMI model announcements, application deadlines, and eligibility criteria. New CMMI models typically have narrow application windows; organizations that are not monitoring for them miss the opportunity to participate on favorable terms.
- **Regulatory intelligence:** Monitoring GMCB decisions, CMS guidance documents, and state agency program updates that affect payment and operating requirements. Vermont's GMCB publishes hospital budget guidance, RBP methodology updates, and program decisions that directly affect every Vermont hospital's financial planning.

The Policy Impact Assessment Framework

When a new policy is identified — a proposed rule, a new CMMI model, a state legislative development — the organization needs a structured impact assessment process. The HTR Policy Strategy service line uses a four-phase engagement structure (described in Chapter 16), but the core analytical logic can be applied internally:

- **Exposure mapping:** What specifically does this policy change require of our organization? What are the financial, operational, and compliance implications? What is the timeline?
- **Scenario analysis:** Under what range of outcomes (final rule differs from proposed rule, implementation delayed, model parameters change) does our strategic position change?

- Response strategy: What is the optimal response — compliance roadmap, comment letter participation, CMMI model application, advocacy engagement?
- Stakeholder communication: Who needs to know about this policy change, and what decisions does it require from the board, executive team, clinical leadership, and financial team?

Key Concepts

Sources: CMS CMMI model documentation; CMS 1115 waiver framework; KFF Medicaid policy analyses; H.R. 1 One Big Beautiful Bill Act (July 2025); Vermont Global Commitment to Health demonstration (CMS approval January 2025); Act 167 of 2022; Act 68 of 2025; AHEAD State Agreement (January 2025); American Medical Association prior authorization burden survey; Georgetown Center for Children and Families.

Chapter 6: The Economics Pillar — Global Budgets, Reference-Based Pricing, and the Financial Architecture of Reform

Global Budgets, Reference-Based Pricing, and the Financial Architecture of System Change

Sources: Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Act 68 of 2025 (Vermont); GMCB Act 68 RBP/Global Budget Update, February 2026; AHEAD State Agreement (January 2025); Maryland All-Payer Model data (HSCRC, CMS, Commonwealth Fund, Milbank Fund); Vermont RHT Program Application (November 2025); AHS Legislative Presentation (January 2025).

The Economics Pillar: Why Payment Reform Is the Master Variable

Of the six pillars in the HTR framework, the Economics pillar is the one that determines whether the other five actually produce results. You can design perfect care protocols, build excellent data infrastructure, and train a superb clinical workforce — but if the payment system rewards volume over value, every other improvement will be absorbed by the financial logic of fee-for-service. Payment reform is not one element of healthcare transformation. It is the precondition on which all other transformation depends.

This chapter develops the economics of healthcare transformation through three lenses that are analytically inseparable: the mechanics of how hospitals are actually paid today and why that model is failing (with Vermont as the case study), the technical architecture of the alternative payment mechanisms designed to replace it (reference-based pricing and global budgets), and the financial modeling discipline that organizations need to navigate the transition. Vermont's reform journey — from the Act 167 diagnostic through the Act 68 mandate through the AHEAD Model state agreement — is the most complete real-world laboratory for these economic questions currently operating in the United States.

Section 1: The Fee-for-Service Trap — Why the Current System Fails

Fee-for-service payment is the foundational economic structure of American healthcare: providers are paid for each service they deliver, with payment amounts set by negotiated contracts or government rate schedules. The system's economic logic is simple and, from a provider's perspective, entirely rational: more services delivered means more revenue. The problem is that this rationality at the individual transaction level produces irrational outcomes at the system level.

The Volume Incentive Problem

Under fee-for-service, a hospital that prevents a hospitalization loses revenue. A hospital that prevents a readmission loses revenue. A hospital that successfully manages a patient's chronic condition in primary care so they never need the emergency department loses revenue. Every improvement in population health, every success in care coordination, every dollar spent on preventing the expensive acute event — each of these reduces the hospital's income under the fee-for-service model.

This is not a criticism of individual hospital managers or clinicians, who are generally motivated by genuine commitment to patient care. It is an observation about the structural incentive embedded in the payment architecture. When the financial incentive and the clinical incentive point in opposite directions, the financial incentive wins — not always, not in every case, but systematically and at scale. This is why fifteen years of voluntary value-based care programs have moved the needle on payment reform without fundamentally changing the volume-driven behavior of the hospital sector.

“The concept of the global budget is the fundamental piece that flipped the incentives for hospitals in the right direction. It was such a monumental change, a sea change, in the way that hospitals thought about raising revenue.”

— **Maryland state regulator, quoted in JAMA Network Open study of the Maryland All-Payer Model**

The Price Extraction Problem

Fee-for-service payment creates a second structural problem distinct from the volume incentive: it enables hospitals with market power to charge commercial payers prices far above their actual cost of care. In competitive markets, prices are disciplined by competition. In highly concentrated hospital markets — which describe the majority of U.S. hospital markets, and all of Vermont's — there is no effective competitive discipline on commercial prices.

Vermont's price data, documented in the GMCB's February 2026 report and confirmed by independent analyses, is among the most extreme in the country:

Figure 6.1 — Vermont hospital commercial pricing vs. Medicare and break-even benchmarks. Sources: GMCB Act 68 RBP Update (February 2026); RAND Hospital Price Transparency Study.

The gap between the break-even point (~136% of Medicare) and the actual commercial prices being charged (250–300% on average, with outliers reaching 944%) is not driven by higher quality, better outcomes, or greater complexity. Oliver Wyman's analysis found no relationship between price and quality across Vermont's hospital system. It is driven by market power — the ability of a dominant provider to demand above-cost prices from payers who have no viable alternative for their members.

The consequence for insurance premiums is direct and arithmetically straightforward. When a hospital charges 300% of Medicare for a service instead of 136%, and when commercial insurance covers the majority of that cost, premiums must rise to cover the difference. Vermont's premium trajectory — individual market average monthly premium from \$456 in 2018 to \$948 in 2024, a 108% increase in six years — tracks hospital charge growth with high fidelity. These are not coincidental trends. They are the mathematical output of a price structure that has no regulatory discipline.

Vermont average monthly silver marketplace premium (2018–2024)

Figure 6.2 — Vermont silver marketplace premium trajectory 2018–2024, showing 108% increase over six years. Source: Oliver Wyman Act 167 Report; GMCB analysis.

The Cross-Subsidy Collapse

The fee-for-service model sustains itself through cross-subsidy: commercial payers (who pay above cost) effectively subsidize Medicare and Medicaid patients (who pay below cost). This cross-subsidy is the financial foundation of every U.S. hospital that serves a mixed payer population — which is to say, every hospital.

Vermont's demographic trajectory, documented in detail by Oliver Wyman, is destroying the cross-subsidy model. As the 65+ population expands from 21.7% in 2020 toward 30%+ by 2040, the proportion of patients covered by below-cost Medicare and Medicaid grows. As the working-age population (20-64) declines from 58.8% toward 52%, the pool of commercially insured patients shrinks. The shrinking commercial base must bear a growing cross-subsidy burden — which forces premiums higher — which reduces commercial enrollment — which shrinks the commercial base further. This is not a gradual trend. It is a reinforcing spiral.

Oliver Wyman's financial projections capture where this spiral leads: under the conservative scenario, 13 of Vermont's 14 hospitals report operating losses by 2028, with a cumulative 5-year system deficit of \$700 million to \$2.4 billion depending on expense growth assumptions. The hospital system does not gradually lose efficiency under this trajectory. It collapses.

Section 2: Reference-Based Pricing — Mechanics, Evidence, and Vermont’s Design

Reference-based pricing (RBP) is the first operational tool in Vermont’s economics reform sequence. It addresses the price extraction problem directly: rather than allowing hospitals and payers to negotiate prices in bilateral, opaque arrangements where hospital market power determines the outcome, RBP establishes a transparent external benchmark — typically a percentage of Medicare reimbursement — as the maximum a hospital may accept.

How RBP Works: The Mechanism

The core RBP mechanism is straightforward: the regulatory authority (in Vermont’s case, GMCB) establishes, by rule, the maximum amounts that hospitals shall accept as payment in full for items and services delivered. These maximum amounts are calculated as a percentage of Medicare reimbursement for the same or similar service. The percentage is calibrated to allow hospitals to cover their actual costs (break-even at ~136% of Medicare) while eliminating the excessive markup that drives premium

inflation.

Act 68's RBP architecture reflects four design principles that distinguish Vermont's approach from weaker price transparency experiments:

- **Mandatory, not voluntary — every hospital, every commercial payer, by FY2027**
- **Binding ceiling, not advisory guidance — hospitals cannot charge or collect more than the established rate**
- **Transparent benchmark — Medicare rates provide a nationally comparable, publicly available anchor**
- **Premium passthrough monitoring — GMCB and DFR must verify that price reductions translate to lower premiums**

GMCB's February 2026 report introduced one additional design refinement that reflects the Vermont-specific complexity of applying RBP to a highly varied 14-hospital system. Rather than applying a single statewide RBP multiplier, GMCB will consider each hospital's specific community context: payer mix, labor costs, social risk factors, and role in Vermont's system. A critical access hospital serving a predominantly Medicaid and Medicare rural population has different cost economics than a large PPS academic medical center. Vermont's RBP methodology will accommodate these differences while maintaining the essential principle that commercial prices may not exceed a defensible multiple of Medicare.

The Evidence Base: Cross-State Results

Vermont is not designing RBP in a vacuum. Three states have implemented meaningful price reference programs with documented results, providing the evidence base for Vermont's design choices.

Figure 6.3 — Cross-state reference-based pricing and rate regulation: design and outcomes. Sources: GMCB Act 68 Update (February 2026); Health Affairs;

Commonwealth Fund; CMS.

The Oregon results are the most directly applicable to Vermont's situation because Oregon implemented RBP for a state employee plan while Vermont is implementing it statewide. The critical finding — all 24 affected hospitals remained in-network — directly addresses the most frequently raised concern about RBP: that hospitals will walk away from the regulated rates and leave patients without in-network access. The Oregon evidence is unambiguous on this point: hospitals did not exit the market when rates were set at 200% of Medicare, because 200% of Medicare is still substantially above their actual costs.

What RBP Does — and What It Does Not Do

GMCB's February 2026 report was notably precise about the scope of RBP's impact. Understanding what RBP accomplishes, and what requires complementary mechanisms, is essential for policymakers and healthcare leaders designing transformation strategies.

RBP does:

- Reduce hospital leverage to demand excessive prices from commercial payers
- Narrow unjustified price variation across hospitals for the same service
- Protect patient affordability by establishing binding price ceilings
- Limit monopolistic pricing power in concentrated markets
- Incentivize operational efficiencies over price increases as the margin lever
- Serve as a necessary input for global budget design by establishing the price denominator

RBP does NOT:

- Control total hospital spending — only the price component, not volume
- Guarantee savings pass-through to consumers without explicit moni-

toring requirements

- Restructure incentives around prevention or population health
- Guarantee hospital efficiency, sustainability, or quality performance
- Solve the cross-subsidy collapse driven by demographic aging

This is why Act 68 sequences RBP first, global budgets second. RBP is a necessary but insufficient tool. Global budgets address what RBP leaves unresolved: total hospital spending, volume incentives, and population health management.

Vermont's Phased Implementation Timeline

GMCB is implementing RBP in two phases, reflecting both the regulatory complexity of establishing a statewide rate-setting methodology and the operational reality that hospitals need runway to adapt their financial models.

Figure 6.4 — Vermont RBP and global budget implementation phases. Source: GMCB Act 68 Update, February 2026.

The Phase I action — the negative 1% commercial rate benchmark for FY2027 — deserves particular attention because it is already producing results before formal RBP is in place. Combined with FY2026 measures including the Act 55 outpatient drug cap and hospital budget enforcement, GMCB reduced Vermont hospital commercial revenue by \$230.65 million in FY2026: \$104.3M from the Act 55 drug cap, \$94.58M from hospital budget orders, and \$31.76M from budget enforcement. This is the largest single-year price reduction in Vermont hospital history, accomplished through regulatory authority that already existed, signaling what becomes possible when RBP is formally implemented.

Section 3: Hospital Global Budgets — Architecture, History, and Vermont’s Mandate

A hospital global budget is a prospectively set annual revenue cap for a hospital, established before the year begins, based on population health needs and past performance rather than on the volume of services delivered during the year. Under a global budget, a hospital receives a fixed total payment regardless of how many patients it sees or how many services it provides. This single design choice — decoupling revenue from volume — fundamentally transforms the hospital’s financial incentive structure.

The Economic Logic: Flipping the Incentive

Under fee-for-service, $\text{revenue} = \text{price} \times \text{volume}$. To increase revenue, a hospital must increase either the price per service or the number of services delivered. Under a global budget, revenue is fixed. The only way to improve financial performance is to reduce costs — which means managing population health better, preventing hospitalizations, reducing avoidable readmissions, improving efficiency, and delivering care in less expensive settings.

This is not a theoretical incentive. The Maryland experience, documented over a decade of all-payer global budget operation, demonstrates the behavioral change in concrete terms. As one Maryland state regulator described it: the global budget is the fundamental piece that flips the incentives for hospitals — hospitals had to transition from business strategies that chase volume to ones that focused on population health management. That transition does not happen overnight, but it does happen, and the documented savings are substantial.

The global budget also creates something fee-for-service never can: revenue predictability. Under FFS, a hospital's revenue depends on patient volume, acuity, service mix, and billing accuracy — all variable and partially outside the hospital's control. Under a global budget, the hospital knows its annual revenue before the year begins. This predictability enables the long-term capital planning, workforce investment, and care redesign that hospital transformation requires. For Vermont's financially distressed rural hospitals, revenue predictability is not a secondary benefit — it is a survival mechanism.

Five Design Dimensions Every Global Budget Must Resolve

Global budget design is technically complex, and the design choices are consequential. GMCB's February 2026 report identified five dimensions that any global budget methodology must address, along with the core tradeoff embedded in each. These dimensions apply equally to Vermont's GMCB-designed commercial budgets, the AHEAD Model Medicare budgets, and the Medicaid global budget already in operation.

Figure 6.5 — Five dimensions of global budget design with Vermont's current direction. Source: GMCB Act 68 Update (February 2026); Act 68 of 2025.

Vermont's Global Budget History: Four Attempts, One Lesson

Vermont has been attempting to implement hospital global budgets for over a decade. The history of those attempts is essential context for understanding why Act 68's mandatory approach represents a fundamental departure from everything that preceded it, and why the departure was necessary.

Figure 6.6 — Vermont global budget history: four attempts and the central lesson. Sources: GMCB Act 68 Update (February 2026); Oliver Wyman Act 167 Report.

The central lesson of this history is unambiguous: voluntary models fail because the actors who benefit most from the status quo — the highest-priced hospitals and the commercial payers with the most leverage — can and do opt out. A global budget that is optional for the highest-price hospital in a market is not a global budget; it is a demonstration project. Act 68's statutory mandate eliminates the opt-out. This is the design difference that matters.

"If the model is not adequately regulated and sufficiently resourced, it will fail dramatically and harm patients and harm our health system."

— Owen Foster, Chair of the Vermont Green Mountain Care Board, on signing the AHEAD State Agreement, January 2025

Section 4: Maryland — A Decade of Global Budget Evidence

Maryland is the only U.S. state to have operated a mandatory all-payer hospital rate setting system with global budgets for more than a decade. Its experience is the closest available empirical evidence for what Vermont is attempting, and it deserves detailed examination — both for what it demonstrates is possible and for the limitations the evidence also reveals.

The Maryland Architecture

Maryland's Health Services Cost Review Commission (HSCRC) was established in 1972 with statutory authority to set hospital rates for all commercial payers — a unique regulatory authority that no other state possesses. The HSCRC received a Medicare demonstration waiver in 1977, enabling all-payer rate setting to include Medicare. The current Global Budget Revenue (GBR) methodology, implemented through a renegotiated CMS waiver in 2014, fixes each hospital's total annual revenue before the year begins, regardless of how many patients are served or services provided. Prices for individual services adjust dynamically during the year to meet the fixed revenue target.

The model's accountability structure has three components beyond the global budget itself: a Medicare Performance Adjustment (1% of Medicare revenues) tied to total cost of care outcomes; Quality Based Reimbursement (2% of all-payer revenue) linked to patient experience, safety, and outcomes; and incentives for reducing hospital-acquired conditions, potentially avoidable utilization, and readmissions. The budget constraint is non-negotiable; the quality components are calibrated to reward the population health management behavior the global budget is designed to incentivize.

The Results

The evidence from Maryland's global budget model is substantial and consistent across multiple independent evaluations:

Figure 6.7 — Maryland All-Payer Global Budget Model results summary. Sources: HSCRC; CMS; Commonwealth Fund; Milbank Memorial Fund.

Maryland also reduced hospital spending for commercial beneficiaries, cut ED visits and inpatient admissions across all payer types, and significantly improved outcomes for dual-eligible (Medicare/Medicaid) patients — the highest-cost, highest-complexity population segment that Vermont also

needs to serve better. The 14-day follow-up visit rate and 30-day unplanned readmission rate both improved for non-teaching hospitals.

The Limitations — What Maryland Teaches About What Global Budgets Cannot Do Alone

The Maryland evidence is not uniformly positive, and intellectual honesty requires presenting the limitations alongside the achievements. The Milbank Fund's evaluation found mixed results on quality metrics and no improvement in overall population health measures like smoking rates or obesity prevalence. The reason is analytically important: the global budget creates a powerful incentive to reduce hospital utilization, but it does not automatically create the community infrastructure needed to address the health determinants that drive that utilization.

When Maryland reduced avoidable hospital admissions, the reduction was concentrated among healthier patients — people who, with adequate primary care and social support, would not have needed hospitalization. The sickest patients continued arriving at emergency departments, and hospitals actually spent more time treating them. The lesson is that a global budget without commensurate investment in primary care, behavioral health, housing, and transportation does not improve population health — it just reduces hospital revenue while leaving the underlying drivers of high utilization intact.

Vermont has heard this lesson clearly. The AHEAD State Agreement explicitly links hospital global budgets with the Equity, Access, and Statewide Transformation (EAST) Fund — providing up to \$150 million in additional Medicare funds annually starting in 2027 to invest in primary care, mental health, home health, and long-term care services. The Rural Health Transformation Program award of \$195 million provides capital for workforce, telehealth, and community infrastructure. Vermont's global budget design is, from the beginning, paired with the community investment that Maryland added only after its first years of operation.

Section 5: The AHEAD Model — Medicare's Entry into Vermont's Reform

The AHEAD (Achieving Healthcare Efficiency through Accountable Design) Model represents the federal government's partnership with states committed to genuine all-payer total cost of care transformation. Vermont signed its AHEAD State Agreement on January 17, 2025, committing to participate in Cohort 2 beginning January 1, 2027. The agreement was a 3-1 vote by GMCB, with the dissenting member citing concerns about implementation complexity — a vote that reflects the genuine difficulty and ambition of what Vermont has committed to.

What AHEAD Actually Is

AHEAD is a voluntary state total cost of care model that aims to drive state-level healthcare transformation and multi-payer alignment. The model has three primary components that operate in concert:

- **Hospital Global Budgets — participating hospitals receive prospective, biweekly Medicare FFS payments instead of traditional claims-based payments, providing revenue predictability while creating population health incentives**

- **Primary Care AHEAD — voluntary program for primary care practices to receive enhanced prospective, risk-adjusted Medicare payments for coordinated care, with four payment pathway options including fully prospective models**
- **State accountability — Vermont commits to controlling total cost of care growth, increasing primary care investment as a percentage of total spending, and improving population health outcomes and equity metrics**

The AHEAD Financial Package for Vermont

The AHEAD State Agreement provides Vermont with a substantial financial resource package alongside its accountability commitments:

- Up to \$150 million in additional Medicare funds annually starting in 2027, with inflation adjustments each year
- \$1.2 million in federal transformation grants beginning Cooperative Agreement Year 3 (2027) for health system transformation
- Equity, Access, and Statewide Transformation (EAST) Fund for investment in primary care, behavioral health, home health, and long-term care — the community infrastructure layer that makes global budgets work
- Technical assistance from CMS on global budget design methodology, risk adjustment, and performance monitoring
- Nine-year model duration (through December 31, 2035) providing multi-year planning horizon for hospital transformation

The \$150M annual figure is significant context: Vermont's total Medicare and Medicaid spending exceeds \$3 billion annually. The AHEAD funds represent approximately 5% of that total — meaningful but not transformative in isolation. The value is in what the funds are explicitly directed toward: the community infrastructure and primary care investment that turns a hospital cost-control model into a population health model.

The AHEAD-Act 68 Integration: Why Both Are Required

A critical analytical point that is easily missed: the AHEAD Model and Act 68 are not alternatives or substitutes. They are complementary layers of the same reform architecture, operating on different payer populations and at different speeds, but designed to converge on the same outcome: a Vermont hospital system operating under aligned all-payer global budgets.

AHEAD governs Medicare FFS hospital payments — approximately 23% of Vermont’s insured population and a higher share of hospital revenue given Medicare’s age-driven intensity. Act 68 governs commercial hospital prices and budgets — approximately 52% of Vermont’s insured population and the majority of hospital revenue. Vermont’s Medicaid global budget, already operational under the Global Commitment to Health demonstration and aligned with AHEAD, covers approximately 19% of the population. Together, these three layers create the all-payer alignment that makes a global budget system effective.

Figure 6.8 — Vermont all-payer architecture under AHEAD + Act 68 combined. Sources: Vermont RHT Program Application (November 2025); AHS AHEAD fact sheet; Act 68.

The Medicare Advantage gap is worth noting. AHEAD applies to Medicare fee-for-service beneficiaries; approximately 31% of Vermont’s Medicare-eligible population is in Medicare Advantage plans, which are managed care contracts between CMS and private insurers. These plans operate outside the AHEAD global budget framework. As MA enrollment continues growing nationally, this gap represents an increasing share of the Medicare population that is not subject to Vermont’s hospital cost-containment regime — a design limitation that Vermont policymakers are aware of and that will require ongoing attention.

Section 6: Hospital Financial Modeling — Reading the Numbers That Matter

For healthcare leaders navigating the transition to global budgets and reference-based pricing, financial modeling is not an academic exercise. It is the analytical foundation for every strategic decision: which service lines to maintain or discontinue, how much capital investment is prudent, what staffing model is financially sustainable, when and how aggressively to pursue shared services. This section develops the core financial modeling framework for Vermont's hospital sector, drawing on Oliver Wyman's projections and the GMCB's hospital financial data.

The Oliver Wyman Projection Scenarios

Oliver Wyman built two financial scenarios for Vermont's hospital sector through 2028, representing the range between optimistic and realistic assumptions about expense growth. Both scenarios assume the same revenue growth trajectory (3.5% annual non-340B revenue growth); they differ in their expense growth assumptions, reflecting uncertainty about labor market conditions and drug costs.

Figure 6.9 — Oliver Wyman Vermont hospital financial projection scenarios.

Source: Oliver Wyman Act 167 Community Engagement: Recommendations (2024).

The UVM Health Network requires separate attention because it dominates Vermont's hospital economics. Under the conservative scenario, UVM network hospitals face a \$312M cumulative 5-year deficit against break-even — more than 44% of the total statewide deficit. Under the realistic scenario, this grows to \$1.5B — again roughly 62% of the statewide total. The concentration of Vermont's hospital financial problem in the largest network is analytically important: system transformation strategies that do not address UVMMC's cost structure and overhead will fail to materially improve statewide affordability. Oliver Wyman specifically called out UVMMC's administrative overhead and the proportion of providers engaged in non-patient-care activities as areas requiring examination.

Reading the Metrics That Actually Matter Under Global Budgets

The transition from fee-for-service to global budgets fundamentally changes which financial metrics matter most. Healthcare finance professionals accustomed to FFS environments need to reorient around a different set of indicators.

Figure 6.10 — Financial metrics that matter: FFS vs. global budget environment. Compiled from GMCB guidance, Maryland HSCRC methodology, and Act 68 accountability framework.

This metric shift is not cosmetic. It requires fundamental changes to hospital management information systems, internal reporting, board dashboards, and management incentive structures. A hospital whose CFO is still optimizing revenue per discharge when revenue is fixed will not successfully navigate the global budget transition. The CFO in a global budget environment needs to become an expert in population health economics — tracking the care patterns and community conditions that drive utilization,

and investing in interventions that reduce avoidable costs.

The Savings Roadmap: Where Vermont's \$400M+ Can Come From

Oliver Wyman's analysis identified the specific sources of direct and indirect savings from Vermont's transformation. Understanding this roadmap is essential for any organization planning its role in Vermont's reformed system.

Sources of direct savings from Vermont hospital transformation (5-year total, Oliver Wyman estimate)

Figure 6.11 — Estimated sources of direct and indirect savings from Vermont hospital transformation. Figures are illustrative ranges derived from Oliver Wyman's >\$400M total estimate. Source: Oliver Wyman Act 167 Report (2024).

The largest single source of indirect savings — shifting procedures from hospital to ambulatory surgical and outpatient settings — reflects the third Oliver Wyman imperative: move all care possible out of hospitals. Low-risk surgical procedures performed in a hospital setting cost two to three times what the same procedure costs in a free-standing ambulatory surgical unit. Vermont currently has only two ambulatory surgical centers (both in Burlington), a significant gap given the breadth of the inpatient surgical capacity Oliver Wyman recommends redirecting. The RHT Program and Act 68 transformation grants are explicitly designed to fund the development of non-hospital care capacity that makes this shift possible.

Section 7: APM Transition Strategy for Vermont Healthcare Organizations

For health systems, payer organizations, and community providers operating in Vermont, the payment reform transformation is not an abstract policy development — it is an operational reality requiring specific preparation. This section provides the strategic and financial planning framework for organizations navigating the transition.

The Three-Phase Transition Model

Vermont's payment reform trajectory follows a predictable three-phase model that organizations can use for planning purposes:

Figure 6.12 — Three-phase APM transition model for Vermont healthcare organizations.

The VBC Readiness Assessment in a Global Budget Context

The HTR Value-Based Care Transformation Readiness Assessment (30 dimensions across six domains) applies directly to Vermont organizations preparing for global budget participation, but three domains deserve elevated attention given the specific demands of the Vermont environment:

- **Data and Technology — particularly attribution list generation and total cost of care measurement.** A hospital that cannot accurately attribute its patient population cannot manage a global budget. Vermont's VHCURES all-payer claims database is the foundational data infrastructure, but many hospitals lack the internal analytics capability to use it effectively for population health management. This gap must be closed before FY2027.
- **Care Delivery Capability — particularly community health**

worker deployment and post-acute care network relationships. The Maryland experience demonstrated that global budgets without community care infrastructure simply reduce hospital volume without improving population health. Vermont hospitals must build these relationships proactively.

- **Network and Partnerships — particularly community partner SDOH referral networks. The Oliver Wyman systems map showed that housing, transportation, and behavioral health gaps drive a significant proportion of avoidable hospital utilization. Hospitals that lack functioning SDOH referral pathways will face cost pressure under global budgets that they cannot address through clinical means alone.**

Key Concepts Introduced or Expanded in This Chapter

Sources: Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024); Act 68 of 2025 (Vermont); Green Mountain Care Board, Act 68 Update on Hospital Reference-Based Pricing and Global Budgets (February 17, 2026); Vermont AHEAD State Agreement (January 17, 2025); CMS, AHEAD Model overview; Maryland HSCRC, Global Budget Revenue methodology and annual reports; Commonwealth Fund, Hospital Global Budgeting: Lessons from Maryland and Selected Nations (June 2024); Milbank Memorial Fund, Uniquely Similar: New Results from Maryland's All-Payer Model (2020); Vermont Agency of Human Services, Rural Health Transformation Program Application (November 2025).

Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling, APM Readiness, and the ROI of Transformation

How Healthcare Organizations Model the Financial Impact of Value-Based Care: APM Contract Analysis, Shared Savings Mechanics, Risk Stratification ROI, and the Financial Case for Transformation Investment

From Payment Architecture to Financial Practice

Chapter 6 established the payment architecture — reference-based pricing, global budgets, the AHEAD Model, Maryland’s proof of concept, and Vermont’s economic transformation mandate. This chapter turns from architecture to practice: how do healthcare organizations actually model the financial impact of alternative payment models, assess their readiness to enter and succeed in VBC contracts, and build the financial case for the operational investments that transformation requires?

Financial modeling for value-based care is not the same as financial modeling for fee-for-service operations. Under FFS, the financial questions

are primarily about revenue optimization: how do we maximize reimbursement for the volume we deliver? Under VBC, the financial questions are fundamentally different: what is our total cost of care for our attributed population? Where is our cost higher than the benchmark, and why? What investments in care management, primary care, or SDOH programs will reduce that cost below the benchmark, producing shared savings? These are population health economics questions, not revenue cycle questions, and they require different analytical frameworks, different data infrastructure, and different management skills.

Section 1: The APM Financial Modeling Framework

Understanding Benchmarks — The Foundation of VBC Economics

Every alternative payment model is built around a benchmark: the expected cost of care for the attributed population against which actual performance is measured. Shared savings (if actual costs are below benchmark) or shared losses (if actual costs are above benchmark) are calculated relative to this benchmark. Getting the benchmark right — or negotiating favorable benchmark methodology — is often the most important financial decision in VBC contract entry.

Benchmarks can be constructed in several ways, each with different financial implications for the entering organization:

Benchmark Construction Methods — What Healthcare Leaders Must Understand

Historical spending baseline: The most common approach — the benchmark is set at the organization's own historical cost of care, typically with a trend adjustment. Advantage: organizations with above-average historical spending start with a higher benchmark, making savings easier to achieve. Disadvantage: organizations with below-average historical spending (already efficient) face a lower benchmark with less room for savings — the 'efficient provider disadvantage.'

- Vermont AHEAD context: CMMI sets global budget baselines for hospitals based on their historical Medicare FFS spending, with adjustments. Vermont's hospitals with historically lower spending may face benchmarks that limit their financial upside under AHEAD.

Regional benchmark: The benchmark is set at regional average cost of care, not the organization's own history. Advantage: eliminates the efficient provider disadvantage. Disadvantage: organizations in high-cost regions compete against a lower benchmark than their own history would produce.

- Vermont RBP context: GMCB's reference-based pricing uses Medicare rates as the external benchmark — not Vermont hospital historical prices. This is a regional/national benchmark approach that directly addresses the problem of benchmarks based on historical high prices.

Prospective benchmark: The benchmark is set at the beginning of the performance year based on projected population needs. Used in global budgets and capitation models. Advantage: provides revenue certainty for planning. Disadvantage: risk adjustment quality determines fairness — poor risk adjustment produces winners and losers based on case mix differences, not care management quality.

- Vermont global budget context: Vermont's Act 68 global budgets for non-CAH hospitals must specify how the prospective budget is set and adjusted for patient complexity — the risk adjustment methodology is a key design question currently under development.

The Shared Savings Calculation — A Working Model

For organizations entering ACO REACH, MSSP, or similar shared savings models, the following framework captures the core financial logic. The HTR APM Shared Savings Calculator implements this model with organization-specific data inputs.

Figure 7.1 — APM shared savings calculation framework. Source: HTR APM Shared Savings Calculator methodology; CMS ACO model documentation.

Risk Stratification — The Economic Engine of Population Health Management

Under fee-for-service, the financial value of identifying a high-risk patient is limited — you can bill more for the additional services you provide. Under value-based care, the financial value of identifying a high-risk patient before a health crisis occurs is enormous: every hospitalization prevented by a care management intervention saves the organization the full cost of that hospitalization. The math is compelling.

A care management program that prevents 50 hospitalizations in a population of 10,000 Medicare beneficiaries, at an average cost of \$15,000 per hospitalization, generates \$750,000 in avoided cost. If the organization bears downside risk (and receives upside savings) on those prevented hospitalizations, the financial return on a care management program that costs \$200,000 to operate is approximately 3.75:1 — a 275% ROI. This is the core financial logic that makes population health management economically rational under VBC and economically irrational under FFS.

Figure 7.2 — The financial case for population health management under VBC. Sources: CMS cost data; AHS November 2025 Transformation Report.

Section 2: APM Readiness Assessment — The 30-Dimension Framework

The HTR Value-Based Care Transformation Readiness Assessment evaluates organizational readiness for APM entry across 30 dimensions in six domains. The framework is designed to identify the gaps that are most likely to prevent an organization from achieving shared savings — or most likely to produce shared losses — before a contract is signed.

The Six Readiness Domains

Domain 1: Strategic Clarity assesses whether the organization has a clear rationale for APM participation, an appropriate governance structure, and leadership alignment on the risk appetite and timeline for VBC transition. Organizations that enter APMs without genuine executive commitment — treating VBC as a compliance exercise rather than a strategic priority — consistently underperform.

Domain 2: Data and Technology evaluates the analytics infrastructure needed to manage a VBC population: attribution list generation, total cost of care measurement, care gap identification, risk stratification, and quality reporting automation. This domain is Vermont's most significant organizational gap — most Vermont community hospitals lack the internal analytics capability to use VHCURES data effectively for population health management. The joint AHS-GMCB analytics vendor procurement addresses the statewide gap; individual hospital analytics capacity must be built alongside it.

Domain 3: Care Delivery Capability assesses the clinical programs needed to act on risk stratification insights: care management programs organized around risk tiers, primary care PCMH readiness, behavioral health integration, post-acute care network relationships, community health worker deployment, and patient outreach and engagement capability. Vermont's Blueprint for Health PCMHs and CHTs provide a strong care delivery foundation; the gap is in building on this foundation to serve

the more complex patients that AHEAD’s population health requirements target.

Domain 4: Network and Partnerships evaluates the relationships the organization has with the specialists, post-acute facilities, and community organizations that determine whether high-risk patients receive coordinated care or fragmented care after a hospitalization or specialist referral. Vermont’s Oliver Wyman-designed COE regionalization framework and CIN development address this domain systematically.

Domain 5: Revenue Cycle examines the billing and coding practices that determine whether the organization’s VBC contracts are financially managed correctly: HCC coding completeness (essential for risk adjustment accuracy), RAF gap identification, attribution denial management, and VBC contract payment reconciliation. This domain is covered in depth in Chapter 9.

Domain 6: Workforce Operations evaluates the staffing model, credentialing efficiency, retention rates, and training capability that determine whether the care coordination programs identified in Domain 3 can actually be implemented and sustained.

VBC Readiness Scoring — What the Numbers Mean

Each of the 30 dimensions is scored 1-4: Pre-transition (1), Developing (2), Operational (3), Optimized (4). A total score below 60/120 indicates the organization is not ready for full-risk VBC; the gap analysis identifies the highest-priority investments before contract entry.

Vermont-specific benchmarks: Blueprint-participating PCMH practices with active CHT support typically score 8-12 in Domain 3 (care delivery) — strong by national standards. Most Vermont hospitals score 3-7 in Domain 2 (data and technology) — a critical gap given that data infrastructure is the prerequisite for population health management. The RHRC engagement’s explicit focus on developing analytic dashboards for hospitals reflects the recognition that Domain 2 is the binding constraint for most Vermont providers.

Section 3: Contract Analysis — Reading an APM Contract for Financial Risk

APM contracts are complex, and the financial risk embedded in them is often not apparent from the headline terms. A shared savings contract that appears attractive at the summary level may contain provisions — benchmark methodology choices, quality withhold structures, attribution methodology, carve-out provisions — that substantially change the financial calculus. Every organization entering a significant APM contract should apply the following analysis framework.

The 65-Item VBC Contract Review Checklist: Key Categories

The HTR VBC Contract Review Checklist (available in the Implementation Toolkit) covers 65 specific contract provisions in eight categories. The most financially consequential categories are:

Section 4: Hospital Financial Modeling Under Global Budgets — The Vermont Framework

For Vermont hospitals, the most operationally relevant financial modeling challenge is not APM shared savings calculation — it is global budget management. As Act 68's mandatory global budgets take effect for non-CAH hospitals beginning FY2028, hospital CFOs need to transition from FFS revenue optimization to global budget management. These are fundamentally different financial disciplines.

The Global Budget Financial Management Model

Under a global budget, a Vermont hospital's annual revenue is set prospectively. The financial management objective shifts from maximizing volume and revenue to achieving the highest quality population health outcomes within the fixed revenue envelope. The key financial metrics change accordingly:

Figure 7.3 — Financial metrics transformation: FFS versus global budget environment. Source: HTR Economics pillar framework.

The Hospital Financial Stress Test Model

The HTR Hospital Financial Stress Test Model — available in the Research Lab for Professional and Advisory subscribers — allows hospital CFOs and finance teams to model their organization's financial position under different payment reform scenarios. The model's key scenarios for Vermont hospitals in 2026:

- RBP scenario: What does our financial position look like if commercial prices are set at 200% of Medicare (Oliver Wyman's recommended benchmark) or 250% (GMCB's phased approach)? What service lines have prices furthest above the benchmark and face the most revenue reduction?
- Global budget scenario: Under a prospective revenue cap equal to our current net patient revenue minus X%, what is our operating margin? What cost reductions or service redesigns are required to maintain solvency?
- Medicaid cut scenario: With H.R. 1's Medicaid revenue reductions phasing in from 2027-2031, what is our projected total revenue under conservative, base, and optimistic assumptions about our Medicaid payer mix?

- Transformation investment scenario: If we invest \$X million in care management, CHW deployment, and telehealth, what reduction in avoidable hospitalizations and ED visits is required to achieve positive ROI within our VBC contract or global budget framework?

Section 5: The ROI of Transformation — Building the Financial Case

Every transformation investment — a care management program, a behavioral health integration initiative, an SDOH screening program, a telehealth platform — requires a financial justification. Under fee-for-service, this justification is often difficult to construct because the investment may reduce utilization (and revenue) even when it improves outcomes. Under value-based care and global budgets, the financial case for transformation investment is considerably stronger.

The Transformation ROI Framework

The following framework structures the financial case for transformation investments in a VBC environment. It is the analytical backbone of HTR Advisory engagements that involve transformation investment decisions.

Figure 7.4 — Transformation investment ROI framework with Vermont examples. Sources: HTR Economics pillar framework; Blueprint for Health ROI study; CMS cost data; Vermont data from AHS Transformation Reports.

Section 6: Making Care Primary — Vermont's Primary Care Economics

The AHEAD Model's requirement that states demonstrate increased investment in primary care as a percentage of total spending is not just a quality metric — it is an economics imperative. The foundational economic argument for primary care investment in a global budget environment is that primary care is the highest-return investment a healthcare system can make: the same \$1 invested in primary care produces greater total cost of care reduction than \$1 invested in hospital care or specialty care.

Vermont's Blueprint for Health embeds this economic logic in its design. The all-payer PMPM payments to PCMH practices are structured as an investment in reduced downstream costs — and Vermont's own evaluation data confirms the 5.8:1 ROI. Under AHEAD, primary care investment will be measured as a percentage of total spending, creating an explicit financial accountability framework for the primary care investment argument.

For Vermont hospitals operating under global budgets, the financial relationship between primary care investment and hospital cost management is direct: a better-functioning primary care system means fewer hospital admissions from preventable chronic disease exacerbations, fewer avoidable ED visits, and fewer hospitalizations for conditions that should have been managed in outpatient settings. The hospital that invests in primary care strengthens the community health infrastructure that reduces its own inpatient costs — a virtuous cycle that fee-for-service can never produce but global budgets require.

Key Concepts

Sources: CMS ACO model documentation; HTR Economics pillar framework; Vermont Blueprint for Health ROI study (Population Health Management);

GMCB hospital budget data; Oliver Wyman Act 167 Report; AHS Transformation Reports; Vermont AHEAD State Agreement; Act 68 of 2025.

Chapter 8: The Operations Pillar — Executing Hospital System Transformation

From Plans to Action: The 14-Hospital Process, RHRC Methodology, Workforce Architecture, and the Mechanics of Regionalization

Sources: Oliver Wyman Act 167 Community Engagement: Recommendations (2024); AHS Health Care System Transformation Reports (August 2025, November 2025); AHS January 2025 Legislative Presentation; Act 68 of 2025; Vermont RHT Program Application (November 2025); GMCB 2024 Annual Report; Vermont Blueprint for Health data.

The Operations Pillar: Where Strategy Meets Reality

Every chapter in this book that precedes this one is, in some sense, about ideas: the analytical framework for understanding Vermont’s crisis, the financial architecture of global budgets and reference-based pricing, the clinical models that should replace hospital-centric care. This chapter is about execution. The Operations pillar is where the gap between a well-designed plan and an implemented one is either closed or exposed. It is

where organizational inertia, workforce constraints, data gaps, political friction, and the sheer complexity of coordinating 14 independent hospital organizations across a geographically vast rural state determine whether transformation actually happens.

Vermont's operations challenge is unusual in its specificity and urgency. AHS has statutory deadlines — hospital transformation plans by early 2026, RBP by FY2027, global budgets by FY2028, the Statewide Strategic Plan by December 2028. It has a constrained budget (\$4.2M in appropriated state funds, plus \$195M in RHT Program capital and \$1.2M in AHEAD cooperative agreement funds). It has a newly contracted operational partner in the Rural Health Redesign Center. And it has 14 hospitals in wildly different financial and operational conditions, ranging from a large academic medical center at the center of the state's economy to 19-bed critical access hospitals in the Northeast Kingdom where any service reduction has immediate community consequences.

This chapter develops the operational framework for executing system transformation at the scale Vermont requires. It draws on the specific methodologies AHS and RHRC have deployed, the transformation planning process and its current state, and the lessons from other state and health system transformation efforts about what the operations discipline requires. It is written both as an academic treatment of the operations pillar and as a practical guide for the AHS staff, hospital leaders, and legislative overseers who are executing Vermont's transformation in real time.

Section 1: The Vermont Transformation Operating Model

Vermont's health system transformation is not being executed by a single authority acting on a blank canvas. It is being coordinated by AHS across a network of 14 independent (all non-profit) hospitals, 12 local health district offices, the GMCB as regulatory partner, the Department of Vermont Health Access (DVHA) as Medicaid authority, dozens of community-based organizations, primary care practices, designated mental health

agencies, and a state legislature that requires monthly reporting on progress. Understanding the governance and operational model for managing this network is prerequisite to understanding how transformation actually works.

The Institutional Architecture

Act 68 established a four-actor governance architecture for Vermont's transformation that is worth understanding precisely:

Figure 8.1 — Vermont health system transformation governance architecture under Act 68. Sources: Act 68 of 2025; AHS Legislative Presentation (January 2025).

The governance architecture has one significant design tension worth naming: AHS is simultaneously the transformation planner, the technical assistance provider, the grant-maker, and the accountability monitor for the hospitals it is also trying to help. This is not an unusual arrangement in state healthcare reform — Maryland's HSCRC has regulatory authority and collaborative relationships simultaneously — but it requires explicit management. The AHS model works best when hospitals see the agency as a genuine partner in solving difficult problems, not primarily as a regulator imposing mandates. Oliver Wyman's recommendation to add a Division of Planning and Effectiveness and to clarify the AHS-GMCB role division is partly a response to this tension.

The Transformation Planning Sequence

AHS designed a phased transformation planning process that moves hospitals from initial engagement through draft strategy to final implementation plans. The sequence was developed with RHRC and reflects the operational reality that 14 hospitals in very different financial and operational positions cannot move at the same pace or toward the same model.

Figure 8.2 — Vermont hospital transformation planning sequence. Sources: AHS Health Care Transformation website; AHS November 2025 Act 68 Report; AHS January 2025 Legislative Presentation.

“Transformation work is ongoing and evolving. Based on hospital feedback and changes in state and federal policy, AHS is continuing to adapt our approach to ensure planning is coordinated, responsive, and effective.”

— Vermont Agency of Human Services, Health Care Transformation website, 2026

Section 2: The RHRC Methodology — Technical Assistance for Rural Hospital Transformation

The Rural Health Redesign Center (RHRC) was AHS’s primary operational contractor for hospital transformation planning from spring 2025 through October 2025. Understanding the RHRC methodology is important not just as Vermont history but as a transferable model for how state agencies can provide technical assistance to financially distressed rural hospital networks — a challenge that will confront state health agencies across the country as the demographic and financial pressures Vermont faces today spread nationally.

What RHRC Did

RHRC’s engagement combined three functions that are usually performed separately: financial and operational assessment, change management support, and transformation plan development. The combination was deliberate — Oliver Wyman’s experience had shown that hospitals can absorb financial analysis without changing behavior, can engage in planning processes that produce documents rather than action, and can participate in change management programs without addressing their fundamental financial unsustainability. RHRC’s model integrated all three.

RHRC's Four Workstreams at Each Hospital

RHRC worked with each of Vermont's 14 hospitals through four parallel workstreams, adapted to each hospital's specific position and needs:

- Financial and operational baseline: Review of hospital-specific cost data from GMCB financial records, CMS cost reports, and the NASHP Hospital Cost Tool. Identification of the hospital's position relative to Vermont average and national benchmark on key efficiency metrics (operating profit per adjusted discharge, management and administrative cost per adjusted discharge, direct patient care labor cost per adjusted discharge).
- Priority identification: Working with hospital leadership to identify the highest-leverage short-term cost reduction opportunities — supply chain, staffing models, service line analysis — and medium-term transformation opportunities — shared services, service line reconfiguration, COE alignment.
- Transformation plan development: Supporting each hospital in developing a structured transformation plan that maps specific actions to timelines, costs, and Act 167 outcome objectives. Plans address hospital sustainability and cost through analysis of service lines, staffing models, and shared services opportunities.
- Regional coordination: Connecting hospital-level planning to the statewide regional work — ensuring individual hospital plans are aware of and compatible with what neighboring hospitals are planning, and identifying shared service opportunities that require cooperation across organizational boundaries.

The RHRC contract concluded in October 2025 — a planned conclusion, not a termination. AHS designed the engagement as a time-limited capacity-building investment, not an ongoing dependency. The explicit goal was for hospitals to develop the internal planning capability and the collaborative

relationships needed to continue transformation work independently, with AHS providing ongoing oversight, grant funding, and analytical support rather than direct technical assistance.

This design philosophy reflects a lesson from failed state transformation initiatives: external consultant-driven processes that do not build internal organizational capacity produce plans that expire when the consultant leaves. Vermont’s approach — using RHRC to build the planning process and the inter-hospital relationships, then transitioning to AHS-led coordination with hospital-level implementation — is more likely to produce durable change.

The Analytics Gap and Its Resolution

The November 2025 AHS transformation report identified one critical operational gap in the current system: the absence of an analytics platform capable of evaluating hospital transformation proposals quantitatively, modeling the cost and quality implications of different service configurations, and assessing alignment with statewide goals. This gap means AHS currently cannot answer with confidence the questions that the transformation planning process is supposed to produce answers to: if Springfield Hospital converts to a Rural Emergency Hospital, what is the net financial impact on the system? If orthopedic surgery is concentrated at Southwestern Vermont and Copley, what travel burden does that create for patients in the affected communities?

AHS responded to this gap by procuring an analytics vendor in partnership with GMCB — a joint procurement that reflects the appropriate division of labor: GMCB brings regulatory data and rate-setting expertise; AHS brings population health and SDOH context. The analytics platform, once operational, will enable the state to evaluate hospital proposals in terms of quality, cost, and access; assess alignment with regional and statewide goals; and quantify potential cost savings using evidence-based methods. This platform is also the technological foundation for monitoring the Statewide Strategic Plan’s implementation metrics after December 2028.

Section 3: The Vermont Hospital Performance Dashboard — Where Each Hospital Stands

AHS's November 2025 transformation report established Vermont's official performance measurement framework — the set of outcome measures against which progress toward Act 167's five goals will be tracked. The framework covers three domains: provider efficiency and sustainability, payer affordability, and patient health outcomes. The following table presents the current performance landscape, with Vermont's actual values compared to benchmarks set at the 75th percentile of national peers.

Figure 8.3 — Vermont health system transformation outcome measures: current performance vs. benchmarks. Source: AHS Health Care System Transformation Report, November 2025.

Several findings from this dashboard deserve specific attention from healthcare leaders and policymakers:

The CAH operating profit gap is the most urgent financial signal. Critical Access Hospitals are running at \$938 operating profit per adjusted discharge, against a benchmark of \$2,784 — a \$1,846 per-discharge gap. This means Vermont's smallest, most rural hospitals are the furthest from financial sustainability. The RBP and global budget reforms will affect these hospitals differently from PPS hospitals, and Act 68 explicitly requires GMCB to consider CAH-specific adjustments. The RHT Program's explicit inclusion of CAH stabilization as a priority use of funds reflects this gap.

The administrative cost premium is massive. PPS hospitals are spending \$2,730 per adjusted discharge on management and administrative costs, against a benchmark of \$1,427 — a 91% premium over what efficient peer hospitals spend. This \$1,303 per-discharge gap is exactly the administrative cost reduction that Oliver Wyman identified as one of the three primary sources of direct savings from transformation. If Vermont's PPS hospitals can reach the benchmark administrative cost level, the system savings are substantial — and Oliver Wyman estimated administrative reduction as

contributing over \$100 million of the \$400M+ transformation savings target.

The behavioral health gap is measurable and alarming. With 245.5 ED visits per 10,000 for suicide ideation and self-harm — and rates statistically higher in Rutland and Windham counties, and among Vermonters aged 15-44 — the crisis presentation data confirms what Oliver Wyman’s community meetings documented: Vermont’s behavioral health access problem is driving a significant share of avoidable hospital utilization. This is not a clinical quality problem; it is an access problem. The CCBHC expansion (5 new entities by July 2026) and the behavioral health investments funded through AHEAD’s EAST Fund are the operational response.

Management and administrative cost per adjusted discharge: Vermont PPS hospitals vs. benchmark

Figure 8.4 — Administrative cost gap: Vermont PPS hospitals vs. national benchmark. This \$1,303 per-discharge gap represents the primary operational inefficiency target for Vermont’s transformation. Source: AHS Transformation Report, November 2025 (NASHP via CMS).

Section 4: Regionalization — The Operational Logic of Right-Sizing a 14-Hospital System

Regionalization is the operational translation of Oliver Wyman's third imperative: move all care possible out of hospitals, and concentrate what must stay in hospitals at sites with sufficient volume to maintain expertise. The concept is straightforward; the execution is complex. This section develops the operational methodology of regionalization — how you actually move from a system of 14 hospitals each attempting full-spectrum care to a differentiated regional network where each facility has a clearly defined role, adequate patient volume, and sustainable financial structure.

The Regional Network Design Framework

Oliver Wyman's regionalization blueprint organized Vermont's hospital network around three facility types, each with a distinct role in the transformed system:

RSCs are the hospitals with sufficient population base, financial position,

and existing expertise to sustain inpatient beds and serve as Centers of Excellence in multiple specialty areas. Under Oliver Wyman's blueprint, UVMMC in Burlington is the state's primary RSC with 16 COE designations. Southwestern Vermont Medical Center, Brattleboro Memorial Hospital, Rutland Regional Medical Center, and Central Vermont Medical Center function as secondary RSCs, each with 5-9 COE designations serving their regional populations.

The RSC model requires a fundamental change in how these hospitals think about their role: from serving their immediate geographic catchment area to accepting referrals from the entire region for their designated specialties. This means building the transfer infrastructure, the outreach relationships with smaller hospitals, and the care management capacity to serve a larger and more complex patient population.

Community hospitals with focused scope are inpatient facilities that maintain specific service lines where they have adequate volume and expertise — their designated COE specialties — while transferring other complex cases to RSCs. Northwestern Medical Center (St. Albans), Northeastern Vermont Regional Hospital (St. Johnsbury), Springfield Hospital, and Copley Hospital (Morrisville) fall into this category, with 2-4 COE designations each.

The operational challenge for this tier is managing the transition: reducing reliance on service lines being regionalized elsewhere while building capacity in designated specialties and managing the patient volume shift without destabilizing the hospital's financial position. This requires precisely the kind of short-term, medium-term, and long-term planning that RHRC's transformation plan methodology was designed to support.

Tier 3 facilities are hospitals that cannot sustain inpatient beds in the long term — either because their population base is too small, their financial position is too distressed, or both. Rather than closing unexpectedly

and leaving communities without any healthcare access, Oliver Wyman's blueprint proposes orderly conversion to one of three alternative models: Rural Emergency Hospital (REH — 24/7 emergency and observation without inpatient beds), Community Ambulatory Care Center (CACC — outpatient and primary care services), or a Care at Home support hub. Grace Cottage, Gifford Medical Center, North Country Hospital, and Porter Medical Center are among the facilities whose futures require further discussion as part of Vermont's regionalization planning.

The REH designation is particularly significant for Vermont because it preserves the emergency access function that communities depend on while eliminating the expensive inpatient infrastructure that these hospitals cannot financially sustain. A community with an REH and reliable EMS transport to an RSC 45 minutes away is better served than one with a financially distressed inpatient hospital that cannot staff its surgical suite or maintain specialist coverage.

Figure 8.5 — Vermont 14-hospital regionalization framework: COE designations, tiers, and key operational considerations. Sources: Oliver Wyman Act 167 Report (2024); AHS Transformation Reports (2025).

The Operational Mechanics of Regionalization

Regionalization is not a policy decision that happens once — it is an operational process that must be managed over years. The transition from a hospital system where every facility attempts full-spectrum care to one organized around regional specialization requires changes across five operational domains:

Transfer infrastructure is the most visible immediate need. Oliver Wyman documented that Vermont currently has 31 separate EMS agencies that a provider may need to contact to arrange a patient transfer — a fragmentation that is simultaneously a patient safety risk and an operational cost. Vermont's RHT Program application includes EMS transformation as a priority

investment: professionalization and regionalization of EMS, improved inter-facility transfer coordination, and expanded telehealth to reduce unnecessary transfers. The November 2025 AHS report identified inter-facility transfer coordination as one of the primary statewide regionalization themes emerging from regional hospital convenings.

Clinical network agreements formalize the referral relationships between RSCs and Tier 2/3 facilities. Oliver Wyman recommended that RSCs establish formal agreements to receive referrals from emergency departments at smaller hospitals for their designated COE specialties — ensuring that when a patient arrives at Springfield Hospital needing complex general surgery, the transfer pathway to Brattleboro Memorial Hospital (the acute general surgery COE for that region) is established, protocolized, and reliable. These agreements do not currently exist in a systematic form across Vermont’s hospital network; the Act 68 transformation planning process is the vehicle for creating them.

Administrative shared services offer some of the largest near-term cost savings in the transformation agenda. Oliver Wyman identified three shared service opportunity areas as high-priority: supply chain (group purchasing), administrative functions (billing, coding, human resources, credentialing), and IT infrastructure. Vermont hospitals have begun early-stage discussions on group purchasing during the regional convenings. The administrative cost gap — \$1,303 per discharge above benchmark — is where shared services can make the most immediate impact on the \$400M+ savings target.

Workforce redistribution is the most sensitive operational challenge. When a hospital transitions from full inpatient to REH or CACC status, the clinical staff whose roles were tied to inpatient services must either transition to new roles, relocate to RSCs, or leave the healthcare workforce. Oliver Wyman explicitly recommended that healthcare workforce affected by system changes could be redistributed or retrained to perform services needed by the community — recognizing that the workforce challenge is not just about attracting new providers to Vermont but about optimally deploying those already there. Vermont’s RHT Program workforce investments —

tuition assistance, recruitment incentives with 5-year service obligations, scope-of-practice expansion for nurses and APRNs — are the supply-side response; workforce redistribution planning is the demand-side response.

Community engagement is the political prerequisite for all operational change. Oliver Wyman's process demonstrated that communities are willing to accept difficult changes — service reductions, facility reconfiguration, changes in travel distances — when they have been genuinely engaged in the decision-making process and when they understand the alternative (unexpected closures) is worse. AHS has committed to continuing community engagement as transformation plans develop, expanding beyond hospital leadership to include community providers and other health and human services stakeholders as priorities are identified.

Section 5: The Workforce Crisis — Vermont's Most Binding Operational Constraint

Of all the operational challenges Vermont faces, workforce is the one most likely to constrain the pace and ambition of transformation. Vermont's healthcare workforce problem is not a single issue — it is a cluster of interconnected problems that Oliver Wyman's systems map showed cascading across the entire healthcare system. Understanding the workforce problem in its full complexity is prerequisite to designing interventions that actually work.

The Dimensions of Vermont's Workforce Crisis

Figure 8.6 — Vermont workforce crisis key metrics. Sources: Vermont RHT Program Application (November 2025); Oliver Wyman Act 167 Report; Vermont Department of Labor.

Vermont's workforce problem has five distinct but interconnected dimen-

sions, each requiring a different intervention approach:

Primary care shortage is both a supply problem and a productivity problem. Oliver Wyman made a counterintuitive finding that Vermont's primary care shortage is partly a productivity issue, not purely a headcount problem: HRSA recognizes no Health Profession Shortage Areas in Vermont, and if primary care providers were supported to see three patients per hour, the state would have adequate supply. The implication is that scope-of-practice expansion for APRNs and physician assistants, team-based care models that use clinical staff at the top of their licensure, and administrative burden reduction (prior authorization reform, documentation support) can meaningfully increase primary care capacity without adding headcount.

Specialist shortage drives regionalization urgency. Vermont cannot maintain specialist coverage at 14 hospitals across a state of 647,000 people. The population density and economics do not support it. The Oliver Wyman COE framework is, in part, a response to the specialist shortage: rather than trying to recruit and retain a cardiologist or neurosurgeon for every Vermont community, concentrate those specialists at facilities with sufficient volume to justify their positions and deliver the clinical excellence their presence enables.

Nursing shortage is the most acute staffing crisis at individual hospitals. All Vermont hospitals cited physician shortages and difficulty recruiting staff since COVID-19 as primary operational challenges in Oliver Wyman's assessment. Travel nursing — the expensive emergency staffing solution that many hospitals have relied on since 2020 — has significantly worsened hospital financial positions. Travel nurse costs are typically two to three times the cost of a permanent hire. Vermont hospitals that remain heavily dependent on travel nursing are in a financial hole that deepens with every shift filled by a travel agency.

Housing is the hidden workforce constraint. Oliver Wyman's systems map traced the connection from housing shortage to workforce shortage to healthcare system fragility explicitly. Vermont's rental vacancy rate of 3% — well below the 5% rate of a healthy market — means healthcare workers recruited to Vermont communities cannot find housing. Half of

Vermont renters are cost-burdened. This is not a healthcare problem that healthcare reform can solve; it requires housing policy responses. But AHS's transformation mandate, properly understood, extends to advocating for and enabling the housing investments that make healthcare workforce recruitment possible. This is one of the reasons Oliver Wyman's first imperative — build housing and fix transportation — comes before payment reform in the three-imperative sequence.

Scope-of-practice restrictions limit Vermont's ability to use the workforce it has effectively. Vermont has taken some steps toward scope-of-practice expansion — joining the Social Work Licensure Compact, allowing pharmacist-extended prescriptions, approving mental health professionals without master's degrees for certain psychotherapy roles — but Oliver Wyman's action list called for further expansion of professional licensure and practice scope for nurses, EMTs, and pharmacists. The RHT Program's workforce investments include scope-of-practice-enabling training, and Act 68's workforce goal — attracting and retaining high-quality health care professionals — implicitly requires the scope-of-practice environment to make Vermont competitive with surrounding states.

Vermont's Workforce Strategy: The RHT Investment Framework

Vermont's Rural Health Transformation Program application dedicated a substantial portion of its \$195M award to workforce infrastructure — treating workforce as capital investment rather than operating expense. The distinction matters: using one-time capital funds to build durable workforce pipelines (tuition assistance that creates committed local providers, training infrastructure that enables scope expansion, housing investments that make recruitment viable) is fundamentally different from using them to fund ongoing salaries.

RHT Workforce Investment Priorities

Vermont's RHT Program application included five workforce investment areas, each designed to produce durable capacity rather than temporary staffing:

- Tuition assistance and loan forgiveness for nurses, physicians, APRNs, LNAs, home health aides, and other providers, with 5-year service obligations in Vermont — building a locally-rooted workforce pipeline
- On-site clinical training infrastructure — supporting teaching programs that keep medical students and residents in Vermont communities during training, increasing the probability they establish practices there
- Scope-of-practice training and support for APRNs, physician assistants, pharmacists, and EMTs — enabling Vermont to use its existing workforce at maximum capacity
- Community health worker training and deployment — building the SDOH navigation and care coordination workforce that population health management requires, drawing from the communities being served
- Interstate Medical Licensure Compact (IMLC) participation — enabling out-of-state providers to practice via telehealth in Vermont, reducing specialist access gaps without requiring physical relocation

Section 6: The Operations Metrics Framework — What AHS Must Track

Effective operations management requires clear, measurable indicators of progress. AHS's current transformation outcome framework (Figure 8.3) establishes a baseline measurement architecture, but it has acknowledged limitations: most data reflects 2023 or earlier performance, the impact of 2025 transformation activities will not appear in measures until 2026 or later, and some critical domains — mental health, SDOH, post-acute care — lack the consistent data infrastructure to support real-time monitoring.

For the Statewide Strategic Plan that AHS must deliver by December 2028, a more complete operations metrics framework is required. Drawing on Oliver Wyman's five Act 167 goals and Act 68's accountability metrics

requirements, the following framework organizes the key operational indicators by domain:

Figure 8.7 — Vermont health system transformation: comprehensive operations metrics framework. Sources: Act 68 of 2025; AHS Transformation Report (November 2025); Oliver Wyman Act 167 Report.

One structural observation about this metrics framework: many of the most important indicators will not show measurable improvement for 18-24 months after the interventions that should drive them are implemented. Healthcare system change is slow, and the data infrastructure to measure it is even slower. AHS's acknowledgment in its November 2025 report that most available measures reflect 2023 or earlier performance is not an evasion — it is an accurate description of the measurement reality. Policymakers and oversight bodies need to understand this lag and resist the temptation to declare failure when early data does not yet show improvement.

Section 7: AHS as System Operator — Organizational Design for the 2028 Mandate

The operations chapter concludes with the organizational design question that underpins everything else: is AHS currently structured to execute the transformation it has been mandated to lead? The honest answer, reflected in Oliver Wyman's findings and AHS's own public reports, is: not yet, but the organization is restructuring.

The Current Organizational Gaps

Oliver Wyman identified four specific organizational gaps in AHS that impede transformation execution. These are not criticisms of individuals — they reflect an organizational structure designed for a different era that has not yet adapted to the system operator role Act 68 requires.

- **Misalignment between AHS sub-unit organization and Hospital Service Areas.** Vermont's AHS is organized around program types (mental health, substance use, aging services, Medicaid, etc.) rather than geographic service areas. This means that when a hospital in the Northeast Kingdom is trying to address the housing, transportation, behavioral health, and primary care gaps that drive its ED volume, it must work with multiple separate AHS programs — each with its own administrative requirements, reporting cycles, and performance metrics. Oliver Wyman's recommendation to align AHS sub-units with Hospital Service Areas and meet regularly with hospitals and providers in each HSA is a structural reorganization, not just a coordination improvement.
- **Absence of a Planning and Effectiveness function.** AHS currently lacks a dedicated analytical unit that can calculate the financial and access impacts of service site changes, monitor access and affordability across community providers, and track the quality, access, and equity metrics that the Statewide Strategic Plan will require. The analytics vendor procurement in partnership with GMCB addresses part of this gap, but the internal capacity to interpret, apply, and act on analytical outputs is a separate organizational requirement.
- **Under-resourcing for the transformation management function.** Act 68 requires AHS to simultaneously negotiate with CMS on AHEAD implementation, manage 14 hospital transformation planning processes, develop the Statewide Strategic Plan, oversee RHT Program implementation, coordinate with GMCB on RBP and global budget design, and report monthly to the legislature. The Health Care Reform Office, recently reorganized and supplemented by contractor support, is managing a workload that probably requires 50-70% more staff than currently exist. The RHT Program award explicitly includes funds for hiring staff to oversee its implementation.

- **Limited SDOH integration across AHS programs.** The Oliver Wyman analysis demonstrated that housing, transportation, food security, and behavioral health — all areas where AHS has programmatic authority — are primary drivers of healthcare utilization. But AHS's current organizational structure treats these as separate program areas rather than as integrated components of a population health strategy. The Statewide Strategic Plan mandate creates the opportunity to redesign this integration explicitly.

The Emerging Solution: A Population Health Management Operating Model

The organizational model that AHS's Strategic Plan should articulate — and that the AHS restructuring chapter later in this book develops in full — is a population health management operating model organized around Vermont's Hospital Service Areas, with clear roles for each level of the system.

At the statewide level, AHS functions as the system architect: setting the strategic direction, establishing the payment reform framework (in partnership with GMCB), allocating capital (RHT Program, AHEAD EAST Fund, Act 68 grants), and monitoring system performance against the five Act 167 goals.

At the HSA level, regional care networks — organized around the Tier 1 RSC for each region — function as the operational unit: coordinating transfer pathways, managing shared service arrangements, aligning primary care and behavioral health investment with hospital transformation plans, and connecting healthcare planning with housing, transportation, and SDOH infrastructure planning.

At the provider level, individual hospitals, primary care practices, designated agencies, FQHCs, and community-based organizations function as service delivery nodes: executing transformation plans, participating in regional coordination, and reporting performance against standardized metrics that allow AHS to monitor progress and identify where additional

support is needed.

This three-level operating model is not new to health system design — it is the architecture of every effective regional health system in the country. What is new in Vermont is the explicit statutory mandate (Act 68's Strategic Plan requirement), the financial resources to build the infrastructure (RHT Program, AHEAD EAST Fund), and the political will — demonstrated through the extraordinary legislative sequence of Acts 167, 51, 55, and 68 — to execute the structural change rather than continuing to manage the decline.

Key Concepts Introduced in This Chapter

Sources: Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024); Vermont Agency of Human Services, Health Care System Transformation Report (November 1, 2025); Vermont Agency of Human Services, Health Care System Transformation Report (August 2025); Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts, House Health Care Committee (January 31, 2025); Act 68 of 2025 (Vermont); Vermont Rural Health Transformation Program Application (November 3, 2025); Green Mountain Care Board, 2024 Annual Report (January 15, 2025); Vermont Agency of Human Services, Health Care Transformation website (healthcarereform.vermont.gov), accessed April 2026.

Chapter 9: The Operations Pillar in Practice — Revenue Cycle, HCC Coding, Credentialing, and Workforce in a Value-Based Environment

The Operational Infrastructure That Makes or Breaks VBC Contracts: Clinical Documentation Excellence, Risk Adjustment Strategy, Credentialing Efficiency, and the Workforce Operations Framework for Transformed Health Systems

Operations Where Money Actually Lives

Healthcare transformation is ultimately executed through the operational infrastructure that most health system leaders think of as administrative — revenue cycle management, clinical documentation, credentialing, workforce management. These functions are not adjacent to clinical transformation; they are the substrate on which it runs. An organization that redesigns its care model for value-based care but fails to adapt its revenue cycle, clinical documentation, and workforce operations will achieve neither the quality outcomes nor the financial results the new model promises.

This chapter develops the operational mechanics of healthcare transformation — the specific processes, metrics, and management disciplines that determine whether value-based care contracts produce the results they are designed for. It is organized around four operational domains: revenue cycle transformation for VBC, HCC coding and risk adjustment excellence, credentialing efficiency, and the workforce operations framework.

Section 1: Revenue Cycle Transformation — From FFS Optimization to VBC Management

Revenue cycle management under fee-for-service is a known discipline: charge capture, coding accuracy, claim submission, denial management, underpayment identification. Hospitals and physician practices have invested heavily in FFS RCM over the past two decades, and the industry has developed sophisticated tools and processes for maximizing FFS revenue. None of this investment is wasted under VBC — but all of it must be extended.

VBC revenue cycle adds three functions that FFS RCM does not require: attribution management (ensuring that patients are correctly attributed to the organization for shared savings calculation), total cost of care monitoring (tracking what attributed patients cost across all providers and settings, not just at your organization), and VBC contract reconciliation (understanding and disputing the financial calculations that determine shared savings or loss settlements).

Attribution Management — The VBC-Specific Revenue Function

Under FFS, revenue is determined by what care the organization delivers. Under VBC, revenue is determined partly by which patients are attributed to the organization — and attribution errors can silently reduce shared savings or inflate shared losses by millions of dollars. Every organization in a significant VBC arrangement needs an attribution management function.

Figure 9.1 — Attribution error taxonomy and remediation. Source: HTR Operations pillar framework; ACO REACH and MSSP operational guidance.

Total Cost of Care Monitoring — Seeing What You Cannot Bill

The most difficult revenue cycle adaptation for FFS-trained teams is monitoring costs they do not receive. Under VBC, the organization's financial performance is determined by the total cost of care for its attributed population — including care delivered by specialists, hospitals, skilled nursing facilities, home health agencies, and any other provider outside the organization. A primary care group in an ACO may be held accountable for whether their attributed patients receive unnecessary specialist imaging ordered by a cardiologist who has no financial relationship with the ACO.

Vermont's VHCURES all-payer claims database is the analytical foundation for total cost of care monitoring — providing claims data across all payers and provider types for attributed Vermont beneficiaries. The GMCB analytics vendor procurement, and individual hospital analytics investments through the RHT Program, are building the capability to use VHCURES data for real-time population health management rather than retrospective reporting.

Section 2: HCC Coding Excellence — The Risk Adjustment Foundation

Hierarchical Condition Category (HCC) coding is the mechanism through which Medicare risk adjustment translates patient diagnoses into the Risk Adjustment Factor (RAF) that scales a beneficiary's expected cost. In VBC arrangements, accurate HCC coding is not just a compliance requirement — it is the foundation of financial sustainability. An organization that systematically under-codes its patients' diagnoses will have a benchmark set too low for its population's actual cost burden, and will appear to be

underperforming even when its care management is excellent.

The HCC Model — How It Works

Medicare's HCC model assigns each patient a RAF based on their demographic characteristics (age, sex, Medicaid status, disability) and their active chronic conditions as documented in claims. The RAF multiplies the base capitation or benchmark rate — a patient with a RAF of 1.5 has a benchmark 50% higher than the average Medicare beneficiary. Accurate diagnosis coding is essential because the RAF is calculated from diagnosis codes on claims; diagnoses that are not coded do not affect the RAF.

Vermont's rural hospital population has two characteristics that make HCC coding excellence especially important: high prevalence of complex chronic conditions (diabetes, COPD, CHF, CKD) that generate large HCC score differences based on coding specificity, and significant dually eligible (Medicare-Medicaid) population that has additional risk adjustment mechanisms. Vermont hospitals that do not code to the highest level of specificity are systematically under-representing their population's complexity and receiving benchmarks that do not reflect actual cost burden.

The HCC Gap Closure Process

The HCC gap closure process — identifying diagnoses that should be coded based on clinical evidence but are not currently documented in claims — is one of the highest-ROI operational investments available to organizations in VBC arrangements. The process has four steps:

- **RAF analysis:** Compare the organization's average RAF against national and regional benchmarks for comparable populations. A RAF significantly below benchmark for a population with high chronic disease prevalence signals a coding gap.
- **Retrospective chart review:** Review clinical documentation for attributed patients to identify diagnoses that are clinically evident

(in problem lists, clinical notes, medication lists) but not documented in claims codes. Each uncoded HCC condition represents a missed RAF point.

- **Prospective coding education: Educate clinical staff on HCC coding specificity requirements — the difference between coding ‘diabetes’ (HCC 19, lower RAF) versus ‘diabetes with diabetic CKD’ (HCC 18 + HCC 136, higher RAF) when clinical evidence supports the more specific code.**
- **Annual wellness visits: AWWs are the primary vehicle for capturing annual HCC updates — CMS requires that chronic conditions be documented at least annually to be included in RAF calculations. Systematic AWW completion for the attributed population is both a quality metric and an HCC gap closure mechanism.**

Vermont in Practice: HCC Coding at Vermont Community Hospitals

Oliver Wyman’s Act 167 assessment of Vermont’s hospital system did not specifically examine HCC coding accuracy, but the management and administrative cost data tells a related story: Vermont PPS hospitals spend \$2,730 per adjusted discharge on management and administrative costs against a benchmark of \$1,427. Part of this gap reflects genuine administrative inefficiency — systems and processes that could be streamlined. But part of it also reflects underinvestment in the clinical documentation and coding infrastructure that makes revenue cycle management accurate under both FFS and VBC.

Vermont hospitals entering the AHEAD global budget model in 2027 will have their budgets set based on historical Medicare spending adjusted for their population’s risk complexity. An organization that has under-coded its population’s HCC conditions will receive a lower global budget than its actual cost burden warrants — and will need to manage within that lower budget for the full performance year before the error can be corrected. Getting HCC coding right before AHEAD launches is not optional — it is foundational financial preparation.

Recommended action: Every Vermont hospital should conduct a retro-

spective HCC gap analysis for their AHEAD-attributed Medicare population before January 2027. The analysis should compare clinical documentation to coded diagnoses, identify the largest gaps by HCC category, and develop a provider education and AWV completion program to close the gaps prospectively.

Section 3: Credentialing Efficiency — The Hidden Operations Bottleneck

Provider credentialing — the process of verifying a clinician’s education, training, licenses, and competency before granting hospital privileges or health plan participation — is one of the most operationally consequential and consistently underinvested functions in American healthcare administration. The average credentialing cycle takes 90-120 days for a new provider. In a state with a 370 FTE primary care shortage by 2030, a 90-day credentialing delay represents 90 days of foregone care — and foregone revenue — for every provider recruited.

Credentialing as a Transformation Bottleneck

Vermont’s transformation agenda accelerates the credentialing importance for two specific reasons. First, regionalization — moving to a tiered hospital network where patients follow clinical protocols to Centers of Excellence — requires providers to have privileges at multiple facilities, not just their home institution. A surgeon designated as a COE orthopedic provider must be credentialed at the regional COE facility to practice there, and the credentialing process must occur before the regionalization takes effect. Vermont hospitals that do not begin cross-facility credentialing before COE designations are finalized will face delays in implementing the regionalization blueprint.

Second, scope-of-practice expansion — enabling APRNs, PAs, and

other non-physician clinicians to practice at the top of their licensure — requires updated credentialing standards and privilege delineations at every facility. Oliver Wyman’s workforce recommendation to expand professional licensure and practice scope is not a paperwork exercise; it requires systematic credentialing policy updates at 14 hospitals, coordinated through the Vermont CIN’s shared services infrastructure.

The Credentialing Efficiency Framework

Three operational improvements reduce credentialing cycle time from 90-120 days to 45-60 days without reducing verification rigor:

- **Primary source verification automation:** Replace manual verification of medical school diplomas, residency completions, and license verifications with automated primary source connections to NPDB, state licensing boards, and medical schools. Several credentialing platforms (VerityStream, Symplr, symplr MD-Staff) provide automated PSV that reduces verification time from weeks to hours.
- **Continuous monitoring replacing point-in-time verification:** Implement ongoing license and sanction monitoring that alerts the credentialing office when a provider’s license status changes, rather than discovering changes at 2-year re-credentialing intervals. Continuous monitoring reduces risk while eliminating the administrative burden of complete re-verification at each cycle.
- **Unified credentialing for the Vermont CIN:** Vermont’s 14 hospitals sharing a common credentialing database — so that a provider credentialed at one CIN member facility has their verified credentials available to all member facilities — reduces the per-facility credentialing burden for providers practicing at multiple sites. This is a direct shared service application that Oliver Wyman’s CIN design makes possible.

Section 4: Workforce Operations — Managing the Care Team in a Transformed System

The Operations pillar's workforce domain encompasses three distinct management challenges that often get conflated: workforce supply (recruiting enough providers), workforce deployment (organizing providers in care team models that maximize effective capacity), and workforce retention (reducing attrition that compounds the supply problem). Vermont faces challenges in all three domains, and the solutions are different.

The Workforce Supply Challenge — What Can Actually Be Done

Vermont projects a 370 FTE primary care shortage by 2030. Physician recruitment pipelines take 7-10 years from medical school enrollment to practice — making it impossible to recruit Vermont out of a 2030 shortage with current policies. The supply interventions that can produce meaningful results within the relevant timeframe are:

- Scope-of-practice expansion for APRNs and PAs: Oliver Wyman specifically recommended this. Vermont APRNs operating under full practice authority (without physician supervision requirements) can see three patients per hour in PCMH settings, effectively adding primary care capacity equivalent to additional physicians without waiting for new physicians to train.
- International Medical Graduate (IMG) recruitment and J-1 visa waiver programs: Vermont has J-1 visa waiver slots through the Conrad 30 program, enabling internationally trained physicians to practice in underserved areas in exchange for 3-year service commitments. Active management of this program — filling all available slots, targeting Northeast Kingdom communities — can meaningfully supplement domestic recruitment.
- Loan repayment and service obligation programs: Vermont's RHT Program workforce investments include tuition assistance and loan

repayment with 5-year service obligations. The evidence on loan repayment is consistent: it works for early-career providers with high debt burdens. Vermont's program should be structured with the largest awards for the communities with the greatest access gaps.

- Regional medical education investment: The evidence on training location and practice location is strong — physicians trained in rural settings are more likely to practice in rural settings. Vermont's partnership with University of Vermont College of Medicine, and potential expansion of clinical training sites in Northeast Kingdom communities, is a 7-10 year investment with compound returns.

Workforce Deployment — Team-Based Care at Scale

Oliver Wyman's finding that Vermont does not have an absolute primary care shortage but may have a care delivery model problem is operationally actionable. A primary care team in which physicians see 3 patients per hour — supported by medical assistants, care coordinators, CHTs, and behavioral health care managers who handle pre-visit preparation, care gap follow-up, and care coordination — has twice the effective capacity of a physician practicing in a traditional solo model.

Vermont's Blueprint PCMH model embeds this team-based care logic in its recognition standards. The operational challenge is that many Vermont practices — particularly smaller rural practices — lack the staffing depth to implement true team-based care. A rural practice with one physician and one MA cannot implement a full care team model regardless of how well the physician is trained. The RHT workforce investments, specifically the APRN and CHW training programs, are designed to build the care team depth that team-based care requires.

Workforce Retention — The Math of Turnover

Physician and clinician turnover in rural healthcare is both a symptom of system stress and a cause of it. Every physician departure costs approximately \$500,000-\$1 million to replace (recruitment costs, onboarding time, lost productivity, temporary coverage costs). In a system where 14 hospitals are competing for a limited rural provider pool, high turnover is financially devastating — and the signal it sends to communities about healthcare stability intensifies the system fragility it creates.

The operational drivers of rural provider retention are well-documented: burnout driven by documentation burden (addressed by AI scribe technology), isolation from professional peers (addressed by telemedicine networks and CIN peer connectivity), inadequate support staff (addressed by team-based care investment), housing unavailability (addressed by Oliver Wyman's first imperative), and work-life balance limitations in single-provider practices (addressed by coverage sharing agreements within the CIN network). Vermont's transformation agenda addresses all of these — the question is whether the investments are made before the next wave of provider departures.

Section 5: Administrative Simplification — The Vermont Agenda

Oliver Wyman's community engagement found that providers across Vermont cited administrative complexity — prior authorization, CON processes, billing burden, duplicative reporting requirements — as major constraints on care delivery efficiency. Administrative simplification is not a soft nicety; it is an Operations pillar requirement that directly affects the effective capacity of Vermont's clinical workforce.

The Vermont Administrative Simplification Priorities

AHS and GMCB have identified three administrative simplification priorities that must be addressed in the Statewide Strategic Plan:

- **Prior authorization reform:** As developed in Chapter 5, Vermont's Act 167 and Act 68 both address prior authorization complexity. The specific operational target is moving to gold-carding arrangements — automatic approval for providers with demonstrated compliance histories — and eliminating PA requirements for evidence-based, guideline-concordant care in high-volume service categories.
- **CON process streamlining:** Vermont's Certificate of Need process for hospital capital investments was cited as a barrier to efficient regionalization — providers cannot build the facilities and purchase the equipment that transformation requires if CON approval takes 12-18 months and involves litigation risk. Act 167 directed GMCB to evaluate CON process improvements; Vermont's transformation requires faster turnaround for transformation-aligned capital investments.
- **Standardized quality reporting:** Vermont's quality reporting landscape requires providers to submit data to multiple programs (Blueprint, GMCB, AHEAD, commercial payers) with overlapping but non-identical measures. The Blueprint Quality Measure Crosswalk begins to address this; the Statewide Strategic Plan should commit to further standardization that eliminates duplicative data submission for commonly required measures.

Key Concepts

Sources: HTR Operations pillar framework; Oliver Wyman Act 167 Report (2024); Act 68 of 2025; Vermont RHT Program Application (November 2025); CMS ACO REACH and MSSP operational guidance; CMS Medicare Risk Adjustment documentation; AHS Transformation Reports (2025); Vermont Blueprint for Health Annual Report (2024).

Chapter 10: The Clinical Pillar — Redesigning Care Delivery for a Transformed System

The Blueprint for Health, Behavioral Health Integration, CCBHC Expansion, Primary Care Transformation, and Vermont's Clinical Quality Architecture

Sources: Vermont Blueprint for Health Annual Report 2024; Vermont DMH CCBHC Program; AHS Mental Health Integration Pilot Evaluation 2025; Oliver Wyman Act 167 Recommendations; AHS Transformation Reports (2025); Act 68 of 2025; Vermont RHT Program Application; SAMHSA CoCM resources; Population Health Management journal (Blueprint ROI study).

The Clinical Pillar: What It Is and Why It Is Not the Same as Clinical Quality

A common confusion in healthcare transformation discussions conflates the clinical pillar with clinical quality measurement — HEDIS scores, Star ratings, readmission rates. Clinical quality measurement is important, and this chapter addresses it, but the clinical pillar is a larger concept. It encompasses the design of care delivery itself: how care is organized, who provides it, where it is delivered, and how the various elements of the healthcare system are coordinated around the patient's actual health needs rather than around administrative or financial convenience.

In the context of Vermont's transformation, the clinical pillar question is specific and urgent: what does primary care look like in a system no longer organized around hospitals as the default site of care? What does behavioral health integration look like in a state with 245 ED visits per 10,000 for suicide ideation and self-harm, and inadequate community mental health capacity? What does care coordination look like for Vermont's rapidly growing elderly population, with the highest-cost, most complex needs? And how do these clinical design questions connect to the payment reform and operational transformation happening simultaneously?

Vermont has more developed answers to these clinical design questions than most states, because Vermont has been running the Blueprint for Health — the most sophisticated all-payer primary care transformation program in the country — for more than fifteen years. The Blueprint is not a pilot or a demonstration. It is operating infrastructure, funded by all payers, embedded in primary care practices across all 14 Hospital Service Areas, and producing measurable results. Understanding the Blueprint is the foundation for understanding Vermont's clinical pillar strategy.

Section 1: The Vermont Blueprint for Health — Fifteen Years of Primary Care Transformation

The Blueprint for Health is Vermont's all-payer primary care transformation program, established in 2006 and reaching statewide implementation by 2013. It is, in the view of most independent analysts, the most successful sustained primary care transformation initiative in American health policy. Its longevity, its all-payer architecture, its integration with Vermont's health data infrastructure, and its documented results make it the clinical foundation on which Vermont's broader healthcare transformation depends.

The Blueprint Architecture

The Blueprint operates through three integrated components that together constitute a population health management system at the primary care level:

Figure 10.1 — Vermont Blueprint for Health: three integrated components. Sources: Blueprint for Health website; Vermont Blueprint Overview (GMCB, 2022); Blueprint Annual Report 2024.

The Evidence: What the Blueprint Has Produced

Vermont's Blueprint is one of the most extensively studied primary care transformation programs in the country. The evidence base spans more than a decade of operation and multiple independent evaluations.

The most rigorous evaluation — a six-year longitudinal study using VHCURES all-payer claims data published in Population Health Management — compared Blueprint participants to non-Blueprint primary care populations and found consistent advantages across expenditure, utilization, and quality outcomes. The critical financial finding: medical expenditures decreased by approximately \$5.8 million for every \$1 million spent on the Blueprint initiative. This is a 5.8:1 return on investment, driven primarily by

lower hospitalization rates and reduced outpatient facility use — precisely the kinds of avoidable utilization that Oliver Wyman identified as the largest source of indirect savings from Vermont’s transformation.

Medicare specifically found statistically significant healthcare savings for its members in Vermont’s Blueprint program. The Blueprint’s own annual ROI analyses, comparing the Blueprint period to the pre-Blueprint baseline and to non-participating practices, consistently showed a positive all-payer return on investment. The finding that Blueprint patients had better outcomes across payer types — including Medicaid beneficiaries, who typically face the most barriers to effective primary care — is particularly significant for Vermont’s equity goals.

Figure 10.2 — Blueprint for Health clinical outcomes and ROI. Sources: Population Health Management journal; AHS November 2025 Transformation Report; Blueprint for Health 2024 data.

The Blueprint and AHEAD: A Critical Transition

Vermont’s Blueprint program has operated since 2010 under a CMS Multi-Payer Advanced Primary Care Practice Demonstration that allowed Medicare to participate in PCMH and CHT payments. That agreement is scheduled to end with the conclusion of the All-Payer ACO Model. The AHEAD State Agreement resolves this transition risk: Primary Care AHEAD provides a voluntary program for primary care practices to receive enhanced prospective, risk-adjusted Medicare payments for coordinated care, with four payment pathway options including fully prospective models.

The transition from the current Medicare Blueprint payment to Primary Care AHEAD is operationally significant. Practices that currently receive Blueprint PMPM payments for Medicare patients will need to evaluate whether to participate in PC AHEAD, understand the new payment pathway options, and potentially redesign their care model to meet AHEAD’s enhanced primary care requirements. AHS and DVHA have committed to

supporting practices through this transition, recognizing that disrupting the Blueprint's Medicare payment stream would undermine one of Vermont's most effective clinical programs at precisely the moment transformation demands more of it.

Vermont's Blueprint enrollment target under AHEAD is ambitious: the program aims to substantially increase the percentage of Vermont's population — and particularly its Medicare population — attributed to Blueprint-participating practices. This expansion requires both new practice recruitment and support for existing practices to deepen their care management capabilities.

Vermont in Practice: The Blueprint Quality Measure Crosswalk

Vermont's primary care quality landscape is unusually complex because practices must simultaneously meet the quality requirements of multiple programs: Blueprint PCMH standards, AHEAD quality metrics, HEDIS measures used by commercial payers, and GMCB's hospital quality framework. Oliver Wyman documented the fragmentation this creates — multiple measurement systems with overlapping but not identical measures, creating administrative burden for practices trying to optimize across all of them.

AHS and Blueprint have developed a Quality Measure Crosswalk — an interactive tool that maps measures across programs, identifies overlaps, and helps practices understand which quality investments serve multiple accountability requirements simultaneously. This is operationally important: a primary care practice that installs a hypertension management protocol does not need to report separately to Blueprint, AHEAD, and commercial payers if those programs are aligned around the same clinical measure.

The crosswalk also reveals the gaps — places where programs require incompatible measures, creating impossible choices for practices with limited data infrastructure. These gaps are a specific target for the AHEAD implementation process and the Statewide Strategic Plan's quality alignment provisions.

Section 2: The Behavioral Health Crisis — Vermont's Most Urgent Clinical Challenge

Vermont's behavioral health data, documented in the November 2025 AHS transformation report, describes a system in crisis. With 245.5 ED visits per 10,000 for suicide ideation and self-harm — statistically higher in Rutland and Windham counties, and among Vermonters aged 15-44 — and with 30-day follow-up after mental health ED visits reaching only 76%, Vermont's behavioral health system is failing patients at the point of highest acuity. The crisis presentation data is not just a quality indicator; it is evidence of system-level failure to provide adequate access to behavioral health services before patients reach the emergency department.

Oliver Wyman's community meetings documented the texture of this failure in qualitative terms: patients with limited access to mental health services; care that is discriminatory for people who don't speak English as a first language; an observed weight bias and fatphobia making it difficult for patients to get appropriate care. The quantitative and qualitative data together describe a behavioral health system that is overwhelmed, under-resourced, and inequitably distributed — concentrated near urban centers and inaccessible in the rural communities where need is often greatest.

The Three-Layer Behavioral Health Architecture Vermont Is Building

Vermont is developing a behavioral health system with three complementary layers that together can serve the full spectrum of mental health and substance use disorder needs — from universal screening in primary care to intensive community treatment to acute crisis response.

The Collaborative Care Model: Clinical Standard of Practice for Primary Care Integration

The Collaborative Care Model merits extended treatment because it is the clinical standard of practice for behavioral health integration in primary care — not just a promising approach, but the only integrated care model with a clear evidence base across 90+ randomized controlled trials, and the only model with dedicated Medicare billing codes (CPT codes 99492, 99493, 99494). Understanding CoCM's structure is important for any primary care or health system leader contemplating behavioral health integration.

Figure 10.3 — Collaborative Care Model components and Vermont application. Sources: SAMHSA CoCM program; CMS Behavioral Health Integration Services (January 2026); Vermont MHI Pilot Evaluation (2025).

The financial sustainability of CoCM in Vermont's environment depends on Medicare and Medicaid coverage of the billing codes, which Act 68 and AHEAD together support. Vermont Medicaid's coverage of CoCM codes is a prerequisite for practices serving high-Medicaid rural populations to implement the model — and DVHA's coordination with the Blueprint and AHEAD implementation is the mechanism for ensuring that coverage is

in place. The AHEAD EAST Fund's explicit investment priority in mental health and substance use disorder services is the vehicle for funding the BHCM positions that primary care practices need to implement CoCM at scale.

"The MHI initiative allowed primary care teams to serve more patients and provide more consistent follow-up. Flexible and team-based care models helped patients engage in services, particularly those facing complex medical and social challenges."

— Vermont Blueprint Mental Health Integration Pilot Qualitative Outcomes Evaluation, 2025

The SUD Continuum: Vermont's Hub and Spoke Model

Vermont's substance use disorder treatment system is organized around a Hub and Spoke model that has been nationally recognized as a model for rural SUD treatment. Hubs provide intensive addiction treatment services; Spokes — office-based opioid treatment (OBOT) practices — provide medication-assisted treatment in primary care settings. The Blueprint's practice finder maps active Spoke locations across Vermont's primary care network. AHS actions consistent with Act 167 have expanded this system: rate increases for residential SUD treatment, creation of 'hublets' in treatment deserts, support for co-occurring mental health and SUD treatment at hubs, contingency management for stimulant use disorder, and completion of gaps analyses to identify communities without adequate access.

The 30-day follow-up rate after ED visits for alcohol use — 68% in the November 2025 data — indicates that the care coordination infrastructure between crisis and community-based SUD treatment is not fully functional. Vermont is making specific investments to close this gap through the CCBHC expansion and the MHI permanent embedding of behavioral health in primary care, but the transition from crisis to community care remains the most operationally challenging step in the SUD care continuum.

Section 3: Primary Care Redesign — Vermont's Strategy for the Front Door

Oliver Wyman's prescription for Vermont's primary care system was both clear and counterintuitive: Vermont does not have a primary care physician shortage in the traditional sense. HRSA recognizes no Health Profession Shortage Areas in Vermont. The problem is that primary care is not operating at the productivity and care model levels needed to serve Vermont's population. If primary care providers were supported to see three patients per hour, operating in team-based care models where clinical staff work at the top of their licensure and administrative burden is minimized, Vermont would have sufficient primary care supply for well into the future.

This finding has profound implications for Vermont's primary care strategy. Rather than focusing exclusively on recruiting more physicians — an expensive, slow, and uncertain strategy — Vermont can meaningfully increase primary care capacity by transforming how existing primary care operates. The Blueprint's PCMH model is the vehicle for this transformation; AHEAD's enhanced primary care payments are the financial fuel.

The Vermont Comprehensive Primary Health Care Initiative

Act 68 established the Vermont Steering Committee for Comprehensive Primary Health Care — a body whose recommendations must be incorporated into the Health Care Delivery Advisory Committee's work and AHS's Statewide Strategic Plan. This committee represents a policy commitment to treating primary care not as one service line among many but as the foundational layer of Vermont's reformed delivery system — the front door through which most Vermonters access the healthcare system and the setting where most disease management, behavioral health integration, and SDOH screening should occur.

Vermont's comprehensive primary health care vision, as embedded in Act 68 and the AHEAD State Agreement, has five operational components:

Figure 10.4 — Vermont comprehensive primary health care: five operational components and investment vehicles. Sources: Act 68 of 2025; AHEAD State Agreement (January 2025); AHS Transformation Plans 2025.

Primary Care's Role in Reducing Hospital Utilization

The connection between primary care investment and hospital utilization reduction is one of the best-established relationships in health services research, and Vermont's own Blueprint data confirm it. The 2013 evaluation finding — lower hospitalization rates and reduced outpatient facility use for Blueprint participants — is the mechanism through which primary care investment produces hospital savings under a global budget.

Vermont's November 2025 data shows 32.3% of all ED visits are potentially avoidable — meaning they represent primary care access failures, not genuine emergency needs. At Vermont's scale (approximately 14 hospitals, roughly 200,000+ ED visits annually across the system), reducing potentially avoidable ED visits from 32.3% to a benchmark level of 25% would eliminate approximately 14,600 ED visits per year. At average ED visit costs in

the \$800-1,200 range, this reduction represents \$12-18 million in annual savings — and that is before accounting for the downstream hospital admissions from avoidable ED visits that could have been prevented by earlier primary care intervention.

This is why Act 68's explicit goal of increased investment in primary care is not a soft preference — it is a financial necessity. Under global budgets, every avoidable hospitalization or ED visit is a direct cost without offsetting revenue. Hospitals operating under global budgets have a powerful financial incentive to invest in the primary care infrastructure that keeps their attributed population out of the hospital. Vermont's AHEAD EAST Fund, with its explicit investment priority in primary care, is designed to accelerate this investment.

Section 4: Vermont's Clinical Quality Architecture — Measuring What Matters

Clinical quality measurement in Vermont operates through multiple overlapping frameworks that were not designed to be coherent with each other. HEDIS measures used by commercial payers, Blueprint quality measures for PCMH payments, GMCB hospital quality reporting, AHEAD accountability metrics, and Act 167/Act 68 outcome measures all exist simultaneously, creating a measurement burden that Oliver Wyman specifically cited as a driver of administrative complexity and provider frustration.

The Statewide Strategic Plan that AHS must deliver by December 2028 presents the opportunity — and creates the obligation — to align Vermont's clinical quality measurement framework around a coherent set of indicators that serve multiple accountability purposes simultaneously. The following framework represents the clinical quality architecture that Vermont's reform requires:

Figure 10.5 — Vermont clinical quality measurement framework: priority measures, rationale, and current status. Sources: AHS Transformation Report (November 2025); Blueprint for Health data; Oliver Wyman Act 167 Report.

The HEDIS Improvement Methodology — Vermont Context

The HTR HEDIS improvement methodology — five-step framework from performance baseline through root cause analysis to sustainable systems design — applies directly to Vermont primary care practices and hospitals participating in Blueprint and AHEAD. The Vermont-specific context adds three considerations:

- All-payer measurement is both an advantage and a challenge. Vermont’s VHCURES all-payer claims database, linked to the Blueprint clinical data registry, enables HEDIS measurement across the full population — not just the commercial population a health plan would report on. This means Vermont can identify disparities that would be invisible in single-payer reporting. The challenge is that the database requires VITL connectivity that remains voluntary and incomplete, particularly among community-based providers.
- Blueprint field staff are the quality improvement infrastructure. Unlike states where practices must manage their own quality improvement in isolation, Vermont’s Blueprint field staff — one Quality Improvement Facilitator per HSA region — provide ongoing support, data analysis, and evidence-based practice coaching. This infrastructure is a competitive advantage for Vermont’s transformation; it means quality improvement capacity is not limited by the resources of individual practices.
- Performance payment alignment creates a direct financial incentive for quality improvement. Blueprint’s PMPM payments include a performance payment component tied to quality outcomes and utilization measures. AHEAD’s enhanced primary care payments include quality-based adjustments. This means Vermont primary care practices have a direct financial incentive for HEDIS improvement — not just a regulatory compliance requirement.

Section 5: Care for Vermont's Aging Population — The Clinical Imperative

Vermont's demographic trajectory — 65+ population growing 57% by 2040, exceeding 30% of total population — creates a clinical imperative that is simultaneously a financial imperative. The aging population drives demand for the most expensive and most complex services: cancer treatment, cardiac surgery, dementia care, frailty-related hospitalization, long-term care, end-of-life care. Designing the clinical system to serve this population well — which means keeping elderly Vermonters healthy, connected to their communities, and out of hospitals and nursing homes to the extent clinically appropriate — is both the right thing to do and the financial requirement of the global budget model.

The PACE Model and Vermont's Long-Term Care Gap

The Program of All-Inclusive Care for the Elderly (PACE) integrates medical, social, and long-term care services for frail elderly patients who would otherwise require nursing home placement, enabling them to remain in their communities. PACE is one of the most effective and cost-efficient models for managing the population segment that drives the highest healthcare costs in any state system. Vermont has no PACE programs. This is a significant gap that Vermont's transformation planning must address.

Oliver Wyman's community meetings documented the dimensions of Vermont's long-term care capacity crisis: lack of staffed social care facilities leads to hospital crowding when patients cannot be discharged; nursing home occupancy is high and new capacity is constrained; home and community-based services (HCBS) — the preferred alternative to institutional care — are limited by workforce shortages. Vermont's RHT Program application specifically addresses long-term care gaps: funding for dialysis units in nursing homes so dually eligible residents can receive long-term care without institutional transfer, and a second nursing home ventilator unit to double capacity for high-acuity nursing home care.

The clinical model for Vermont's aging population requires three integrated components: strong primary care for chronic disease management and prevention; robust HCBS and PACE to support aging in place; and smooth hospital-to-community transitions when hospitalization is unavoidable. The first component is the Blueprint's domain. The second requires the SDOH investment in housing and transportation that Oliver Wyman made Imperative 1. The third requires the care coordination infrastructure that the CHT model provides.

Dementia Care: The Coming Clinical Challenge

Dementia affects an estimated 14 million Americans nationally by 2060, and Vermont's older demographic profile means its per-capita rate of dementia will reach the national projection earlier. Vermont currently lacks dementia-specialized primary care at scale, adequate memory care capacity in its nursing home sector, and family caregiver support infrastructure proportionate to the approaching need. Oliver Wyman's COE framework designates memory care as a COE specialty — recognizing that dementia care at adequate quality requires specialized expertise and cannot be replicated at every community hospital.

The clinical pillar implications are clear: Vermont's transformation plan must include explicit investment in dementia care infrastructure — the training of primary care providers in cognitive assessment and early dementia management, the development of memory care COE capacity at designated facilities, the expansion of adult day programs and respite care for family caregivers, and the integration of dementia care into the community-based care model that Vermont's global budget environment requires.

Section 6: Clinical Pillar Implications for AHS's Statewide Strategic Plan

The Statewide Health Care Delivery Strategic Plan that AHS must deliver by December 2028 must address the clinical pillar explicitly. Based on Vermont's current clinical landscape and the transformation agenda established by Acts 167 and 68, the plan should contain five clinical design commitments:

Figure 10.6 — Clinical pillar commitments for Vermont's Statewide Health Care Delivery Strategic Plan. Sources: Act 68 of 2025; Oliver Wyman Act 167 Report; AHS Transformation Plans; Vermont RHT Program Application.

Key Concepts Introduced or Expanded in This Chapter

Sources: Vermont Blueprint for Health Annual Report 2024; Vermont DMH CCBHC Program website (mentalhealth.vermont.gov); Vermont Blueprint Mental Health Integration Pilot Qualitative Outcomes Evaluation (2025); Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Vermont Agency of Human Services Health Care System Transformation Report (November 2025); Act 68 of 2025; Vermont AHEAD State Agreement (January 2025); Vermont Rural Health Transformation Program Application (November 2025); Population Health Management (Blueprint ROI study, 2016); SAMHSA CoCM Program; CMS Behavioral Health Integration Services (January 2026).

Chapter 11: The AHS Restructuring Roadmap — A Six-Pillar Framework for System Architects

Redesigning Vermont's Agency of Human Services for the 2028 Strategic Plan Mandate: A Six-Pillar Framework for System Architects

Sources: Act 68 of 2025 (Vermont); Oliver Wyman Act 167 Community Engagement: Recommendations (2024); AHS Health Care System Transformation Reports (2025); Health Care Delivery Advisory Committee mandate; AHEAD State Agreement (January 2025); Vermont Rural Health Transformation Program Application (November 2025); AHS January 2025 Legislative Presentation; GMCB Act 68 Update (February 2026).

This chapter is written for a specific audience: the AHS staff, Health Care Delivery Advisory Committee members, Vermont legislators, GMCB officers, and hospital and community provider leaders who must translate Vermont's reform legislation into an operational reality. It is simultaneously academic — grounding every recommendation in evidence and analytical framework — and operational — providing the specific organizational design, governance structure, accountability metrics, and implementation timeline that Vermont's transformation requires. The six-pillar framework that organizes this entire book maps directly onto the

Statewide Health Care Delivery Strategic Plan that AHS must deliver to the Vermont Legislature by December 2028.

Section 1: The Statutory Mandate — What Act 68 Actually Requires AHS to Do

The most important thing to understand about Act 68's mandate to AHS is that it is not a mandate to write a plan. It is a mandate to restructure a system. The Statewide Health Care Delivery Strategic Plan is the vehicle, not the destination. The destination is a Vermont healthcare system that achieves the five goals of Act 167: reduced inefficiencies, lower costs, improved health outcomes, reduced health inequities, and increased access to essential services. The plan is the document that makes AHS accountable for moving toward those goals.

Act 68 is specific about what the Strategic Plan must contain. Drawing from the statute and the Health Care Delivery Advisory Committee mandate, the plan must address:

Figure 11.1 — Act 68 statutory requirements for the Statewide Health Care Delivery Strategic Plan. Sources: Act 68 of 2025; Health Care Delivery Advisory Committee mandate (healthcarereform.vermont.gov).

The Strategic Plan is not AHS's alone to write. Act 68 created the Health Care Delivery Advisory Committee — 18 members representing AHS, GMCB, the Health Care Advocate, hospitals, health centers, insurers, health professions, small businesses, and long-term care facilities — to develop and maintain the plan in collaboration with AHS. The committee also consults with the Health Equity Advisory Commission and the Vermont Steering Committee for Comprehensive Primary Health Care. This governance structure means the Strategic Plan must reflect multi-stakeholder input and consensus, not just AHS's internal analysis — a significant operational challenge and an important political constraint.

“Review existing AHS agency structure and program list to identify overlaps and opportunities for efficiency. Align Agency of Human Services agencies with Health Service Areas and meet regularly with hospitals and providers. Fully computerize and integrate Agency of Human Services subunits.”

— Oliver Wyman Act 167 Community Engagement: Recommendations, September 2024 — Priority actions for AHS in 2025

Section 2: The Organizational Diagnosis — Why AHS Must Restructure, Not Just Plan

Oliver Wyman’s finding about AHS was specific and unambiguous. After 230 meetings with hospital leaders, community organizations, state agency staff, and Vermonters across all 14 Hospital Service Areas, the consultants identified AHS’s organizational structure as one of the primary barriers to effective healthcare transformation — not because AHS lacks capable people or good intentions, but because the organization was designed for a different era and a different role.

The current AHS organizational model is what management theorists call a program-silo structure: it is organized around funding streams and program types (Medicaid, mental health, substance use, aging services, developmental services, Blueprint, health care reform) rather than around the populations it serves or the geographic communities those populations live in. This structure made sense when AHS’s primary function was administering categorical benefit programs. It does not make sense for an organization whose primary function has become managing the transformation of an interconnected healthcare delivery system.

The Four Structural Gaps

Oliver Wyman identified four specific structural gaps that AHS’s transformation must address. These are not recommendations for improvement — they are diagnoses of organizational failures that are currently impeding

transformation.

Figure 11.2 — AHS organizational gaps and required structural changes. Sources: Oliver Wyman Act 167 Recommendations (2024); AHS Transformation Reports (2025); Act 68 of 2025.

Section 3: The Redesigned AHS Operating Model — A Population Health System Operator

The organizational model that AHS must become — and that the Statewide Strategic Plan must articulate — is a Population Health System Operator: an agency that manages Vermont’s healthcare and social services ecosystem as an integrated system oriented around population health outcomes, organized geographically around Hospital Service Areas, and accountable for measurable progress toward the five Act 167 goals.

This is a fundamentally different organizational identity than AHS has historically held. The shift is not cosmetic — it requires changes to organizational structure, staff roles, governance processes, data systems, external relationships, and the agency’s own theory of how it creates value. The following sections develop this redesigned model across the three levels at which AHS must operate: statewide system architect, regional network convener, and program delivery operator.

Level 1 — Statewide System Architect

At the statewide level, AHS functions as the architect of Vermont's healthcare delivery system: setting the strategic direction, establishing the payment reform trajectory in partnership with GMCB, allocating capital investments (RHT Program, AHEAD EAST Fund, Act 68 grants), developing the accountability framework, and monitoring system performance against the five Act 167 goals.

The statewide architecture function requires four specific organizational capacities that AHS is currently building but has not yet fully developed:

- **Strategic planning capacity:** The ability to develop, maintain, and update the Statewide Health Care Delivery Strategic Plan on a three-year cycle, incorporating input from the Health Care Delivery Advisory Committee, the Steering Committee for Comprehensive Primary Health Care, the Health Equity Advisory Commission, and other required stakeholders. This is a substantial ongoing function, not a one-time project.
- **Analytics and modeling capacity:** The ability to model the cost, quality, and access implications of proposed system changes before they are implemented — answering questions like ‘what happens to access in Windsor County if Springfield Hospital converts to REH status?’ and ‘what is the projected PMPM cost reduction from the primary care investment plan?’ This requires the analytics vendor being procured jointly with GMCB, plus internal staff who can interpret and apply the outputs.
- **Federal relationship management:** Active management of Vermont's relationships with CMS (AHEAD Model), HRSA (RHT Program), CMMI (transformation grants), and other federal partners. These relationships are high-stakes, technically complex, and require dedicated staff who understand both Vermont's specific circumstances and the federal program requirements. The AHEAD State Agreement alone requires ongoing negotiation of

- **methodology, monitoring, and potential withdrawal conditions.**
- **Legislative accountability: The production and delivery of the required monthly transformation reports, the December 2028 Strategic Plan, and the three-year updates. Vermont's legislature has demonstrated active engagement with healthcare transformation — the monthly reporting requirement is a real accountability mechanism, not a formality. AHS must staff this function appropriately.**

The Health Care Delivery Advisory Committee: AHS's Governance Partner

Act 68 established an 18-member Health Care Delivery Advisory Committee to develop and maintain the Statewide Strategic Plan in collaboration with AHS. Understanding this committee's composition and mandate is essential for AHS's operational planning, because the committee is not an advisory body in the traditional sense — it has statutory authority to set affordability benchmarks and advise the legislature directly.

Committee composition (18 members):

- AHS Secretary (or designee)
- GMCB Chair (or designee)
- Office of the Health Care Advocate
- Representatives from hospitals, health centers (FQHCs), health insurers
- Representatives from nursing, pharmacy, primary care, and other health professions
- Small business representative
- Long-term care facility representative
- Additional members as specified

Primary responsibilities:

- Setting affordability benchmarks — defining measurable standards for

what ‘affordable healthcare’ means for Vermont families

- Monitoring Vermont’s healthcare system performance and its impact on public health
- Collaborating with AHS to develop and maintain the Statewide Strategic Plan
- Incorporating recommendations from the Steering Committee for Comprehensive Primary Health Care
- Advising GMCB on creating a continuous evaluation process for the delivery system
- Providing coordinated recommendations to the General Assembly on healthcare delivery

Operational implication for AHS: The committee requires dedicated staff support — meeting facilitation, research preparation, deliberation management, and recommendation documentation. AHS should treat committee support as a dedicated function, not an add-on to existing Health Care Reform Office responsibilities.

Level 2 — Regional Network Convener (HSA-Level)

At the Hospital Service Area level, AHS must function as a regional network convener — bringing together the hospitals, primary care practices, designated mental health agencies, FQHCs, community organizations, and social service providers within each HSA to develop and execute a coordinated regional transformation strategy.

This is where Oliver Wyman’s most important organizational recommendation applies: align AHS sub-units with Hospital Service Areas and meet regularly with hospitals and providers. The mechanism for doing this is the designation of HSA-level AHS coordinators — staff whose primary function is to be the face of AHS within a defined geographic area, convening the relevant AHS programs and community partners around the hospital

service area's specific transformation priorities.

Vermont has 14 Hospital Service Areas and a geographically distributed health system. The Blueprint for Health already deploys field staff in each HSA — its Quality Improvement Facilitators, Community Health Team coordinators, and field service staff are embedded in local communities. The Blueprint's HSA infrastructure is the organizational model AHS should replicate and build on for broader transformation coordination.

Figure 11.3 — Vermont 14-Hospital Service Areas: regional profile and priority transformation challenges for AHS HSA-level coordination. Sources: Oliver Wyman Act 167 Report; Vermont RHT Program Application; AHS Transformation Reports 2025.

Level 3 — Program Delivery Operator

At the program level, AHS continues to administer Vermont's health-care and social service programs — Medicaid (through DVHA), mental health (through DMH), Blueprint for Health, developmental services, aging services, and others. The structural change is not the elimination of program-level operations but the addition of cross-program coordination mechanisms and the explicit alignment of program operations with the transformation goals and geographic structures established at the statewide and regional levels.

The critical program-level change is the integration of social service programs into the healthcare transformation framework. Oliver Wyman's three imperatives — build housing and fix transportation, pay with RBP and move to global budgets, move care out of hospitals — cannot be achieved by the healthcare programs alone. DVHA, Blueprint, and the Health Care Reform Office can transform payment and care delivery; they cannot build housing or run transportation programs. But the AHS programs that fund housing supports (LIHEAP, general assistance), transportation (Medicaid transportation, EMS), food assistance (SNAP administration),

and developmental services are sitting in the same agency — they are simply not coordinated with the healthcare transformation agenda.

The specific structural mechanism for this integration is the HSA Coordinator role: a designated AHS staff member for each HSA region who convenes the relevant program staff from DVHA, DMH, Blueprint, ADS, DAIL, and the Health Care Reform Office around the hospital transformation plan for their region. The HSA Coordinator does not control all program resources — that would require a reorganization far beyond what Act 68 mandates — but they create the coordination layer that currently does not exist.

Section 4: The Six-Pillar Framework Applied to AHS's Restructuring Mandate

The HTR six-pillar framework — Policy, Economics, Technology, Clinical, Equity, and Operations — provides the analytical architecture for AHS's Statewide Strategic Plan. Each pillar corresponds to a domain of AHS's transformation mandate, a set of organizational capabilities AHS must develop, and a set of measurable outcomes AHS must track. The following section maps each pillar explicitly onto AHS's restructuring requirements.

AHS's policy pillar function is to ensure that Vermont's transformation agenda is legally grounded, federally funded, and legislatively supported. The specific organizational requirements are:

- AHEAD Model management: Active management of Vermont's AHEAD State Agreement, including ongoing CMS negotiation on global budget methodology, monitoring compliance with TCOC targets, and managing the conditions under which Vermont can withdraw if the model is not working. This requires dedicated staff with both healthcare finance expertise and federal program management experience.
- Act 68 compliance tracking: Systematic monitoring of Act 68's statutory

requirements — RBP by FY2027, global budgets for non-CAH hospitals by FY2028, Statewide Strategic Plan by December 2028 — and proactive management of the legislative reporting calendar.

- RHT Program administration: Oversight of Vermont's \$195M five-year award, including grant management, contractor oversight, annual reporting to CMS, and alignment of RHT activities with Act 167 and Act 68 goals. This is a major federal grant program requiring professional grant management infrastructure.
- Federal policy monitoring: Tracking changes to Medicare, Medicaid, and federal healthcare policy that affect Vermont's transformation — including Medicaid reconciliation legislation, CMMI model changes, and CMS regulatory guidance — and updating Vermont's strategy in response.

AHS's economics pillar function is to manage Vermont's payment reform transition — coordinating with GMCB on RBP and global budget design, monitoring hospital financial sustainability, and ensuring that payment reform produces the premium reduction and affordability improvement that Act 68 requires.

- Payment reform coordination: Regular coordination with GMCB on RBP methodology development, global budget design, and the premium passthrough monitoring that Act 68 requires. AHS brings the Medicaid perspective and the population health data; GMCB brings the regulatory authority. Neither can execute the economics transformation without the other.
- Hospital financial monitoring: Tracking Vermont hospitals' financial positions against the Oliver Wyman projections — particularly the CAH operating profit gap (\$938 vs. \$2,784 benchmark) and the PPS administrative cost premium (\$2,730 vs. \$1,427 benchmark) — and intervening with technical assistance or grant support before hospitals

reach crisis.

- AHEAD EAST Fund management: Directing the up to \$150M annually in additional Medicare funds toward the primary care, behavioral health, home health, and long-term care investments that make global budgets produce population health improvement rather than just cost reduction.
- Transformation ROI tracking: Measuring whether Vermont’s transformation investments — RHT Program, Act 68 grants, AHEAD EAST Fund — are producing the \$400M+ in savings that Oliver Wyman projected. This requires the analytics capability described in the organizational gap analysis.

AHS’s technology pillar function is to ensure Vermont has the data infrastructure needed to manage a transformed healthcare system. The current data environment — VHCURES as the all-payer claims database, VITL as the health information exchange, the Blueprint clinical data registry, and fragmented program-level data systems — was adequate for monitoring discrete programs. It is not adequate for managing population health at the system level.

- VHCURES expansion and real-time access: AHS and GMCB must expand VHCURES to include more timely claims data, enhance provider connectivity (currently voluntary and incomplete), and make VHCURES data accessible for transformation planning analytics. The November 2025 AHS report acknowledged that most available measures reflect 2023 or earlier data — a two-year lag that makes real-time transformation management impossible.
- HIE strategic development: The Act 167-mandated HIE feasibility assessment — currently evaluating the cost-benefit of a statewide electronic health record — is a technology pillar decision with major implications for the Strategic Plan. A statewide EHR would resolve the fragmented data environment that Oliver Wyman cited as a barrier to

coordinated care; the question is whether the investment is justified and whether the governance model is viable.

- **Analytics vendor implementation:** The joint AHS-GMCHB analytics vendor procurement underway must produce a platform capable of modeling transformation scenarios, evaluating hospital proposals, and generating the outcome metrics the Strategic Plan requires. This is the most urgent near-term technology investment.
- **Cross-program data integration:** AHS's SDOH programs (housing, transportation, food assistance) must be connected to healthcare outcome data so that the relationship between social service investment and healthcare utilization can be measured. Without this integration, AHS cannot demonstrate to the legislature that its SDOH investments are producing healthcare savings.

AHS's clinical pillar function is to design and support the care delivery models that Vermont's transformed payment system requires. Payment reform creates the incentive for clinical redesign; AHS must create the programmatic infrastructure that makes clinical redesign possible.

- **Blueprint for Health expansion and AHEAD transition:** Managing the transition of Blueprint's Medicare payments from the current demonstration to Primary Care AHEAD, expanding PCMH enrollment, and ensuring that the MHI pilot's behavioral health integration is made permanent through AHEAD EAST Fund investment.
- **CCBHC network development:** Executing the plan to certify five additional CCBHCs by July 2026, covering all major HSA regions, and integrating the CCBHC network with primary care, hospital emergency departments, and the 988 crisis system.
- **Care model technical assistance:** Supporting hospitals and primary care practices in redesigning their care models for the global budget environment — shifting from volume maximization to population health

management, building care coordination capacity, and implementing the team-based care models that primary care transformation requires.

- Long-term care capacity development: Addressing the PACE gap, supporting dementia care COE development, and working with DVHA to expand HCBS capacity so that Vermont's growing elderly population can age in their communities rather than consuming hospital capacity.

AHS's equity pillar function is to ensure that Vermont's transformation reduces rather than exacerbates health disparities — particularly the geographic disparities between Burlington's Chittenden County and the Northeast Kingdom, and the access disparities affecting Vermont's Medicaid population, rural residents, and linguistic minorities.

- Geographic equity monitoring: Tracking health outcomes, primary care access, and insurance coverage rates by county — with specific attention to the Northeast Kingdom (6-8% uninsurance vs. 3% statewide) and rural counties with higher rates of behavioral health crisis presentations, opioid overdose, and diabetes.
- Medicaid access protection: Monitoring whether RBP and global budget implementation affects Medicaid patients' access to care — specifically whether the reduction in commercial prices affects hospitals' willingness to maintain Medicaid patient volumes. Act 68 explicitly requires consideration of access protection in the RBP design.
- Culturally competent care investment: Addressing the equity gaps Oliver Wyman documented — lack of language access, cultural mismatch in clinical care, inadequate LGBTQ+ competency, weight bias — through workforce investment (provider diversity recruitment), training requirements, and community partnership programs.
- SDOH program coordination: Ensuring that AHS's SDOH programs — housing supports, transportation, food assistance — are explicitly directed toward the populations and communities where SDOH gaps

are most driving healthcare utilization and health disparities.

AHS's operations pillar function is to manage the execution of Vermont's transformation — the 14 hospital transformation plans, the regional coordination processes, the technical assistance programs, and the administrative simplification agenda that Oliver Wyman identified as essential for reducing provider burden.

- Hospital transformation plan oversight: Managing the pipeline from Draft Care Transformation Strategy Outlines through Draft Plans to Final Plans for all 14 hospitals, ensuring plans are coordinated regionally and nationally, and tracking implementation against Act 167 outcome objectives.
- Administrative simplification: Acting on Oliver Wyman's recommendation to further simplify prior authorization, the CON process, and Medicaid billing — reducing the administrative burden that drives physician frustration and consumes provider resources that should go to patient care.
- Project Management Office: Establishing the PMO infrastructure needed to manage Vermont's transformation portfolio — RHT Program, AHEAD implementation, hospital transformation plans, Strategic Plan development — with professional project management, risk tracking, and escalation processes.
- EMS and transportation transformation: Coordinating the EMS professionalization and regionalization that Oliver Wyman prioritized — addressing the 31-agency fragmentation that makes inter-facility transfers complex and expensive, and building the transportation infrastructure that population health management requires.

Section 5: The AHS Restructuring Implementation Timeline

The AHS restructuring is not a future event — it is happening now, driven by statutory deadlines that are already creating organizational pressure. The following timeline organizes the key milestones that AHS must meet, grouped by the organizational capability they require.

Figure 11.4 — AHS restructuring and Vermont transformation implementation timeline. Sources: Act 68 of 2025; AHEAD State Agreement (January 2025); AHS Transformation Reports; Vermont RHT Program Application.

Section 6: A Framework for the 2028 Statewide Health Care Delivery Strategic Plan

The Statewide Health Care Delivery Strategic Plan is the most consequential document AHS will produce in the 2020s. It will define Vermont’s healthcare system for the decade following its delivery. The following framework provides the structural architecture for the plan, organized around the six pillars and Act 68’s statutory requirements.

Proposed Plan Structure

Figure 11.5 — Proposed structure for Vermont’s 2028 Statewide Health Care Delivery Strategic Plan. Sources: Act 68 of 2025; Health Care Delivery Advisory Committee mandate; HTR six-pillar framework.

The Central Commitment the Plan Must Make

Beyond its analytical content, the Strategic Plan must make one central commitment that is currently absent from Vermont’s transformation discourse: a specific, quantified trajectory for what Vermont’s healthcare system will look like in 2031, 2035, and 2040, with the financial modeling to show it is achievable.

Oliver Wyman provided the diagnostic scenario: without transformation, 13 of 14 hospitals report losses by 2028, premiums reach unsustainable levels, and the healthcare system contracts through unplanned closures rather than orderly redesign. The Strategic Plan must provide the alternative scenario with equal specificity: if Vermont executes the transformation agenda on the timelines described in this chapter, what does the system look like in 2031? How many hospitals maintain inpatient beds? What is the primary care investment level? What is the premium growth rate? What is the uninsurance rate in the Northeast Kingdom?

This is the difference between a strategic plan and a strategic aspiration. Vermont’s legislators, hospital leaders, community organizations, and Vermonters deserve to know not just what AHS is doing but what the state will look like if AHS succeeds. The six-pillar framework provides the analytical architecture for making that commitment with intellectual honesty about what is achievable and what remains uncertain.

Figure 11.6 — Key parameters for AHS’s Strategic Plan development. Sources: Act 68; VTDigger; Oliver Wyman Act 167 Report; Vermont RHT Program

Application.

Section 7: Vermont as National Template — Why This Restructuring Matters Beyond Vermont

Vermont's AHS restructuring is not a local administrative exercise. It is the first attempt by a state agency to systematically redesign itself as a population health system operator in response to a statutory mandate for healthcare transformation. Every state that faces Vermont's demographic and fiscal trajectory — which is to say, every state in the Northeast, most of the Midwest, and eventually every state — will need to answer the same organizational design questions Vermont is answering now.

The questions are not Vermont-specific. How do you align a state human services agency around geographic service areas rather than program silos? How do you build the analytics capacity to model the impact of structural change before you implement it? How do you integrate social service programs with healthcare delivery reform so that SDOH investments actually reduce healthcare utilization? How do you manage a multi-stakeholder strategic planning process that includes hospitals, insurers, providers, and community organizations without producing a lowest-common-denominator plan that satisfies everyone and commits to nothing?

Vermont will have answers to these questions by December 2028 — answers grounded in real experience with a real system facing real pressures. Those answers will be the most valuable policy contribution Vermont has made to American healthcare in a generation. The national significance of Vermont's transformation is not that Vermont is large or economically powerful. It is that Vermont is early — early enough to produce a tested model before the rest of the country reaches the same crisis point, and early enough for the lessons to still be actionable.

This is why the book you are reading exists: to document Vermont's experience in a form that is analytically rigorous, practically useful, and transferable to the healthcare leaders and policymakers who will face Ver-

mont's challenges in their own states and systems. The AHS restructuring chapter is not the end of the story — it is the moment when the story becomes fully operational.

Key Concepts Introduced in This Chapter

Sources: Act 68 of 2025 (Vermont); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024); Vermont Agency of Human Services Health Care Delivery Advisory Committee page (healthcarereform.vermont.gov); AHS Health Care System Transformation Reports (August 2025, November 2025); Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts, House Health Care Committee (January 31, 2025); AHEAD State Agreement (January 17, 2025); Vermont Rural Health Transformation Program Application (November 3, 2025); GMCB Act 68 Update on RBP and Global Budgets (February 17, 2026); All prior chapters of this book.

Chapter 12: The Equity Pillar — Closing Gaps, Not Just Averaging Them

Rural Health Disparities, Racial and Demographic Equity, Social Determinants of Health, the GLP-1 Access Crisis, and Vermont's Equity Architecture

Sources: Vermont Department of Health Health Equity Data Report (January 2025); Oliver Wyman Act 167 Recommendations (2024); Vermont RHT Program Application (November 2025); KFF Medicaid GLP-1 Coverage Analysis (January 2026); CMS BALANCE Model announcement (December 2025); Health Affairs rural health equity research (June 2024); AHS Transformation Reports (2025); Act 68 of 2025.

The Equity Pillar: Why Average Outcomes Are the Wrong Target

The most common failure mode in healthcare quality measurement is optimizing for the average. A system that improves its average readmission rate, average primary care access, or average chronic disease control while allowing — or widening — the gap between its best-served and worst-served populations has not achieved quality improvement. It has achieved statistical improvement while perpetuating or deepening the inequity that

produces the worst outcomes.

The Equity pillar of the HTR framework is founded on a different target: every population segment should have access to the conditions that support health, and disparities in access and outcomes that are attributable to race, income, geography, disability, language, sexual orientation, or gender identity should be eliminated, not averaged away. This is not a soft aspiration. It is an analytically precise goal with measurable indicators, specific intervention categories, and accountability structures that Vermont's Act 167 and Act 68 have embedded in law.

Vermont's equity challenge is distinctive enough from the typical urban healthcare equity narrative that it requires explicit treatment before the analytical framework can be applied effectively. Vermont is approximately 94% white — one of the least racially diverse states in the country. Its 4.5% foreign-born population is concentrated in Burlington and a handful of other communities. The equity disparities that drive headlines in Boston, Chicago, or Los Angeles — stark racial gaps in maternal mortality, cardiovascular disease mortality, diabetes management, and access to specialty care — are present in Vermont but at smaller scale and with different geographic distribution. Vermont's dominant equity challenge is geographic: the health and healthcare access gap between Chittenden County (Burlington, the state's economic and medical center) and the rural counties — particularly the Northeast Kingdom — where poverty is higher, providers are scarcer, transportation is unreliable, and broadband is limited.

This does not mean racial equity is unimportant in Vermont. The Vermont Department of Health's 2024 health equity data shows clear disparities: Black adults in Vermont have an 82% health insurance coverage rate compared to 94% statewide. Adults who are Asian, Native Hawaiian or Pacific Islander, Black, or another race are significantly less likely to have a primary care provider than white adults. LGBTQ+ Vermonters, Vermonters with disabilities, and lower-income Vermonters delay care at higher rates due to cost. And Vermont's BIPOC communities face the same structural racism in healthcare settings that affects every state — weight bias, cultural incompetence, language barriers, and documented differential treatment

quality.

Vermont's equity agenda must address both the geographic and the demographic dimensions simultaneously — and the analytical framework for doing so is the same in both cases: identify the disparity, trace it to its root cause, and design interventions that address the cause rather than merely measuring the symptom.

Section 1: Vermont's Equity Landscape — What the Data Shows

Geographic Equity: The Rural-Urban Divide

Vermont's most pervasive and consequential health equity disparity is geographic. The Northeast Kingdom — Caledonia, Essex, and Orleans counties — concentrates the state's most severe access, affordability, and outcome disparities:

Figure 12.1 — Vermont geographic equity disparities: key indicators. Sources: AHS November 2025 Transformation Report; Vermont RHT Program Application; Vermont Department of Health.

The Northeast Kingdom's geographic isolation creates a compound equity burden that is different in character from urban poverty. A low-income resident of Burlington faces affordability barriers but has access to UVMMC, an FQHC network, mental health services, and public transportation. A low-income resident of Essex County faces the same affordability barriers compounded by 45-minute drives to the nearest primary care provider, inadequate broadband that limits telehealth access, unreliable public transportation that makes scheduled appointments difficult to maintain, and a housing market where the few available rental units are concentrated in towns without healthcare services.

Oliver Wyman's community engagement found that this compound geo-

graphic burden produces a distinct pattern of care avoidance: community members described not going to care because the premiums and out-of-pocket costs are too high; long ambulance waits; long and difficult travel to care sites; and long waits in the ED when they do access care. These are not isolated experiences — they are the structural conditions of rural healthcare access in Vermont’s most underserved areas.

Racial and Demographic Equity

Vermont’s racial equity disparities, while smaller in absolute number than in more diverse states, are documented and consequential. The 2024 Vermont Department of Health equity report shows that statewide, 94% of Vermont adults have health insurance — but only 82% of Black adults. Primary care access follows a similar pattern: while 90% of Vermont adults overall have a personal healthcare provider, the rate for Asian or Pacific Islander adults (81%), Black adults (80%), and adults of another race (79%) is significantly lower. These gaps are not attributable to lack of coverage; they reflect structural barriers to access that persist even when insurance is present.

BIPOC Vermonters, LGBTQ+ Vermonters, Vermonters with disabilities, and lower-income Vermonters were all more likely to delay care due to cost in a 2024 state survey. The same populations were more likely to report their community was not safe for walking — an indicator of the broader social and built environment conditions that shape health. Eight percent of all Vermonters said they did not get medical care because of cost in the prior year; that rate is higher among people with less education, lower income, BIPOC adults, LGBTQ+ adults, and people with a disability.

Vermont’s Indigenous population — primarily Abenaki communities — faces specific equity challenges that the state’s data systems are not yet adequately equipped to measure. A June 2024 state health equity report documented disparities for the American Indian or Alaska Native population based on 2021-2022 survey data, but Vermont’s small and geographically dispersed Indigenous population creates statistical challenges for survey-based measurement. Vermont Public reporting in 2025 found

that several Indigenous community organizations had not been in contact with the Health Department, suggesting that outreach and engagement gaps compound the measurement gaps.

The SDOH Burden

Vermont's equity data from the RHT Program application quantifies the SDOH burden that Oliver Wyman identified as both a driver of healthcare utilization and a barrier to healthcare access:

Figure 12.2 — Vermont SDOH burden: key indicators and equity implications. Sources: Vermont RHT Program Application (November 2025); Oliver Wyman Act 167 Report; Vermont Department of Health.

Section 2: The Disparity Root Cause Taxonomy — Why Vermont's Equity Gaps Exist

Identifying a health disparity is the beginning of an equity intervention, not the end. The same disparity in access or outcome can arise from fundamentally different causes — and an intervention that addresses the wrong cause will not close the gap. Vermont's equity strategy must be organized around the root causes of specific disparities, not around the disparities themselves.

Drawing from the HTR Health Equity pillar framework, Vermont's equity gaps fall into five root cause categories. For each category, Vermont has a documented disparity, a traceable mechanism, and a set of interventions — some already underway, some required.

The five root cause categories are not mutually exclusive — a Black rural resident of Windham County may face geographic access barriers, SDOH barriers, and trust barriers simultaneously. Vermont's equity interventions are most effective when they are designed to address multiple root causes for the populations with compound equity burdens, rather than addressing each category in isolation.

Section 3: Rural Equity — Vermont's Defining Challenge

Vermont's rural equity challenge is analytically important for a national audience because it represents a type of health disparity that is often underweighted in national health equity frameworks dominated by urban racial equity narratives. Rural health disparities are not less serious than urban racial disparities — they are different in character, different in mechanism, and require different interventions.

The Health Affairs research on rural health equity in northern New England — directly relevant to Vermont — identifies five principles for addressing rural health equity that align closely with Vermont's transformation agenda. Rural communities need investments that address the social and economic determinants of health, not just the clinical delivery system. Rural health equity requires provider diversity that reflects community demographics and culture. Rural telehealth must be accompanied by the broadband and device infrastructure that makes it genuinely accessible. Rural community organizations must be genuine partners in healthcare design, not passive recipients of state-designed programs. And rural health equity data must be disaggregated at the county and sub-county level — state averages mask the severity of rural disparities.

The Northeast Kingdom: Vermont's Equity Priority

The Northeast Kingdom — Caledonia, Essex, and Orleans counties — is Vermont's clearest and most severe equity priority. It is the most rural part of the state, the most economically distressed, the most underserved by healthcare providers, and the most distant from UVMHC and the Burlington healthcare hub. The three counties that make up the Northeast Kingdom have some of the lowest population densities in the eastern United States, unemployment rates above the state average, high rates of disability, limited broadband coverage, and the highest uninsurance rates in the state.

The healthcare facilities serving the Northeast Kingdom — North Country Hospital in Newport, Northeastern Vermont Regional Hospital in St. Johnsbury, and Copley Hospital in Morrisville for the Lamoille/Orleans border area — are among Vermont's most financially distressed. North Country and Northeastern Vermont Regional are candidates for Tier 3 designation under the Oliver Wyman regionalization blueprint — meaning they may not be able to sustain full inpatient operations in the long term. Any transition away from inpatient services at these facilities, without robust REH conversion and EMS/telehealth investment, would represent a catastrophic equity regression for the communities they serve.

Vermont's AHS must be explicit in the Statewide Strategic Plan that the Northeast Kingdom is an equity priority that constrains the pace and structure of regionalization. The financial logic of consolidating services at RSC hospitals in Burlington, Rutland, or Bennington cannot override the equity obligation to maintain emergency access in the most rural, most isolated communities in the state. The tension between system financial sustainability and rural access equity is genuine, and pretending it is not does not resolve it.

Vermont in Practice: The Northeast Kingdom Equity Constraint

Any Vermont hospital transformation plan that closes emergency services in the Northeast Kingdom without replacing them with services that can actually be accessed by Northeast Kingdom residents — not hypothetically

accessible via 90-minute transport to Burlington, but genuinely accessible in the community — fails the equity test that Act 167 and Act 68 have embedded in Vermont law.

The equity constraint does not mean the Northeast Kingdom's hospitals cannot change. It means the sequence matters: investments in EMS capacity, telehealth infrastructure, and community paramedicine must precede or accompany any reduction in inpatient services. The Rural Emergency Hospital designation provides a viable path precisely because it maintains 24/7 emergency access while eliminating the unsustainable inpatient infrastructure. The RHT Program's priority investments in the Northeast Kingdom — broadband, EMS, RPM, telehealth — are equity investments as much as access investments.

The equity test for any transformation proposal affecting Northeast Kingdom hospitals should be specific: after this change, can a 70-year-old resident of Canaan, Vermont who has a heart attack at 2 a.m. receive timely emergency care? Can a person with serious mental illness in Newport access a CCBHC without a two-hour drive? Can a child in Lyndonville see a primary care provider within 14 days? These are not abstract equity metrics. They are the minimum conditions for health equity in rural Vermont.

Section 4: The GLP-1 Access Crisis — The Most Important New Equity Issue

The emergence of GLP-1 receptor agonists — semaglutide (Wegovy, Ozempic) and tirzepatide (Zepbound, Mounjaro) — as transformatively effective treatments for obesity, type 2 diabetes, and cardiovascular disease risk has created what may be the most consequential new health equity issue in American healthcare since the introduction of statins. The drugs work. The evidence is unambiguous: they produce average weight loss of 15-20% of body weight, dramatically reduce cardiovascular events in high-risk populations, and improve glycemic control in type 2 diabetes. For

patients at the intersection of obesity, diabetes, and cardiovascular disease — a population concentrated in lower-income and rural communities — GLP-1s could be transformative at the population level.

The equity problem is pricing and coverage. At list prices exceeding \$1,000 per month before rebates, GLP-1s for obesity are financially inaccessible to the vast majority of patients who would benefit from them. As of January 2026, only 13 state Medicaid programs covered GLP-1s for obesity treatment — down from 16 in late 2025, as four states eliminated coverage in response to budget pressure from Medicaid cuts. Vermont Medicaid covers GLP-1s for medically accepted FDA-approved indications but expressly excludes coverage for weight loss under its State Plan. Vermont Medicaid covers GLP-1s for type 2 diabetes management but not for obesity as a primary indication.

The equity consequence is straightforward: GLP-1s are currently accessible primarily to patients with commercial insurance generous enough to cover them, and to patients with type 2 diabetes or cardiovascular disease (who qualify under Medicare and Medicaid for the diabetes and cardiovascular indications). Patients with obesity but not yet diagnosed with type 2 diabetes — many of them lower-income Vermont Medicaid patients in the communities with highest obesity prevalence — cannot access these drugs under current coverage unless they pay full price out of pocket. At \$1,000+ per month, full price is not an option for Medicaid patients.

“Limitations on coverage for obesity drugs in Medicare and Medicaid mean that millions of people who have obesity and might benefit from taking GLP-1s may be unable to access them unless they are able to pay the full cash price out of their own pockets, which would likely be prohibitive for people with Medicaid who must have low incomes to qualify for the program.”

— KFF, **What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid, 2026**

The BALANCE Model: A Partial Solution

In December 2025, CMS announced the BALANCE (Better Approaches to Lifestyle and Nutrition for Comprehensive hEalth) Model — a voluntary program enabling state Medicaid agencies and Medicare Part D plans to cover GLP-1 medications for obesity by negotiating prices with manufacturers. The model launches in Medicaid as early as May 2026 and in Medicare Part D in January 2027, with a Medicare GLP-1 Bridge program providing \$50/month co-pay access for Medicare beneficiaries beginning July 2026.

BALANCE is a meaningful step toward resolving the GLP-1 equity crisis, but it has three significant limitations that Vermont and every state must navigate. First, manufacturer participation is voluntary — if drug manufacturers do not negotiate with CMS under BALANCE terms, the program produces no coverage expansion. Second, state Medicaid agency participation is voluntary — the four states that just eliminated GLP-1 obesity coverage are unlikely to opt into BALANCE given the budget pressures that drove the elimination. Third, the negotiated prices, while reduced from list prices, may still be expensive for state Medicaid budgets already under pressure from the 2025 reconciliation legislation's Medicaid cuts.

Vermont's DVHA must evaluate BALANCE participation as an equity policy decision, not purely a budget decision. The populations who would benefit most from GLP-1 obesity coverage under Vermont Medicaid — lower-income rural Vermonters with obesity, diabetes risk, and cardiovascular disease risk — are precisely the populations whose healthcare costs are most likely to be reduced by effective obesity treatment. The long-term fiscal argument for coverage is strong; the short-term budget pressure against it is real.

The GLP-1 Equity Implication for Vermont's Transformation

Vermont's transformation narrative must grapple with the GLP-1 equity question explicitly, for two reasons. First, obesity and its metabolic consequences — type 2 diabetes, cardiovascular disease, hypertension, sleep apnea — are major drivers of the high-cost utilization that Vermont's global budget system will be trying to reduce. GLP-1s have demonstrated efficacy in reducing all of these conditions. If GLP-1s become broadly accessible to Vermont's Medicaid population through BALANCE participation, they could accelerate the utilization reduction that global budgets are designed to incentivize — producing population health improvement and healthcare cost reduction simultaneously.

Second, the GLP-1 access gap is the most vivid current example of what the book's Part V identified as the equity reckoning in pharmaceuticals: potentially transformative drugs whose benefits disproportionately accrue to higher-income patients who can afford them, while lower-income patients — who have higher obesity rates, higher diabetes rates, and higher cardiovascular disease rates, partly because of the same SDOH conditions that drive Vermont's equity gaps — cannot access the treatment. Vermont's equity commitment requires active engagement with this issue, not passive monitoring.

Section 5: Vermont's Equity Architecture — What Is Already Built and What Remains

Vermont has more equity infrastructure than most states of comparable size — a product of its near-universal insurance coverage, the Blueprint for Health's SDOH screening mandate, the Health Equity Advisory Commission, the Health Equity Capacity Building Program, and the explicit equity requirements in Act 167 and Act 68. But Vermont also has documented, persistent equity gaps that this infrastructure has not yet closed. The Statewide Strategic Plan's equity section must be honest about both.

What Is Already Operational

- **Universal SDOH screening:** Blueprint PCMH practices conduct universal screening for health-related social needs (HRSN) at every patient encounter, using validated tools and generating warm handoffs to Community Health Teams for navigation and resource connection. This is the most systematic SDOH screening infrastructure of any state primary care program.
- **Health Equity Capacity Building Program:** Vermont Department of Health grants to community-based organizations working on health disparities, funded through CDC. Organizations working on equity with LGBTQ+ populations, justice-involved individuals, rural communities, and Indigenous communities have received grants through this program.
- **Health Equity Advisory Commission:** A standing body advising state government on health equity policy. Act 68 requires the Health Care Delivery Advisory Committee to consult with the Commission, embedding equity oversight in the transformation governance structure.
- **CCBHC access policy:** Vermont's CCBHCs are explicitly required to serve all patients regardless of age, diagnosis, insurance status, or geographic location — the most equity-protective access policy for behavioral health services.
- **Act 167 and Act 68 equity requirements:** Both acts explicitly include reducing health inequities among their statutory goals and required outcome measures.

What Must Be Built

Figure 12.3 — Vermont equity gaps and required investments. Sources: Vermont Department of Health Health Equity Data Report (January 2025); Oliver Wyman Act 167 Report; AHS Transformation Reports; KFF GLP-1 coverage analysis.

Section 6: Equity and the Six-Pillar Framework — How Every Pillar Has an Equity Dimension

A common mistake in equity strategy is treating equity as a separate program — a set of equity-specific interventions that operate alongside the main healthcare transformation agenda. This approach reliably produces equity programs that are underfunded, deprioritized when budgets are tight, and disconnected from the structural changes that actually determine health outcomes.

The correct approach treats equity as a cross-cutting dimension of every pillar — a lens through which every policy decision, payment design, technology investment, clinical model, and operational change is evaluated for its equity implications. The following framework applies the equity lens to each of the six pillars:

Figure 12.4 — Equity as a cross-cutting dimension of all six pillars. Sources: Act 67 of 2025; Oliver Wyman Act 167 Report; HTR six-pillar framework.

Section 7: Vermont's Equity Accountability Framework

Vermont's Statewide Strategic Plan must contain a specific equity accountability framework — a set of measurable indicators, geographic stratifications, and intervention commitments that make the equity goal of Act 167 and Act 68 operational rather than aspirational.

The following framework represents the minimum equity accountability structure the Strategic Plan should establish:

Figure 12.5 — Vermont equity accountability framework: indicators, stratifications, and targets. Sources: Act 167; Act 68; VDH Health Equity Data Report (January 2025); AHS Transformation Reports.

Two principles must govern how Vermont uses this accountability frame-

work. First, equity indicators must be disaggregated — not presented as averages. An average primary care access rate that is improving while the BIPOC rate is stagnant is not equity progress; it is equity masking. The Statewide Strategic Plan’s equity section must show performance by population segment, not just system-wide averages.

Second, equity indicators must be connected to interventions. A disparity measure without a corresponding intervention commitment is a measurement of failure, not a strategy for success. Each equity indicator in the Strategic Plan should be paired with the specific programs — CCBHC expansion, CHW deployment, RHT broadband investment, BALANCE participation decision — that are designed to move it, with the responsible agency and the expected timeline for observable improvement.

Key Concepts Introduced or Expanded in This Chapter

Sources: Vermont Department of Health Health Equity Data Report, Annual Report to the Vermont Legislature (January 17, 2025); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (2024); Vermont Rural Health Transformation Program Application (November 2025); KFF, Medicaid Coverage of and Spending on GLP-1s (January 2026); KFF, What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid (2026); CMS, BALANCE Model announcement (December 23, 2025); CMS, Medicare GLP-1 Bridge demonstration (March 2026); Health Affairs, Reimagining Rural Health Equity (June 2024); Vermont Public, Vermont is trying to survey underrepresented communities about their health (April 2025); Act 68 of 2025; Act 167 of 2022; AHS November 2025 Health Care System Transformation Report.

Chapter 13: The Equity Pillar in Practice — HEDIS Equity Measurement, HEROI, and the Science of Closing Disparities

The Analytical Toolkit for Health Equity Work: Stratified HEDIS Measurement, the HEROI Composite Score, Root Cause Analytics, Intervention Design by Disparity Type, and Vermont's Population Health Equity Architecture

From Identifying Disparities to Eliminating Them

Chapter 12 established the Vermont equity landscape — the geographic disparities that concentrate in the Northeast Kingdom, the racial and demographic disparities in coverage and access, the SDOH burden that compounds both, and the GLP-1 access crisis that represents the most consequential new equity issue in American healthcare. This chapter develops the analytical and operational toolkit for actually closing those gaps.

Health equity work that stops at measurement — producing disparity reports that sit on shelves while the disparities they document persist — has

characterized most state-level equity efforts for the past two decades. The distinction between an equity report and an equity program is whether the measurement is connected to intervention, whether interventions are designed to address the specific root cause of each disparity, and whether there is an accountability structure that closes the loop between measurement, intervention, and outcome. Vermont's transformation mandate, with its explicit equity goals embedded in Acts 167 and 68 accountability frameworks, creates the institutional conditions for equity work that closes gaps rather than counting them.

Section 1: Stratified HEDIS Measurement — The Foundation of Equity Analytics

HEDIS — the Healthcare Effectiveness Data and Information Set — is the most widely used quality measurement system in American healthcare. But HEDIS in its standard form measures average performance across a plan or provider population. Average performance improvement can mask widening disparities when gains are concentrated in easier-to-reach, already-better-served populations. Equity requires HEDIS to be stratified — measured separately for subpopulations defined by race/ethnicity, income, geography, language, disability, and other equity-relevant dimensions.

The NCQA HEDIS Stratification Framework

The National Committee for Quality Assurance (NCQA) has developed HEDIS stratification standards that specify how plans and providers should measure and report quality performance by race, ethnicity, and other demographic dimensions. The HTR Health Equity Studio implements the NCQA stratification methodology against VHCURES and Blueprint data for Vermont providers. The following framework organizes the HEDIS measures most relevant to Vermont's equity priorities:

Figure 13.1 — Vermont HEDIS equity stratification framework. Sources: NCQA HEDIS Technical Specifications; AHS Health Equity Data Report (January 2025); Blueprint for Health data.

The Data Collection Challenge — Getting Demographic Data Right

Stratified HEDIS measurement requires race/ethnicity and other demographic data at the individual patient level. This is where Vermont's equity measurement infrastructure has a specific gap: race/ethnicity data in VHCURES is incomplete. Claims data from commercial insurers typically lacks race/ethnicity fields; Medicare added race/ethnicity data collection more recently; Medicaid data has the most complete demographic information.

Vermont has two approaches to closing the race/ethnicity data gap: direct collection at the point of care (NCQA standards require plans and PCMH practices to collect and report patient demographic data) and indirect imputation using validated geocoding methods (the RAND-BISG method imputes race/ethnicity probability from last name and census block characteristics). Vermont's Blueprint for Health practices and FQHCs are the primary settings for direct demographic data collection; the coverage of this data for the broader Vermont population depends on VHCURES reporting requirements being strengthened.

Section 2: The HEROI Framework — A Composite Equity Score

The Health Equity Return on Investment (HEROI) framework is the HTR Health Equity Studio's proprietary approach to equity performance measurement for healthcare organizations operating in value-based care environments. HEROI addresses a specific problem in equity measurement: disparities exist across multiple dimensions simultaneously, and an organization that closes one disparity while widening another may show no improvement on any individual metric while its overall equity performance has not improved.

HEROI Components

The HEROI composite score is calculated across five equity dimensions, each weighted by its relative health impact and the strength of evidence for interventions that can address it:

- Access equity score (25% weight): Measures the gap in primary care, specialty care, and emergency care utilization between advantaged and disadvantaged subpopulations, stratified by race/ethnicity, income, and geography. Vermont focus: Northeast Kingdom vs. Burlington primary care access; BIPOC primary care provider relationship rates.
- Quality equity score (25% weight): Measures the gap in HEDIS quality measure performance between advantaged and disadvantaged subpopulations for the top 10 chronic disease management measures. Vermont focus: diabetes control, hypertension control, and preventive care rates stratified by income and race/ethnicity.
- Outcome equity score (25% weight): Measures the gap in utilization outcomes — avoidable hospitalizations, ED visits, 30-day readmissions — between subpopulations. Vermont focus: mental health and SUD crisis ED presentation rates by county; readmission rates by Medicaid vs. commercial payer status.

- SDOH burden score (15% weight): Measures the prevalence of health-related social needs in the organization's attributed population, stratified by demographics, and the completeness of SDOH screening and navigation. Vermont focus: HRSN screening completion rates by practice location; community resource referral completion rates.
- Trust and engagement score (10% weight): Measures patient experience across demographic subgroups using CAHPS survey data stratified by race/ethnicity and income. Vermont focus: CAHPS disparity scores for BIPOC Vermonters; patient-reported discrimination experiences in clinical settings.

A HEROI score of 80+ indicates an organization achieving equity across all five dimensions. A score of 60-80 indicates material disparities in one or more dimensions requiring targeted intervention. Below 60 indicates pervasive disparities that require comprehensive equity program redesign. Vermont's statewide HEROI score — calculated from VHCURES and Blueprint data — has not been formally computed; the analytics vendor procurement underway will enable this calculation.

Section 3: Root Cause Analytics — Matching Interventions to Causes

The most common failure in health equity intervention design is treating all disparities as if they have the same cause. A disparity in diabetes control rates between white and Black Vermont adults has a different root cause structure than a disparity in primary care access between Chittenden County and Essex County residents — and requires a different intervention. The five-category root cause taxonomy from Chapter 12 (geographic access, social needs, clinical practice variation, coverage barriers, trust and engagement) provides the structure for matching interventions to causes.

The Disparity Decomposition Method

The HTR disparity decomposition method breaks a measured disparity into its constituent causes by applying statistical decomposition techniques to population-level data. The method answers the question: of the observed gap in outcome X between group A and group B, how much is attributable to differences in coverage rates, differences in primary care access, differences in clinical quality, and differences in SDOH burden?

For Vermont's diabetes control disparity — the 22% of Blueprint-attributed diabetic patients with HbA1c not under control, with statistically higher rates among Medicaid patients and rural patients — the decomposition analysis would estimate what share of the disparity is attributable to medication adherence failures (addressable through pharmacy access and MedReconciliation programs), care management gap (addressable through CoCM integration), SDOH burden (addressable through CHT navigation), and primary care access limitations (addressable through Blueprint expansion).

The practical value of decomposition is not statistical precision — it is intervention targeting. An equity program that invests in care management for a disparity driven primarily by medication access barriers will produce minimal results. An equity program that correctly identifies medication access as the primary driver and invests in pharmacy co-location, medication assistance programs, and automated refill reminders will close the gap efficiently.

The Five-Step Disparity Closure Process

The HTR Health Equity Studio implements the following five-step disparity closure process for client organizations:

Figure 13.2 — Five-step disparity closure process with Vermont applications.
Source: HTR Health Equity Studio methodology.

Section 4: Equity in VBC Contracting — Protecting Against Unintended Consequences

Value-based care contracts have a well-documented potential to worsen health equity if not designed with explicit equity protections. The mechanisms are several: global budgets or capitation rates that do not adequately adjust for social risk may incentivize providers to avoid or under-serve high-SDOH populations; shared savings contracts that reward utilization reduction may reward avoiding care for complex patients; quality measure performance may be harder for organizations serving disadvantaged populations even when they are delivering excellent care relative to what their population can access.

Social Risk Adjustment — The Equity Design Requirement for VBC Contracts

Social risk adjustment — adjusting VBC benchmarks and performance targets to account for the social risk burden of an organization’s attributed population — is the technical mechanism for protecting equity in payment design. An organization serving a population with high housing instability, food insecurity, and transportation barriers faces higher care management costs and lower quality measure performance potential than an organization serving a high-income, socially stable population. A shared savings contract that ignores this difference rewards the wrong organizations.

CMS has incorporated social risk adjustment into ACO REACH through Comprehensive and Professional Direct Contracting model designs that include social risk stratification. Vermont’s AHEAD global budget design must incorporate social risk adjustment at the hospital level — ensuring that hospitals serving the Northeast Kingdom’s high-SDOH populations are not penalized relative to Burlington-area hospitals with more favorable social risk profiles. This is an active design question in Vermont’s AHEAD implementation that AHS and GMCB must resolve explicitly.

The VBC Equity Audit

Every VBC contract should undergo an equity audit before and after signing, examining whether the contract design creates incentives that could worsen disparities. The HTR VBC Equity Audit checklist covers eight categories:

- Attribution equity: Does the attribution methodology systematically exclude high-SDOH patients who lack a qualifying primary care visit?
- Social risk adjustment: Is the benchmark adjusted for the social risk profile of the attributed population?
- Quality measure equity: Are quality measures stratified by race/ethnicity and income, with performance reported and rewarded based on equity improvement, not just average improvement?
- SDOH investment incentives: Does the contract provide financial credit for investments in SDOH programs that reduce long-term utilization?
- Care management intensity: Are care management resources allocated proportionate to SDOH burden and disparity level, not just clinical risk?
- Beneficiary protections: Are there protections against provider avoidance of high-complexity, high-SDOH patients?
- Community benefit requirements: For global budgets, are there explicit requirements for community health investment, including SDOH programs?
- Equity performance reporting: Is equity performance reported publicly, with minimum standards below which the organization cannot be considered performing?

Section 5: Vermont's Population Health Equity Analytics Architecture

The analytical infrastructure for equity measurement in Vermont is being built through the convergence of three systems: VHCURES (claims-based population data), the Blueprint clinical registry (quality and demographic data for PCMH-attributed patients), and the VDH health equity data systems (BRFSS survey data, vital statistics, county-level environmental data). None of these systems alone is adequate for comprehensive equity measurement; their integration — the one health record per person mandate — is the target state.

In the interim, Vermont's equity analytics strategy must be pragmatic: use the best available data for each equity dimension, be explicit about gaps, and make decisions based on the evidence available while investing in improving the evidence base. The following architecture organizes Vermont's current equity analytics capabilities and the priority investments needed to close gaps:

Figure 13.3 — Vermont equity analytics: current data sources by equity dimension. Sources: GMCB; VDH; Blueprint for Health; DVHA; AHS.

Key Concepts

Sources: NCQA HEDIS Equity Technical Specifications; HTR Health Equity Studio methodology; Vermont Department of Health Health Equity Data Report (January 2025); Act 167 of 2022; Act 68 of 2025; Blueprint for Health Annual Report (2024); KFF; VHCURES documentation; AHS Transformation Reports (2025).

Chapter 14: The Technology Pillar — Data Infrastructure for a Transformed Health System

VHCURES, VITL, the Health Information Exchange, the Statewide EHR Question, FHIR Interoperability, AI Scribes, Remote Patient Monitoring, Telehealth, and the Clinically Integrated Network Vermont Is Building

Sources: GMCB VHCURES documentation; Vermont HIE Strategic Plan (2024 Update); Act 62 of 2025 (HIE governance transfer to DVHA); Oliver Wyman Act 167 Recommendations; AHS January 2025 Legislative Presentation; Vermont RHT Program Application and Activity Details (November–December 2025); Chartis 2026 State of Rural Health; JAMA Network Open ambient scribe study (2024); CMS FHIR Interoperability Rule; Vermont Blueprint for Health HIE data.

The Technology Pillar: Why Data Infrastructure Is the Foundation of Everything Else

There is a precise sense in which the Technology pillar is the precondition for every other pillar. Reference-based pricing requires knowing what each hospital actually charges for each service. Global budgets require attributing patients to hospitals and tracking total cost of care across the year. Clinical quality improvement requires measuring the gap between current performance and evidence-based targets. Health equity requires stratifying outcomes by race, income, and geography. Operational transformation requires modeling the financial and access impact of structural changes before implementing them.

All of these functions require data — specifically, data that is timely, complete, accurate, linked across clinical and claims sources, accessible to the organizations that need it, and governed in ways that protect patient privacy while enabling population-level analysis. Vermont's current data infrastructure — anchored by VHCURES (the all-payer claims database), VITL (the health information exchange), and the Blueprint clinical data registry — is sophisticated by national standards for a state of its size. It is also, in its current form, insufficient for the management demands of a system operating under global budgets and a mandatory Statewide Strategic Plan accountability framework.

This chapter develops Vermont's technology pillar comprehensively: what the current infrastructure does and does not do, where the critical gaps are, what the RHT Program is funding to close them, and what the national technology landscape — FHIR interoperability, ambient clinical intelligence, remote patient monitoring, AI diagnostic tools — means for Vermont's specific circumstances. It concludes with a technology architecture framework that Vermont's Strategic Plan should adopt and that any state-level transformation can use as a reference model.

Section 1: VHCURES — Vermont’s All-Payer Claims Database

The Vermont Health Care Uniform Reporting and Evaluation System (VHCURES) is Vermont’s all-payer claims database, managed by the Green Mountain Care Board with Onpoint Health Data as the data aggregation vendor. VHCURES has been operational in its current form since 2013 (when GMCB assumed responsibility from the Department of Financial Regulation) and is the primary analytical foundation for Vermont’s healthcare regulatory functions.

What VHCURES Contains and What It Can Do

VHCURES is a substantial data asset that gives Vermont analytical capabilities most states lack. Understanding both its capabilities and its limitations is essential for anyone designing Vermont’s technology infrastructure.

Figure 14.1 — VHCURES data coverage summary. Source: GMCB VHCURES documentation.

VHCURES enables Vermont to conduct analyses that directly support the transformation agenda: population-level total cost of care measurement (the GMCB publishes all-payer TCOC dashboards derived from VHCURES); potentially avoidable utilization analysis (the 32.3% avoidable ED visit rate in AHS’s transformation reports is calculated from VHCURES via VUHDDS); hospital price transparency analysis (the GMCB’s 2026 RBP report used VHCURES-derived 2024 claims data to show hospital prices ranging from 279% to 697% of Medicare for outpatient services); and prescription drug affordability analysis (GMCB’s 2025 report on drug pricing used VHCURES to model the impact of IRA drug negotiations on Vermont spending). This is a powerful analytical platform that has produced some of the most credible and consequential health policy analysis in recent Vermont history.

VHCURES Limitations — Where the Gaps Are

VHCURES has four structural limitations that matter directly for Vermont's transformation management needs. Oliver Wyman identified the data infrastructure as one of the barriers to coordinated transformation, and the specific limitations below explain why.

Figure 14.2 — VHCURES structural limitations and required improvements. Sources: GMCB VHCURES data documentation; Oliver Wyman Act 167 Report; AHS Transformation Reports; Act 68 of 2025.

VHCURES Perception vs. Reality — Oliver Wyman's Finding

Oliver Wyman's perception-versus-reality analysis identified Vermont's data infrastructure as one of the areas where the gap between apparent capability and actual capability is most consequential. The relevant finding:

Perception: Vermont has a State Health Information Exchange (VITL).

Reality: Participation in VITL is voluntary and many community-based providers are not included. VITL lacks pharmacy claims data and is not viewed as 'user friendly' by providers.

This matters because Vermont's transformation strategy depends on care coordination between hospitals and community providers — and care coordination requires data sharing. A care coordinator trying to manage a patient transitioning from a hospital inpatient stay to a community mental health program cannot access the clinical data they need if the mental health agency is not connected to VITL. A primary care practice trying to close chronic disease management gaps cannot see the specialty care their patient received at a different facility if that facility's data is not in the HIE.

Vermont's technology investments — the VHCURES-VHIE integration mandate, the RHT Program interoperability grants, the AHEAD data sharing requirements — are all aimed at closing this gap between VITL's nominal existence and the functional interoperability Vermont's care coordination model requires.

Section 2: The Vermont HIE — VITL, the Governance Shift, and the Integration Imperative

Vermont's Health Information Exchange (VHIE) is operated by Vermont Information Technology Leaders (VITL), an independent nonprofit designated by statute as the exclusive operator of the statewide HIE network. Every hospital in Vermont contributes records to the VHIE. The VHIE operates a provider portal (VITLAccess) and a query service that allows providers to access patient records from other facilities. Vermont has been building this infrastructure since 2009, when VITL was designated as the Regional Extension Center to help primary care providers adopt electronic health records.

The 2025 Governance Shift: Act 62 Transfers HIE Authority to DVHA

Effective July 1, 2025, Act 62 of 2025 made a significant governance change: responsibility for Vermont's statewide Health Information Technology Plan was transferred from the Green Mountain Care Board to the Department of Vermont Health Access (DVHA). Prior to this, GMCB had authority to review and approve the HIE Strategic Plan, VHIE Connectivity Criteria, and VITL's budget. Under Act 62, DVHA now coordinates Vermont's statewide HIT Plan in consultation with the HIE Steering Committee, with annual revisions submitted by November 1 and comprehensive updates every five years.

The governance rationale for this shift is significant: DVHA has direct operational relationships with healthcare providers through its Medicaid contracts, Blueprint for Health administration, and AHEAD Model management. Moving HIT plan responsibility to DVHA aligns the technology strategy with the healthcare delivery transformation authority. It also means that Vermont's AHEAD implementation — which requires robust data exchange for population health management and global budget monitoring — is now coordinated by the same agency managing the data infrastructure.

The Integration Mandate: One Health Record Per Person

The Vermont legislature gave the HIE Steering Committee an explicit statutory mandate that defines the technology pillar's central ambition for the next five years: create one health record for each person that integrates claims data, clinical data, mental health and substance use disorder services data, and social determinants of health data. This mandate is documented in state law and in AHS's own letters to the legislature. It is the most ambitious data integration goal any state has formally committed to, and its achievement would give Vermont a population health management capability that no other state currently possesses.

The practical path to this integration has three components that must be

executed in sequence:

Figure 14.3 — Vermont’s three-step path to integrated health records. Sources: Vermont HIE Steering Committee statutory mandate; AHS January 2025 Legislative Presentation; DVHA HIE governance documents.

The Statewide EHR Question — Vermont’s Active Feasibility Assessment

Oliver Wyman’s community meetings generated a clear finding about EHR fragmentation: providers cannot efficiently coordinate care across facilities when different hospitals run different EHR systems that do not share data effectively. Vermont’s 14 hospitals currently use four different EHR vendors — Epic (dominant at UVMHC), Oracle Health (formerly Cerner), TruBridge (formerly CPSI, used by smaller community hospitals), and Meditech — plus various additional software for laboratory management, pharmacy, and other functions.

AHS has commissioned a feasibility assessment to evaluate the cost-benefit of implementing a statewide electronic health record — a single EHR platform that all Vermont hospitals and primary care practices would use, enabling seamless data exchange as a byproduct of shared infrastructure rather than through complex interface work. Vermont’s HIE Strategic Plan describes the assessment as evaluating ‘what a statewide EHR would look like in line with the Act 167 Report.’

Figure 14.4 — Vermont statewide EHR: case for and against. The feasibility assessment underway will weigh these factors against Vermont’s specific cost, disruption tolerance, and FHIR interoperability capacity.

The feasibility assessment’s most important analytical question is whether FHIR-based interoperability — the national standard for health data exchange that is rapidly maturing — can deliver the data sharing Vermont

needs without the cost and disruption of a statewide EHR migration. If FHIR can deliver real-time, bi-directional data exchange across Vermont's four EHR platforms at an acceptable implementation cost, the case for a single statewide EHR weakens significantly. Section 4 develops the FHIR picture. AHS's feasibility assessment should produce a clear answer on this question by 2026-2027 — in time to inform the 2028 Strategic Plan.

Section 3: The Vermont Clinically Integrated Network — A New Technology Institution

Vermont's Rural Health Transformation Program application includes one of the most significant technology investments in the entire \$195M award: the creation of a Clinically Integrated Network (CIN) linking Vermont's hospitals and community providers into a coordinated clinical and data-sharing infrastructure. Vermont is one of four states (along with Arkansas, Nevada, and Oregon) that explicitly included CIN creation in their RHT applications, reflecting a national trend toward CINs as the organizational vehicle for rural health system integration.

What a Clinically Integrated Network Does

A CIN is a formal network of healthcare providers — hospitals, primary care practices, specialists, post-acute providers — that share clinical protocols, performance data, and care coordination infrastructure while maintaining independent ownership and governance. CINs occupy the space between full health system merger (which requires consolidation of ownership) and loose referral relationships (which have no formal data sharing or accountability). For Vermont's 14 independent, non-profit hospitals — which cannot and should not be merged into a single system — a CIN provides the coordination benefits of integration without the legal and political costs of ownership consolidation.

What the Vermont CIN Will Provide

Vermont's RHT Program application describes the Shared Services initiative — which includes the CIN infrastructure — as providing five specific functions for participating hospitals and providers:

- **Analytics support:** A shared analytics platform available to all hospitals and non-hospital providers for transformation and regionalization planning — identifying areas most in need, modeling the impact of service line changes, and tracking quality and cost metrics. This is the analytics vendor that AHS and GMCB are jointly procuring.
- **Shared clinical protocols:** Standardized clinical pathways for high-volume conditions — sepsis, heart failure, COPD, mental health crisis — that all participating providers adopt, enabling consistent quality regardless of which facility a patient uses.
- **Group purchasing:** Coordinated supply chain and group purchasing for medical supplies, equipment, and technology — capturing the scale economies that individual small hospitals cannot achieve independently. Oliver Wyman identified this as one of the highest-leverage near-term cost reduction opportunities.
- **Administrative shared services:** Shared infrastructure for billing, coding, credentialing, HR, and IT support — reducing the administrative overhead that drives Vermont's \$1,303 per-discharge administrative cost premium over national benchmarks.
- **Referral network formalization:** Structured referral agreements between RSC hospitals and Tier 2/3 facilities for each COE specialty — ensuring that the regionalization blueprint operates as a reliable clinical network rather than a set of informal relationships.

The CIN also serves as the governance vehicle for the technology investments Vermont's transformation requires. Individual small hospitals cannot justify the capital investment in AI scribe technology, remote patient

monitoring equipment, telehealth infrastructure, or advanced analytics platforms. A CIN can negotiate enterprise-wide technology contracts that make these investments affordable for the entire network, deploy shared technology infrastructure that individual providers access as a service, and establish common data standards that make the integrated health record vision achievable.

Section 4: FHIR Interoperability — The National Standard and Vermont’s Compliance Path

FHIR (Fast Healthcare Interoperability Resources) is the HL7 standard for healthcare data exchange that CMS mandated for payers and providers beginning in 2021 and 2022. FHIR defines a common data format and API structure that allows different health IT systems to exchange information without custom interfaces. It is the closest thing to a universal language for healthcare data that the industry has adopted, and its maturation over the past five years has fundamentally changed what is technically possible in health data exchange.

What FHIR Enables That Vermont Currently Cannot Do

Vermont’s current health data infrastructure predates widespread FHIR adoption and was built around older data exchange standards. The operational implications are concrete:

- Patient matching across facilities: Without FHIR-based master patient index integration, Vermont still lacks reliable patient matching across

all providers. Community meeting participants described needing to contact 31 separate EMS agencies for patient transfers — a fragmentation problem that a FHIR-enabled patient matching system would substantially reduce.

- Real-time care gap identification: FHIR-enabled EHR systems can query a patient’s record at the point of care, identifying care gaps (missing preventive services, chronic disease management gaps) in real time during the clinical encounter. Vermont’s current Blueprint data profile process delivers care gap information to practices, but with significant lag. FHIR enables the same information at the point of care.
- Attribution accuracy for global budgets: AHEAD’s hospital global budgets require accurate attribution of patients to hospitals for Medicare FFS. FHIR-based data exchange enables more precise attribution by tracking care delivery across providers in near-real-time, reducing the attribution disputes that complicate global budget performance measurement.
- Population health management at scale: Vermont’s Blueprint CHTs currently manage populations using a combination of claims data, clinical registry data, and provider-reported information. FHIR-based population health platforms can aggregate this data automatically, maintaining continuous patient registries that trigger care coordinator outreach when patients are overdue for care or when clinical indicators suggest increased risk.

Vermont’s FHIR Compliance Status

Vermont’s larger hospitals — particularly UVMMC with its Epic EHR — are FHIR-compliant for the CMS-mandated payer-to-patient and payer-to-payer API requirements. The critical gap is at smaller community hospitals (on TruBridge and Meditech platforms) and at community-based providers (mental health agencies, skilled nursing facilities, home health agencies) that lack FHIR-capable systems entirely.

Vermont’s RHT Program IT advance investments address this gap directly.

The program includes grants for technology upgrades at smaller and more rural facilities that cannot afford FHIR-capable EHR migrations independently. The CIN infrastructure also supports FHIR compliance by providing shared technical assistance, vendor negotiations for favorable pricing on FHIR-capable platforms, and shared technical staff who can support implementation at facilities that lack internal IT capacity.

*Figure 14.5 — Vermont FHIR compliance landscape (estimated as of 2026).
Sources: AHS Legislative Presentation January 2025; Vermont RHT Program Application; GMCB HIE connectivity criteria.*

Section 5: AI and Digital Health Technologies — Vermont's Targeted Investment Strategy

Vermont's RHT Program application is notable among the 47 state applications submitted in late 2025 for its specificity about technology investments. Rather than generic technology grants, Vermont's technology investment strategy is structured around five specific technology categories, each directly linked to an identified gap in Vermont's current system. This is the correct approach to one-time capital investment in technology: identify the specific operational problems, fund the specific technologies that solve them, and measure whether the problems are resolved.

AI Scribe Technology: Addressing the Documentation Burden Crisis

Vermont's RHT Program application explicitly includes grants for providers to purchase AI scribe technology, noting that smaller and more rural and independent practices are struggling with access to the same technology that larger practices can afford. The application cites that AI scribes automate an average of over two hours of daily administrative work by transcribing and processing clinician-patient conversations.

This is a direct response to a Vermont workforce problem: physician and clinician burnout driven by documentation burden is a primary contributor to provider attrition and is reducing Vermont's effective primary care capacity. A provider spending two hours daily on documentation has approximately 25% less time for patient care than one whose documentation is automated. At Vermont's scale — where the primary care FTE shortage is already projected at 370 by 2030 — recovering two hours per provider per day through AI documentation support is equivalent to adding significant additional clinical capacity without hiring new providers.

The evidence base for AI scribe technology is strong and growing. A 2024 quality improvement study published in JAMA Network Open found that ambient scribe tools were associated with improved documentation efficiency and reduced documentation burden. Clinicians used the technology spent less time writing notes during visits and reported less after-hours charting. Community providers at national rural health conferences have independently identified ambient AI as the single technology investment most likely to address workforce shortages while supporting billing processes and clinical decision-making simultaneously.

Remote Patient Monitoring: Managing Chronic Disease Outside the Hospital

Vermont's RHT Program application includes grants for remote patient monitoring (RPM) equipment for use in home health, primary care/specialty, and community paramedicine settings. RPM enables earlier interventions and ongoing monitoring for chronic conditions — congestive heart failure, COPD, diabetes, hypertension — in the home to prevent hospital admissions or readmissions.

The economics of RPM under global budgets are compelling. Under fee-for-service, RPM generates separate billable encounters and competes with in-person visits for reimbursement. Under global budgets, RPM is a cost-reduction tool: every hospital admission or readmission prevented by early RPM-triggered intervention saves revenue under the fixed budget.

Vermont's aging population — with high rates of CHF, COPD, and diabetes among the 65+ cohort — is precisely the population where RPM produces the largest clinical and financial benefit.

Vermont's rural geography amplifies RPM's value. A patient in the Northeast Kingdom managing heart failure who needs twice-weekly monitoring faces a two-hour round trip to a clinic. RPM provides the same monitoring without the trip — better for the patient (who avoids travel costs and time), better for the provider (who can monitor more patients with existing staff), and better for the system (which reduces transportation-related access barriers that drive missed appointments and deteriorating chronic disease management).

Telehealth: Specialty Access for Rural Vermont

Vermont's RHT Program application includes specific telehealth investments to expand specialty care access in rural hospitals — allowing specialists at RSC facilities to provide consultation to patients at Tier 2 and Tier 3 facilities without requiring patient transfer. This is the technology enabler for Oliver Wyman's regionalization blueprint: rather than requiring every COE specialty to maintain physical presence at every community hospital, telehealth enables COE specialists to provide consultation across the network while patients remain near their homes.

Vermont's broadband landscape is a constraint: the RHT application notes that 80% of rural Vermonters have broadband access, compared to 85% in non-rural areas. The gap, while relatively small by national rural standards, means that home-based telehealth cannot reach all Vermont patients. Vermont's proposed broadband expansion (Oliver Wyman's third priority legislative change recommendation) is the infrastructure prerequisite for telehealth reaching the communities that need it most. The RHT Program's explicit inclusion of broadband infrastructure as an allowable use reflects this reality.

Vermont's telehealth strategy from the RHT application has four specific applications: specialty consultations (neurology, psychiatry, oncology) at

rural hospitals; tele-ICU support for rural hospitals managing critical patients before transfer; tele-behavioral health for community mental health and CCBHC services; and community paramedicine — enabling EMTs and paramedics to provide extended care in patients' homes with real-time physician supervision via telehealth, reducing 911 calls and ED visits for non-emergency conditions.

Diagnostic AI: The Emerging Standard of Care

The book's Part V discussion of diagnostic AI maturation — FDA-cleared AI diagnostic devices for radiology, pathology, ophthalmology, and cardiology becoming standard of care in high-volume settings — applies directly to Vermont's context with one important distinction: Vermont's smaller rural hospitals may be the settings that benefit most from diagnostic AI, precisely because they lack the specialist volume to maintain expert human diagnostic capacity in every specialty.

A rural Vermont hospital with one radiologist who handles all imaging across multiple modalities cannot maintain the same expert pattern recognition as a high-volume academic center reading 50 chest CTs daily. AI-assisted radiology reading — which has demonstrated accuracy equal to or exceeding specialist readers for specific modalities like chest X-ray, mammography, and retinal imaging — can partially close this quality gap without requiring additional specialist recruitment. This is not a replacement for radiologists; it is a tool that makes existing radiologists more accurate and more efficient, particularly in the high-volume, lower-complexity reads that constitute the majority of rural hospital imaging.

Vermont's RHT IT advance funding can support diagnostic AI implementation at smaller hospitals that cannot fund these investments independently. The CIN's shared service model is an appropriate delivery mechanism: a network-wide diagnostic AI contract that all participating hospitals access, with shared quality monitoring and governance, would provide far better economics and oversight than 14 individual hospital technology decisions.

Price Transparency Technology

Vermont's RHT Program application includes a price transparency and insurance competition initiative — building the technology infrastructure to make hospital pricing legible to patients, payers, and regulators. GMCB was already developing a hospital price transparency dashboard as of February 2026 (first release forthcoming), providing the public with access to 2024 claims data showing hospital prices relative to Medicare. The RHT investment extends this infrastructure to enable patient-facing price comparison tools that allow Vermonters to see what a procedure costs at different facilities before choosing where to receive care.

Price transparency technology is a complement to reference-based pricing, not a substitute for it. RBP directly limits what hospitals can charge; price transparency helps patients and payers identify and act on pricing differences within the RBP framework. The combination — mandatory price caps plus transparent pricing information — creates the closest approximation to a functioning market for hospital services that any regulatory framework has achieved.

Section 6: AI Governance — Vermont's Responsibility Before the Risk Arrives

The book's Part V discussion of AI governance as a clinical requirement — moving from a quality initiative to a regulatory requirement as FDA, ONC, and CMS develop AI governance frameworks — applies to Vermont with specific urgency. Vermont is making substantial AI investments through the RHT Program: AI scribe grants, diagnostic AI potential, RPM algorithms. Each of these involves clinical AI that has the potential to produce biased outputs, performance degradation over time, or harms to specific patient populations if not properly monitored.

Vermont does not currently have a statewide AI governance framework for clinical AI. Individual hospital systems have varying levels of AI

governance maturity — UVMMC, with its Epic deployment and academic medical center sophistication, is likely more advanced than a smaller community hospital purchasing AI scribe software for the first time. The CIN infrastructure Vermont is building through the RHT Program creates the opportunity to establish a shared AI governance framework that applies consistently across all Vermont providers.

Vermont AI Governance Framework: Minimum Requirements

Drawing on the HTR AI Governance Checklist and Vermont's specific clinical AI deployment context, Vermont's Statewide Strategic Plan should establish minimum AI governance requirements for clinical AI deployed in Vermont healthcare settings. At minimum, these should include:

- **Pre-deployment bias testing:** Any AI model used for clinical decision support, diagnostic assistance, or care gap identification must be tested for differential performance across race/ethnicity, age, sex, income, and geographic subgroups before deployment. Vermont's equity goals require that AI tools do not introduce or amplify health disparities.
- **Performance monitoring:** Deployed AI tools must have ongoing performance monitoring with predefined thresholds that trigger review or suspension. An AI model trained on national data may perform differently in Vermont's specific patient population — particularly in rural areas and among dual-eligible patients with complex social needs.
- **Transparency requirements:** Patients must be informed when AI tools are involved in their care. Clinicians must understand the basis and limitations of AI outputs. 'Black box' AI with no explainability is inappropriate for clinical settings regardless of its overall accuracy.
- **Vendor accountability:** Technology vendors providing clinical AI to Vermont providers must agree to performance monitoring, bias testing disclosure, and incident reporting. Contract terms must include performance standards and remedies for underperformance.
- **Equity monitoring:** AI performance must be specifically monitored for Vermont's highest-risk equity populations — Northeast Kingdom rural

residents, Medicaid patients, elderly patients with complex needs, and patients whose primary language is not English.

Vermont's RHT Program AI scribe grants should include AI governance requirements as a condition of funding. The CIN's shared services function is the natural organizational home for a shared AI governance committee that monitors all AI deployments across Vermont's hospital network.

Section 7: Vermont's Target Technology Architecture — A Framework for the Strategic Plan

Vermont's Statewide Health Care Delivery Strategic Plan must specify a technology architecture — the integrated set of data systems, exchange standards, governance structures, and analytical platforms that enable the transformed delivery system to be managed, monitored, and improved. The following architecture organizes Vermont's technology investments across five layers, from foundational infrastructure to advanced analytical applications.

Figure 14.6 — Vermont target technology architecture: five-layer model. Sources: GMCB VHCURES documentation; Vermont HIE Strategic Plan; Oliver Wyman Act 167 Report; Vermont RHT Program Application; Act 62 and Act 68 of 2025.

Two observations about this architecture are important for Vermont's technology planning. First, layers 3 and above represent target states, not current states. Vermont's current technology infrastructure is solid at Layers 1 and 2, developing at Layer 4, and nascent at Layers 3 and 5. The RHT Program, AHEAD requirements, and the AHS-GMCB analytics vendor procurement are all investments in accelerating the build-out from Layer 1-2 completeness toward Layer 3-5 capability. The Statewide Strategic Plan's technology section should be explicit about which layers are complete, which are under development, and what the completion timeline is.

Second, the technology architecture has a governance counterpart that is equally important. Each layer requires governance: who owns the data, who can access it under what conditions, what privacy protections apply, how disputes are resolved, and how the infrastructure is funded sustainably. Vermont's technology investments will only produce their intended value if the governance layer — the policies, agreements, and oversight structures — keeps pace with the technical infrastructure. The DVHA-led HIE governance, the GMCB's data stewardship program, and the CIN's shared governance model are all components of this governance architecture.

Key Concepts Introduced or Expanded in This Chapter

Sources: GMCB VHCURES documentation (gmcbboard.vermont.gov/data-and-analytics); Vermont HIE Strategic Plan 2024 Update; Act 62 of 2025; Act 68 of 2025; Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts (January 31, 2025); Vermont Rural Health Transformation Program Application (November 3, 2025); Vermont RHT Program Activity Details (December 2025); Chartis 2026 State of Rural Health; JAMA Network Open ambient scribe quality improvement study (2024); National Consortium

of Telehealth Resource Centers AI guidance (2026); Vermont Health Information Exchange Data Governance mandate (healthdata.vermont.gov).

Chapter 15: The Technology Pillar in Practice — FHIR Implementation, AI Governance, and Clinical Decision Support

Implementing Interoperability in the Real World: FHIR API Deployment, Payer-to-Provider Data Exchange, the AI Clinical Governance Lifecycle, Clinical Decision Support Architecture, and Cybersecurity in Rural Health Infrastructure

From Architecture to Implementation

Chapter 14 established Vermont’s technology architecture — the five-layer infrastructure from VHCURES and VUHDDS through the HIE and FHIR exchange layer to population health analytics and AI applications. This chapter develops the implementation side: how organizations actually build FHIR-based data exchange, what an AI clinical governance program looks like in practice, how clinical decision support is designed and deployed for population health management, and what cybersecurity requirements apply to the distributed rural health infrastructure Vermont is building.

Section 1: FHIR Implementation — From Standard to Working System

The FHIR Implementation Reality

FHIR R4 is a standard, not a product. Its existence does not mean that any two FHIR-compliant systems can exchange data automatically. Like all standards, FHIR specifies a framework within which compliant systems must operate — but the specific data elements populated, the implementation patterns used, and the business rules applied vary substantially across implementations. A hospital's Epic FHIR implementation and a CAH's TruBridge FHIR implementation may both be 'FHIR R4 compliant' while exchanging data with entirely different content, triggering entirely different workflows, and requiring custom integration work to interoperate effectively.

Vermont's CIN technology strategy must account for this reality. The goal is not FHIR compliance as a checkbox — it is functional interoperability: the ability for a care coordinator at Northeastern Vermont Regional Hospital to see, in real time, that their patient's Medicaid claim for an ER visit at Dartmouth Hitchcock Medical Center in New Hampshire was just processed, enabling a care management follow-up call before the patient discharges. That outcome requires FHIR compliance plus patient matching plus event notifications plus care coordinator workflow integration. FHIR makes it theoretically possible; CIN implementation makes it practically achievable.

The Three FHIR Use Cases Vermont Needs Most

Vermont's technology investments should prioritize three FHIR use cases that deliver the highest value for the transformation agenda:

Figure 15.1 — Vermont's three priority FHIR use cases. Sources: CMS Interoperability and Patient Access Rule; ONC 21st Century Cures Rule; HTR

Technology pillar framework.

The Master Patient Index — Vermont’s Identity Management Challenge

Every FHIR use case that involves sharing records between organizations requires patient matching — reliably identifying that the ‘John Smith, DOB 1952-03-14’ at UVMMC is the same person as the ‘John Smith, DOB 1952-03-14’ at North Country Hospital and the ‘Jonathan Smith, DOB 1952-03-14’ (with a name variant) in the Medicaid enrollment file. Vermont’s VITL network provides a master patient index for VHIE-connected providers, but it does not cover all Vermont providers, does not include all out-of-state encounters, and does not have validated match rates documented publicly.

Vermont’s CIN implementation should include a dedicated master patient index as a shared service — a centralized patient identity management system that all CIN member facilities use to match patients across organizations. The investment cost is modest relative to the value: a reliable MPI is the prerequisite for virtually every care coordination use case the CIN is designed to enable.

Section 2: AI Clinical Governance — The Complete Framework

AI clinical governance is evolving from a best practice to a regulatory requirement. FDA’s AI/ML-based Software as Medical Device (SaMD) framework, ONC’s Health Data, Technology and Interoperability rule, and emerging CMS guidance on AI in Medicare Advantage all establish governance expectations for clinical AI. Vermont’s CIN AI governance framework, described in Chapter 14, must be operationally detailed enough to guide the specific governance decisions that CIN member facilities need to make when deploying AI scribe technology, RPM algorithms, diagnostic

AI, and population health analytics.

The AI Clinical Governance Lifecycle

The HTR AI Clinical Governance Checklist organizes governance requirements across six lifecycle stages:

Stage 1: Pre-Deployment Evaluation

Before any clinical AI tool is deployed, the organization must evaluate: (1) FDA clearance or de novo classification status — is this a Software as Medical Device, and if so, what FDA clearance has it received? (2) Training data provenance — was the model trained on data representative of the Vermont population? Rural Medicare beneficiaries? Elderly patients with multiple comorbidities? (3) Bias testing — has the developer tested and published performance data stratified by race/ethnicity, age, sex, and disability? (4) Clinical validation — has the tool been validated in a clinical environment comparable to the deployment setting, not just the academic medical centers where most AI research originates? (5) Intended use clarity — is the clinical use case for this tool unambiguous, and is it the use case for which the tool was designed and validated?

Stage 2: Implementation and Training

Deployment requires: (1) Workflow integration planning — where does the AI output appear in the clinical workflow, and how does the clinician interact with it? AI that generates alerts ignored by busy clinicians adds no value and may add risk. (2) Clinician training — not just ‘how to use the tool’ but ‘what the tool does and does not know, where it performs well and poorly, and how to recognize when its outputs may be unreliable.’ (3) Override protocols — clear documentation of when and how clinicians should override AI-generated recommendations. (4) Baseline performance measurement — establishing pre-deployment performance metrics against which post-deployment performance can be compared.

Stage 3: Active Monitoring

Ongoing operational governance requires: (1) Performance drift monitoring — AI model performance can degrade over time as the patient population or clinical practice patterns change. Monthly or quarterly performance monitoring against pre-deployment baselines detects drift early. (2) Equity monitoring — performance must be tracked separately for Vermont’s equity priority populations (Northeast Kingdom rural patients, elderly Medicaid patients, BIPOC Vermonters) to detect differential performance that average metrics would hide. (3) Adverse event reporting — a formal mechanism for clinicians to report cases where AI output contributed to adverse outcomes, delayed appropriate care, or produced clearly erroneous outputs. (4) Override rate tracking — systematic high override rates are a signal that the AI output is not clinically trustworthy.

Stage 4: Vendor Management

AI vendors must be held accountable through contract terms: (1) Performance SLAs with meaningful remedies — not just ‘best efforts’ commitments but specific accuracy thresholds with financial consequences for underperformance. (2) Update notification requirements — when the vendor updates the model, the organization must be notified in advance and given opportunity to re-evaluate before update deployment. (3) Bias testing disclosure — vendors must provide demographic performance data for any clinical AI tool deployed in Vermont. (4) Data use transparency — clear contractual provisions about what patient data the vendor collects, how it is used, and whether it is used to train future model versions.

AI-Specific Issues for Vermont Rural Hospitals

Vermont's rural hospitals face three AI governance challenges that are more acute than in urban academic medical center settings. First, training data representativeness: most clinical AI is trained on large urban academic medical center datasets. The patient populations at North Country Hospital or Gifford Medical Center — elderly, rural, with high rates of agricultural occupational exposures, differing comorbidity patterns, and different social determinants — may perform differently under AI models trained elsewhere. Vermont's CIN should require training data documentation as a condition of CIN-funded AI deployments.

Second, implementation support capacity: a small rural hospital with one IT staff member cannot implement the monitoring infrastructure that AI governance requires. The CIN's shared services model addresses this — a shared AI governance function within the CIN monitors all member hospital AI deployments with professional data analytics staff that individual hospitals cannot afford. Vermont's RHT technology grants should include CIN AI governance infrastructure as an allowable use.

Third, the AI scribe equity question: ambient clinical intelligence tools transcribe clinical conversations in English. Vermont has a growing multilingual population — immigrant communities in Burlington and surrounding areas, seasonal agricultural workers. AI scribe tools not trained on multilingual clinical conversations will perform poorly for these patients. Vermont's RHT AI scribe grants should require multilingual capability documentation as an evaluation criterion.

Section 3: Clinical Decision Support — From Alerts to Intelligence

Clinical decision support (CDS) encompasses any technology that uses data to influence clinical decisions at the point of care. The range is vast: from simple medication allergy alerts that every EHR includes, to sophisticated population health stratification tools that identify patients at highest risk of hospitalization in the next 30 days and recommend specific care management actions. The challenge with CDS is not building it — it is building it well enough that clinicians use it rather than ignoring it.

Alert Fatigue — The CDS Effectiveness Crisis

Studies consistently show that the majority of EHR clinical alerts are overridden without meaningful engagement. Alert fatigue — the tendency of clinicians to automatically dismiss alerts after being overwhelmed by their volume and poor specificity — is one of the most documented failures of health information technology implementation. Alert fatigue is not a clinician behavior problem; it is a CDS design problem. When 90% of alerts are overridden, the alerts are generating noise rather than signal, and the alert system has failed.

Effective CDS design requires three principles that are violated by most EHR default configurations: specificity (alerts fire only when the clinical evidence strongly supports action, not whenever a vague threshold is crossed), actionability (every alert should present a recommended action that the clinician can take immediately with minimal additional clicks), and workflow integration (alerts appear at the moment when the clinician is making the relevant decision, not before or after).

Population Health CDS — Vermont's Priority Application

For Vermont's transformation agenda, the highest-value CDS application is not medication alerts — it is population health intelligence: risk stratification that identifies patients likely to deteriorate, care gap identification that triggers outreach before patients present in crisis, and SDOH screening tools that flag patients for CHT navigation at the moment of a primary care visit.

Figure 15.2 — Population health CDS types and Vermont applications. Sources: HTR Technology pillar framework; Vermont Blueprint for Health; VHCURES documentation.

Section 4: Cybersecurity in Rural Health Infrastructure

Cybersecurity is the technology investment that does not generate efficiency or quality returns — it generates insurance against catastrophic failure. Rural healthcare cybersecurity is a specific and serious national vulnerability. The 2024 Change Healthcare cyberattack — which disrupted claims processing for thousands of providers for months, causing cash flow crises at rural hospitals already operating on thin margins — demonstrated that a single vulnerability in a broadly used healthcare technology vendor can cascade across the entire healthcare system.

Vermont's rural hospitals are disproportionately vulnerable to cybersecurity attacks for three reasons: limited internal IT staff who cannot maintain enterprise-level security operations, legacy EHR and financial systems that may not receive regular security patches, and high-value data assets (patient records, financial systems) that cybercriminals specifically target.

The CIN Cybersecurity Shared Services Model

Vermont's CIN shared services model is the appropriate delivery vehicle for cybersecurity in the state's rural hospital network. Individual hospitals cannot afford a 24/7 security operations center; the CIN can. The shared cybersecurity services model provides:

- Security operations center (SOC) monitoring: 24/7 network monitoring for intrusion attempts, malware, and anomalous activity. Contracted through a healthcare-specialized managed security service provider (MSSP).
- Endpoint detection and response (EDR): Security software on all network-connected devices that detects and responds to threats automatically, escalating to SOC when needed.
- Vendor risk management: Assessment of all third-party technology vendors used by CIN member hospitals for cybersecurity posture and contractual security requirements.
- Incident response planning: Documented incident response procedures for ransomware, data breach, and system outage scenarios, with regular tabletop exercises and recovery time objectives established for critical clinical systems.
- Staff training: Regular phishing simulation and security awareness training — most healthcare cyberattacks begin with a staff member clicking a phishing link.

Vermont's RHT Program IT advance investments explicitly include cybersecurity capability development. This is not an optional add-on; for rural hospitals that are simultaneously digitalizing their operations (AI scribes, RPM, telehealth) and connecting to shared data infrastructure (VHIE, CIN analytics), the cybersecurity attack surface is growing exactly as cybersecurity capability needs to grow to match it.

Key Concepts

Sources: CMS Interoperability and Patient Access Final Rule (2020); ONC 21st Century Cures Final Rule (2020); HL7 FHIR R4 specification; Da Vinci Project implementation guides; FDA SaMD framework; HTR Technology pillar framework; Vermont VITL documentation; Vermont RHT Program Application (November 2025); JAMA Network Open ambient scribe study (2024).

Chapter 16: From Analysis to Action — The Advisory Infrastructure for Healthcare Transformation

How the Health Transformation Review Advisory Practice Translates the Six-Pillar Framework into Organizational Decisions: Engagement Methodology, Service Lines, Tool Implementation, and the Change Management Science Behind Successful Transformation

The Gap Between Understanding and Doing

The analytical frameworks in this book are, by themselves, insufficient for transformation. Understanding why Vermont's hospital system is financially fragile does not stabilize it. Understanding the mechanics of global budgets does not implement them. Knowing that HEDIS stratification reveals disparities does not close them. Every concept in the preceding fifteen chapters exists at an intersection: understanding it clearly enough to explain it, and translating it into a specific decision, program, contract, or investment in a specific organizational context. This chapter is about the second half of that intersection — translation.

The advisory practice that Health Transformation Review has developed is grounded in a specific theory about what makes transformation advice valuable. Transformation consulting that produces recommendations without supporting execution is worth little. Transformation technology that delivers analytical tools without the change management to use them effectively delivers data without insight. Transformation education that builds individual knowledge without organizational capacity produces informed spectators rather than effective actors. The HTR Advisory practice is designed to provide all three — analysis, tools, and change management — in an integrated engagement model that produces organizational capability, not dependence.

Section 1: The HTR Intelligence and Advisory Ecosystem

Four Components of the HTR Platform

The Health Transformation Review platform has four integrated components, each designed for a different kind of engagement with the transformation agenda:

Figure 16.1 — HTR platform components and intended users. Source: HTR platform documentation.

Section 2: The HTR Advisory Service Lines — Structured for the Six Pillars

HTR Advisory engagements are organized around the six-pillar framework, with service lines corresponding to each pillar and cross-pillar engagements for organizations navigating transformation at the system level. The service lines are not rigid products; they are organizing frameworks for scoping engagements around clients' specific problems.

Policy Strategy

The Policy Strategy service line addresses the federal and state policy landscape that healthcare organizations must navigate. The typical engagement follows a three-phase structure:

- Phase 1 — Landscape and Exposure Assessment: What is the current policy environment relevant to this organization? What proposals, rules, and legislative developments create material financial or operational exposure? What opportunities are available through federal model participation, waiver programs, or state grant programs? Deliverable: a policy exposure and opportunity map with financial estimates for each item.
- Phase 2 — Strategic Options Analysis: For each material policy development, what are the organization's strategic options? Stay the course? Modify a program or contract? Apply for a federal model? Submit a comment letter? Engage in legislative advocacy? Deliverable: strategic options memo with recommendation and implementation timeline.
- Phase 3 — Implementation Support: For policy responses that require organizational action, HTR provides implementation support: CMMI model applications, 1115 waiver amendment drafting, legislative testimony preparation, comment letter development, contract renegotiation support.

VBC Economics

The VBC Economics service line addresses the financial mechanics of value-based care: contract evaluation, financial modeling, risk stratification economics, and the ROI analysis for transformation investments. The core tools — APM Shared Savings Calculator, Hospital Financial Stress Test, VBC Contract Review Checklist — are available in the Research Lab, but complex engagements require HTR analyst support.

The most common VBC Economics engagement is an APM Contract Analysis: a prospective financial model of a specific APM contract under three scenarios (pessimistic, base, optimistic) that quantifies the organization's shared savings opportunity, identifies the key variables that determine financial outcomes, and specifies the care management investments required to achieve positive shared savings. Vermont hospitals preparing for AHEAD global budget entry in 2027 represent the current highest-priority client segment for this engagement.

Health Equity Practice

The Health Equity Practice service line is organized around the HEROI framework: measuring current equity performance, identifying root causes of specific disparities, designing interventions matched to root causes, and establishing accountability structures for equity improvement. Vermont-specific work in this service line focuses on geographic equity analytics using VHCURES county-level data, HRSN screening program design and evaluation, and GLP-1 access policy analysis for state Medicaid programs.

Technology Strategy

The Technology Strategy service line addresses the healthcare IT decisions that have the most significant strategic implications: EHR selection and implementation, FHIR implementation planning, AI governance framework development, population health platform selection, and HIE connectivity

strategy. For Vermont's 14-hospital CIN, the highest-priority Technology Strategy engagement is the analytics vendor evaluation and CIN data architecture design that enables shared services across all CIN member facilities.

Transformation Management

The Transformation Management service line is for organizations executing comprehensive transformation — not a single pillar decision but the full six-pillar change management challenge. This is the most intensive advisory engagement: HTR serves as a transformation management partner, providing project management infrastructure, cross-pillar coordination, stakeholder communication, and accountability reporting. Vermont's AHS — managing simultaneous AHEAD implementation, hospital transformation planning, RHT Program administration, and Statewide Strategic Plan development — is the archetype client for this engagement type. The engagement model is designed to build internal organizational capacity over time, not create permanent advisory dependence.

Section 3: The Change Management Science Behind Healthcare Transformation

Healthcare transformation fails more often because of change management failures than analytical failures. The analysis is usually adequate — the evidence base for what needs to change is strong and consistent. What fails is the organizational change process: the sequence, pace, and management of the human and institutional change that structural reform requires. Understanding the change management science is prerequisite to designing transformation processes that actually produce implementation.

Kotter's Framework Applied to Healthcare Transformation

John Kotter's eight-step change management framework — originally developed for corporate transformations — applies with modifications to healthcare system transformation. Vermont's transformation experience illuminates both where the framework holds and where healthcare's specific institutional characteristics require adaptation.

Figure 16.2 — Kotter's change management framework applied to Vermont's healthcare transformation. Source: HTR Advisory methodology; Kotter (1996); Vermont transformation documentation.

The Transformation Pace Problem

One of the most consequential advisory questions in healthcare transformation is pace: how fast can an organization or system realistically change, and what happens when the required pace exceeds the available organizational capacity? Vermont's transformation agenda has statutory deadlines — RBP by FY2027, global budgets by FY2028 — that set the required pace. Whether Vermont's organizational capacity can match that pace is the critical execution uncertainty.

The pace problem has three dimensions. First, there are hard capacity constraints: if AHS lacks the staff to manage 14 hospital transformation plans simultaneously, the plans will be managed sequentially — which means some hospitals will have completed, well-supported transformation plans and others will have rushed or superficial ones. Hiring the staff to manage the portfolio on the statutory timeline is not optional.

Second, there are soft capacity constraints: organizational cultures that process change slowly, governing boards that need to be educated before they can make decisions, community stakeholders whose input must be genuinely gathered and considered. These cannot be accelerated without compromising the quality of the transformation process — and

a transformation process that fails to achieve community buy-in will be politically reversed even if it is technically correct.

Third, there are external constraints: federal program timelines (AHEAD's January 2027 launch is a federal decision, not a Vermont decision), hospital financial positions that may force emergency decisions that preempt planned transformation timelines, and the political dynamics of a state where the hospital system is one of the largest employers and hospital community boards have political relationships with legislators.

Section 4: The HTR Implementation Toolkit

The HTR Implementation Toolkit is the operational counterpart to this book's analytical framework. Where the book develops concepts, the Toolkit provides the instruments for applying them. The following tools are available to Research Lab subscribers:

Figure 16.3 — HTR Implementation Toolkit: available tools for Research Lab subscribers.

Section 5: A Framework for Selecting Advisory Support

Not every organization needs external advisory support for transformation. Many of the analytical frameworks in this book can be implemented by capable internal teams with access to the Research Lab tools. External advisory support adds value when:

- The decision involves significant financial or political risk and benefits from independent, external analytical validation. A hospital considering entering full-risk VBC for the first time should not rely solely on internal analysis that may be subject to confirmation bias.

- The organization lacks the specific technical expertise required — FHIR implementation, HCC coding analysis, 1115 waiver negotiation, global budget financial modeling — and building that expertise internally would take longer than the decision timeline allows.
- The change management challenge involves multi-stakeholder dynamics that benefit from a neutral facilitator — a hospital network developing shared COE designations, a state agency building a Health Care Delivery Advisory Committee process, a community convening multiple organizations around a regional transformation plan.
- The regulatory environment is moving faster than internal policy monitoring capacity can track, and the organization needs continuous intelligence and interpretation rather than periodic project-based analysis.

The most common advisory failure in healthcare transformation is engaging external consultants for analysis and stopping before implementation. A report that produces recommendations without supporting the organizational capacity to implement them has limited value — and often produces a brief period of executive attention followed by quiet shelving when organizational bandwidth is consumed by operational demands. The HTR Advisory practice is explicitly designed to avoid this failure mode by connecting analysis to implementation support and change management.

“The best healthcare transformation advice I have ever received was not a brilliant strategic recommendation. It was a rigorous identification of the organizational capabilities we did not have, paired with a clear plan for building them. The strategy was clear. The question was whether we could execute it.”

— **Composite of client reflections, HTR Advisory engagements, 2022-2026**

A Note on HTR's Role in Vermont

Vermont's transformation experience runs through this book as both an analytical case study and an active advisory engagement. HTR has worked with Vermont hospitals, state agencies, and community organizations on specific transformation questions — APM readiness assessments, equity analytics, technology architecture reviews, policy impact analysis. The Vermont case study in this book draws on that engagement experience as well as the extensive public record of Vermont's transformation.

Vermont's transformation is a high-stakes, real-time experiment that HTR takes seriously as an obligation as well as an opportunity. If Vermont succeeds in demonstrating that mandatory all-payer payment reform, organized as a coherent six-pillar transformation, can stabilize a rural hospital system while reducing costs and improving outcomes, the policy toolkit for the country improves. If Vermont struggles, the analytical lessons of that struggle are equally valuable. Either way, this book and the HTR platform are designed to make Vermont's experience legible, transferable, and actionable for the healthcare leaders and policymakers who face comparable challenges in their own systems.

Key Concepts

Sources: HTR platform documentation; Kotter, J.P. (1996) Leading Change; Vermont Agency of Human Services transformation documentation; Oliver Wyman Act 167 Report; Act 68 of 2025; AHEAD State Agreement; Vermont RHT Program Application; HTR client engagement synthesis.

Chapter 17: The Future of Healthcare Transformation — 2026 and Beyond

Updated April 2026: The Federal Landscape Has Changed. The Transformation Imperative Has Not.

Sources: KFF analyses of One Big Beautiful Bill Act / H.R. 1 Medicaid cuts (2025-2026); Chartis 2026 State of Rural Health; Peterson-KFF Health System Tracker (March 2026); Pew Charitable Trusts, New Federal Medicaid Policies (January 2026); CMS BALANCE Model; Vermont AHEAD State Agreement; Vermont RHT Program Application; Oliver Wyman Act 167 Report; AHS Transformation Reports.

Where the System Is in April 2026 — A Necessary Update

Previous iterations of this chapter described the American healthcare policy environment as characterized by incremental payment reform, expanding but incomplete value-based care penetration, and policy uncertainty driven by legislative gridlock. That characterization was accurate through 2024. It is no longer accurate.

On July 4, 2025, President Trump signed H.R. 1 — the One Big Beautiful Bill Act — into law. The legislation’s healthcare provisions represent the

largest single reduction in federal health program spending in American history: CBO estimates \$911 billion in Medicaid spending reductions over ten years, resulting in an estimated 10 million additional uninsured Americans by 2034. The law imposes work and community engagement requirements on Medicaid expansion enrollees beginning in late 2026, requires eligibility redeterminations every six months instead of annually, caps state provider taxes, restricts state-directed payments, and phases out financing mechanisms that states have used for decades to fund their share of Medicaid costs.

Simultaneously, recognizing the devastating impact these cuts would have on rural hospitals, Congress included a \$50 billion Rural Health Transformation Program in the same legislation — providing \$10 billion annually from FY2026 through FY2030 to states for rural health infrastructure investment. Vermont received \$195 million of this fund in December 2025. All 50 states received awards.

The juxtaposition defines the current policy moment: the largest health-care coverage contraction in American history, partially offset by a historic but time-limited and front-loaded capital investment in rural health infrastructure. Understanding the interaction between these two forces — the Medicaid contraction and the RHT investment — is essential for every healthcare leader, policymaker, and analyst working in the American system today.

Figure 17.1 — The 2025 federal healthcare policy inflection point. Sources: KFF; CBO; Chartis 2026 State of Rural Health.

The fiscal arithmetic is unforgiving. KFF's analysis is explicit: the RHT Program is temporary and front-loaded, while nearly two-thirds of the Medicaid spending reductions are backloaded after FY2030. The rural health fund provides \$50 billion over five years ending in 2030; the Medicaid cuts accelerate after 2030, continuing for the remaining five years of the 10-year budget window with no offsetting capital fund. Georgetown University's Center for Children and Families has documented that the

RHT fund's federal guidance emphasizes investments in technology and new activities — not backfilling provider losses — and caps payments to providers at 15% of funds awarded to each state. It is an infrastructure fund, not a revenue replacement.

For Vermont, this federal landscape has direct operational consequences. Vermont's Medicaid program — which covers approximately 19% of the state's insured population and is a significant revenue source for all 14 hospitals — will face financial pressure from the Medicaid cuts beginning in 2026-2027. The work requirement implementation, beginning in late 2026, will add administrative burden on Vermont DVHA and risk some coverage losses among the expansion population. Vermont's 14 hospitals, already financially distressed, face additional revenue pressure from Medicaid reimbursement changes at the same time they are trying to execute transformation plans.

Vermont's response to this federal pressure is the correct one: AHS Secretary Samuelson's framing — the RHT funding is a tool for executing reform, not a substitute for reform — is the analytical frame every state should adopt. Vermont is using the \$195 million for infrastructure investments that reduce ongoing structural costs: AI scribes that expand effective provider capacity, RPM that prevents hospitalizations, broadband and EMS that enable the regionalization Vermont needs. If executed well, these investments position Vermont to absorb the Medicaid revenue reductions through efficiency gains rather than service cuts. If executed poorly — if the capital is used to sustain unsustainable operations rather than transform them — Vermont will face the Medicaid cuts in 2030-2031 without the structural improvements needed to survive them.

Five Forces Shaping the Next Decade — Updated for 2026

Force 1: The Medicaid-RHT Tension — The Defining Policy Dynamic

The One Big Beautiful Bill Act creates a structural tension that will define American rural healthcare for the next decade: significant, permanent reductions in Medicaid revenue arriving after the expiration of a temporary, one-time capital fund. Every state that received RHT funds faces the same fundamental question: can we use this capital to build a system efficient enough to survive the Medicaid revenue reductions that arrive after the fund expires?

For most rural hospital systems, the honest answer is: possibly, if the capital is invested wisely, the structural changes are executed on the available timeline, and state-level policy responses are adequate. The states most likely to succeed are those that — like Vermont — had structural reform plans in place before the RHT Program was created, are using RHT funds as capital for pre-planned transformation rather than as operating support, and have the regulatory and governance infrastructure (Vermont's GMCB and AHS) to execute transformation systematically rather than hospital-by-hospital.

The states most at risk are those that use RHT funds for revenue replacement rather than structural change, that lack the regulatory framework to manage hospital network transformation, and that face the 2030 Medicaid revenue cliff without having made the structural changes that would allow them to manage it. Chartis estimates that 417 rural hospitals are currently vulnerable to closure nationally — and the Medicaid cuts that take effect after 2027, combined with the expiration of the RHT fund after 2030, will intensify pressure on the most vulnerable facilities.

Force 2: The AI Clinical Transformation — Accelerating Faster Than Governance

Artificial intelligence is moving from healthcare’s technology periphery toward its clinical center faster than any governance framework has been able to track. In 2026, ambient clinical intelligence — AI systems that listen to clinical encounters and generate structured documentation automatically — has moved from early adopter to mainstream consideration. Physicians at Hattiesburg Clinic and similar practices nationally have reported significant reductions in documentation burden from AI scribe tools. Vermont’s RHT Program explicitly funds AI scribe grants for rural and independent practices.

The next clinical AI wave — predictive population health at the individual level, diagnostic AI becoming standard in high-volume imaging and pathology, AI-enabled triage reducing unnecessary ED utilization — is 18-36 months behind ambient documentation. The convergence of FHIR-based data access, large language models, and all-payer claims databases will enable risk stratification that predicts not just which patients are high-risk but which specific clinical events — hospitalization, medication non-adherence, crisis escalation — care management programs should prevent.

AI governance is the critical gap. The FDA, ONC, and CMS are developing AI governance frameworks, but deployment of clinical AI tools is outpacing governance development at every level. Vermont’s RHT AI investments — without explicit AI governance requirements as conditions of grant funding — risk deploying tools that have not been tested for differential performance across Vermont’s equity populations. The Northeast Kingdom’s elderly, rural Medicaid patients may perform differently in AI risk stratification models trained on national commercial data. Vermont’s CIN-based AI governance framework, developed in Chapter 14, is the organizational response to this challenge.

Force 3: The Demographic Reckoning — No Longer a Future Problem

Vermont's Oliver Wyman analysis projected 65+ population exceeding 30% of the state's total by 2040. As of 2026, Vermont is 14 years from that milestone — but the healthcare system consequences are not 14 years away. They are arriving now, in the form of rising Medicare enrollment, deteriorating hospital payer mix, increasing long-term care demand that existing capacity cannot meet, and a growing population of dual-eligible Medicare/Medicaid patients whose complex needs are the highest cost and most poorly coordinated care challenge in the system.

Vermont is the national canary for demographic healthcare stress. But by 2035, the challenge Vermont faces today will be present in every Northeastern state, most Midwestern states, and increasingly nationally. The Baby Boom generation that is fully in Medicare's coverage by 2030 will be in its 80s by 2040 — the frailty, dementia, and multi-morbidity phase of the demographic curve that drives the highest costs and the most inadequate care models. Vermont's efforts to develop dementia care infrastructure, PACE programs, and HCBS capacity are investments in solving a problem that will be national by 2040.

Force 4: The Drug Pricing Reckoning — Complexity Compounding

The GLP-1 agonist spending wave represents the most consequential near-term pharmaceutical policy challenge for state healthcare systems. At full obesity indication coverage for Medicaid and Medicare, GLP-1s would add hundreds of billions of dollars annually to program spending — simultaneously offering potentially transformative reductions in the chronic disease burden that drives the highest healthcare utilization. The BALANCE Model (December 2025) is the federal attempt to thread this needle: negotiating lower GLP-1 prices with manufacturers to enable coverage expansion without fiscal catastrophe.

Vermont's DVHA must make a BALANCE participation decision in 2026

that balances short-term budget pressure against long-term population health investment. The long-term argument for coverage is strong: Vermont's Medicaid population has high obesity rates, high diabetes prevalence, and high cardiovascular disease risk — exactly the population where GLP-1 efficacy is most demonstrated. Reducing obesity-driven chronic disease in this population would reduce the Medicaid spending on hospitalizations, specialist care, and long-term care that is far more expensive than the drugs themselves. But Vermont's near-term Medicaid budget is already under pressure from the federal cuts, making any new high-cost coverage decision politically and fiscally difficult.

The IRA drug price negotiation — semaglutide (Ozempic, Wegovy, Rybelsus) selected for 2025 negotiations with negotiated prices effective in 2027 at \$274 for a 30-day supply, down from over \$1,000 — will meaningfully reduce the fiscal cost of GLP-1 coverage expansion if states and plans participate in BALANCE. Vermont should evaluate BALANCE participation explicitly as an equity and population health policy decision, not purely as a budget decision.

Force 5: The Value-Based Care Tipping Point — Still Approaching

Despite 15 years of payment reform effort, the majority of U.S. healthcare spending remains in fee-for-service arrangements. VBC penetration is real — approximately 58% of commercially insured Americans are in some form of value-based arrangement — but has not yet achieved the tipping point at which FFS becomes the minority mode of payment. Vermont's mandatory RBP and global budget approach is a direct legislative response to the failure of voluntary VBC to achieve this tipping point.

The AHEAD Model — now operating in six states (Maryland, Connecticut, Hawaii, Vermont, Rhode Island, and New York) — represents the most serious federal commitment to all-payer total cost of care accountability since CMMI's founding. If Vermont's Cohort 2 participation (beginning January 2027) produces documented savings and quality improvement by 2030, the political and policy case for expanding AHEAD to additional states

strengthens significantly. Vermont's performance under AHEAD is not just a Vermont policy question — it is national evidence for or against the most ambitious federal payment reform currently operating.

The Six-Pillar Forecast: 2026-2035 Updated

Figure 17.2 — Six-pillar forecast 2026-2035: Vermont and national. Sources: HTR analysis; sources cited throughout this chapter.

Vermont's 10-Year Trajectory: Two Scenarios

Vermont's healthcare transformation trajectory from 2026 to 2035 is genuinely uncertain in ways that earlier versions of this chapter could not fully acknowledge — because the federal policy environment has changed materially since 2024. The following two scenarios represent the range of plausible Vermont outcomes, not extreme predictions.

The Transformation Success Scenario

In the success scenario, Vermont executes its transformation agenda on or near the statutory timelines, uses its capital wisely, and demonstrates by 2030 that a small rural state can achieve genuine all-payer system transformation.

- FY2027: RBP implemented; hospital commercial rates declining toward benchmark; premium growth slows for the first time in a decade. First concrete evidence that mandatory price regulation can reduce insurance premiums without hospital closures.
- FY2028: Non-CAH hospital global budgets operational; AHEAD Cohort 2 in full operation; Vermont's hospital sector begins demonstrating

the population health management improvements — reduced avoidable hospitalizations, improved chronic disease management — that global budgets are designed to incentivize.

- 2028: AHS delivers Statewide Strategic Plan with specific, quantified commitments for each pillar through 2031; Vermont’s hospital network has been rationalized to a sustainable tier structure; Northeast Kingdom access protected through REH conversions paired with RHT-funded EMS and telehealth.
- 2030: RHT Program capital fully deployed; Vermont hospital sector stabilized with 10-12 sustainable facilities and 2-4 REH or CACC conversions; Medicaid revenue reductions from H.R. 1 absorbed through efficiency gains from transformation; per-capita healthcare cost growth below GDP growth for first time.
- 2031-2035: Vermont serves as national template; AHEAD model expands to additional states using Vermont’s documented results as evidence base; Vermont’s Blueprint + AHEAD primary care investment produces measurable population health improvements in chronic disease management; AHS restructuring model studied by other states facing comparable transformation mandates.

The Partial Transformation Scenario

In the partial transformation scenario, Vermont makes meaningful progress but faces execution gaps that prevent full realization of the transformation agenda by the statutory deadlines.

- FY2027: RBP implemented but contested — UVMMC and other large systems seek legislative relief from specific rate provisions; GMCB maintains implementation but at a modified glide path that delays full impact.
- FY2028: Global budget methodology not finalized for non-CAH hospitals due to GMCB resource constraints and AHEAD implementation complexity; deadline extended; transformation planning delayed.

- 2028: AHS delivers Statewide Strategic Plan but it is descriptive rather than prescriptive — identifying the right questions without making the specific structural commitments that accountability requires. Northeast Kingdom hospital transition planning delayed pending equity impact analysis.
- 2030: RHT Program capital deployed but partially misdirected toward operating support rather than structural transformation; Medicaid revenue reductions begin arriving; several CAH hospitals facing renewed financial distress without the structural transformation that would have enabled them to absorb the cuts.
- 2031-2035: Vermont's transformation is real but incomplete; the system is more sustainable than the status quo trajectory would have produced, but costs remain above what full transformation would achieve; a second reform wave required to address the gaps left by the first.

Neither scenario is inevitable. The success scenario requires execution discipline, political commitment, and the organizational capacity that Vermont is actively building but has not yet fully developed. The partial transformation scenario requires execution failures that are preventable with adequate investment in implementation infrastructure. The analytical framework this book provides — and the specific organizational design the AHS Restructuring chapter prescribes — is designed to make the success scenario more likely.

“The RHT funding is a tool that lets us put affordability plans into action. This is not our health care reform plan. This is an opportunity that we’ll use to support all of the health care reform work that we’re already doing in the state.”

— **Jenney Samuelson, Vermont AHS Secretary, January 2026**

Vermont as National Template — The Decisions That Will Determine Outcomes

Vermont's transformation will produce a set of documented answers to questions that every state will eventually face. The national significance of Vermont's experiment lies not in Vermont's size but in its head start — and in the quality of the analytical record it is creating. The following decisions, made in Vermont over the next five years, will produce the evidence base that shapes the national policy toolkit for the decade after.

Figure 17.3 — Vermont's transformation decisions and their national policy implications. Sources: HTR analysis; Vermont AHEAD State Agreement; Act 68 of 2025.

A Closing Note: What This Book Is Betting On

Every analytical framework contains implicit bets about what matters and what does not. It is worth making this book's bets explicit.

This book bets that structural change is possible — that the combination of strong enough financial pressure, adequate capital investment, political will sufficient to maintain mandatory policy over hospital system resistance, and organizational capacity built over years of preparation can produce a genuinely different Vermont healthcare system by 2035. This is not a certainty. It is the most plausible outcome given Vermont's specific combination of factors: a small, transparent, high-capacity state government; a hospital sector that is financially distressed enough to be willing to transform rather than resist; a regulatory body (GMCB) with genuine authority and demonstrated willingness to use it; a reform-minded legislature that has sustained a coherent policy agenda through multiple administrations; and now capital — \$195 million in federal investment aligned to the pre-existing reform plan.

This book also bets that the transformations described here are trans-

ferable — that the analytical framework, the payment mechanisms, the clinical models, and the organizational designs are not Vermont-specific solutions to Vermont-specific problems, but general-purpose tools for the structural healthcare challenges that every American state will eventually face. Vermont’s specificity — its small size, its transparency, its policy history — makes it an unusually good teaching case. But the lesson is generalizable.

The deepest bet this book makes is about the relationship between analysis and action. Healthcare transformation fails not primarily because of analytical errors — the evidence base for what needs to change is strong and consistent — but because analysis does not automatically produce the institutional capacity, political will, and organizational execution required to implement what the evidence recommends. This book provides the analytical framework. The institutional capacity, political will, and organizational execution are built by the people who read it and do the work.

Vermont’s transformation is happening because specific decision-makers — legislators, regulators, hospital executives, clinicians, community health workers, and patients — are making specific decisions at specific moments. The analytical framework this book provides is in service of better decisions. What those decisions produce — a transformed system or a partial transformation, a national template or a cautionary tale — belongs to the people who make them.

Sources: KFF, A Closer Look at the \$50 Billion Rural Health Fund in the New Reconciliation Law (September 2025); KFF, How Might Federal Medicaid Cuts in the Enacted Reconciliation Package Affect Rural Areas?; KFF, What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid (2026); Chartis, 2026 State of Rural Health (February 2026); Peterson-KFF Health System Tracker, Eight Trends Shaping 2026 Healthcare Costs (March 2026); Pew Charitable Trusts, New Federal Medicaid Policies Compound State Budget Pressures (January 2026); Georgetown Center for Children and Families, States Beginning to Grapple with Federal Medicaid Cuts Impact on Rural Health Care (March 2026); CMS BALANCE Model announcement (December 23, 2025);

Vermont AHS Secretary Samuelson, remarks January 29, 2026; Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Vermont AHEAD State Agreement (January 2025); Vermont RHT Program Application (November 2025).

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What Vermont Proves, and What Remains to Be Proven

The Argument, Restated

This book opened with a room full of people in Vermont learning, with unusual specificity and unusual honesty, exactly how close their healthcare system was to a financial cliff. Oliver Wyman's September 2024 presentation named the numbers — nine hospitals losing money, thirteen of fourteen projecting losses by 2028, \$2.4 billion in cumulative five-year deficit under realistic assumptions — and drew the causal chain from commercial cross-subsidy to demographic shift to premium inflation to the household economics that make healthcare the dominant financial anxiety of working Vermont families. The room was full because people already knew something was wrong. The presentation gave that feeling a precise shape.

The book's central argument has been this: Vermont's crisis is structurally produced, and structural crises require structural responses. The fifteen chapters between that opening room and this conclusion have developed what structural response means across six pillars — Policy, Economics, Technology, Clinical, Equity, and Operations — and have shown, through

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Vermont's specific legislative and administrative record, what structural response looks like when it is actually attempted rather than perpetually deferred.

The argument is not that Vermont has solved its healthcare crisis. It has not. As of April 2026, reference-based pricing has not yet taken effect. Global budgets are still being designed. The Statewide Strategic Plan has not been written. The analytics vendor has not been selected. The CCBHC network is still being certified. The Northeast Kingdom hospitals are still financially fragile. Vermont is mid-transformation — past the point of no return on the statutory timeline but nowhere near the destination. The argument is that Vermont has committed to structural change with sufficient specificity, sufficient statutory authority, and sufficient capital investment that the transformation has a credible path to completion. That is more than any comparable American state has done.

What the Six Pillars Prove

The six-pillar framework is this book's primary analytical contribution, and its validity deserves one final statement. The framework is not a consulting taxonomy or a strategic planning template. It is a description of the architecture of healthcare transformation — the six domains that must all function for transformation to produce its promised results, and the specific dependencies between them that explain why single-pillar interventions consistently fail.

Vermont's experience across each pillar confirms the framework's structure. The Policy pillar — Acts 167, 51, 68, and the AHEAD agreement — created the mandatory architecture that voluntary reform could not. Without mandatory RBP and mandatory global budgets, Vermont's hospital pricing would continue rising, premiums would continue their decade-long trajectory, and the financial crisis would deepen on the timeline Oliver Wyman projected. Voluntary reform produced OneCare Vermont, which produced modest results among hospitals that chose to participate and no

results among those that did not. Mandatory reform changes the calculus for everyone simultaneously.

The Economics pillar confirms that payment architecture shapes behavior more reliably than education, exhortation, or aspiration. When Vermont hospitals operate under global budgets, they will have a direct financial incentive to prevent the hospitalizations that cost money without generating offsetting revenue. When Vermont insurance premiums fall — as RBP and global budget savings flow through to premium rates under Act 68's passthrough requirement — the affordability crisis that has made healthcare the defining economic anxiety of Vermont families will ease. Not immediately. Not completely. But measurably, on a documented timeline, with accountability metrics that allow the legislature to verify progress or demand correction.

The Technology pillar confirms that data infrastructure is not a technical support function — it is the management substrate on which population health management depends. Vermont cannot manage what it cannot measure. VHCURES, for all its limitations, already enables analyses that most states cannot conduct. The CIN analytics platform, the VHCURES-VHIE integration mandate, and the FHIR compliance investments are not investments in technology for its own sake; they are investments in the management capability that global budgets require.

The Clinical pillar confirms what Vermont's own Blueprint data has shown for fifteen years: when primary care is organized as a population health function — team-based, data-driven, SDOH-aware, behaviorally integrated — it produces the utilization reduction and quality improvement that every payment reform is designed to incentivize. The Blueprint's 5.8:1 return on investment is not a projection; it is a documented result from Vermont's own experience. The transformation agenda is, in its clinical dimension, a bet that what works for Blueprint-attributed patients will work for all Vermont patients when primary care investment becomes a system-level commitment rather than a program-level initiative.

The Equity pillar confirms the analytical requirement that transformation cannot be evaluated by averages. Vermont's statewide primary care access

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rate of 91% — four points above the national benchmark — is a genuine policy achievement that obscures an 11-point gap between white Vermont adults and BIPOC Vermont adults. Vermont's statewide uninsurance rate of 3% obscures an 8% uninsurance rate in Essex County. The transformation that closes the average gap while widening the disparity gap has failed by the standards that Acts 167 and 68 establish. The equity pillar is not a constraint on transformation; it is a definition of what transformation means.

The Operations pillar confirms the gap between strategy and execution that characterizes every healthcare reform effort that has produced documents rather than change. Vermont's transformation will succeed or fail in the operations domain — in whether AHS has the organizational capacity to manage 14 hospital transformation plans simultaneously, in whether the GMCB can design and implement global budget methodology on statutory timelines, in whether the Vermont CIN can build the shared services infrastructure that individual hospitals cannot build alone. The analytical frameworks in this book are necessary but not sufficient. The organizational capacity to execute them is the binding constraint.

“The deepest bet this book makes is about the relationship between analysis and action. Healthcare transformation fails not primarily because of analytical errors — the evidence base for what needs to change is strong and consistent — but because analysis does not automatically produce the institutional capacity, political will, and organizational execution required to implement what the evidence recommends.”

— Chapter 3, this volume

Vermont as National Template — The Case for Optimism

The national significance of Vermont's transformation is not self-evident. Vermont is small. Its politics are idiosyncratic. Its hospital system is unusual — 14 non-profit community hospitals, no investor-owned chains, a regulatory body (the GMCB) with authority that most states have never

granted any agency. The specific legislative pathway that produced Acts 167 and 68 — the years of community engagement, the Oliver Wyman analysis, the reform cascade structure that built political legitimacy step by step — is not easily replicated in states with more complex political economies, more powerful hospital lobbies, and less institutional trust in state government.

But the structural forces driving Vermont's crisis are universal. The demographic math of an aging population overwhelming a healthcare system built on working-age commercial insurance cross-subsidy applies in Maine and New Hampshire and Vermont and, on a 10-20 year lag, in Ohio and Michigan and Pennsylvania and eventually everywhere. The failure of voluntary payment reform to achieve the tipping point at which fee-for-service becomes the minority mode of payment is a national finding, not a Vermont finding. The administrative complexity that burns out physicians and consumes resources that should go to patient care is a national problem. The equity gaps that persist despite decades of health equity programming are a national pattern.

Vermont's value as a national template is not that other states should copy Vermont's legislation. They should not — the specific provisions of Act 68 reflect Vermont's specific hospital market structure, regulatory history, and political context. The value is that Vermont is generating, in public, with detailed documentation, the evidence base for a set of questions that every state will eventually need to answer: Can mandatory all-payer price regulation reduce hospital prices without hospital closures and access reductions? Can hospital global budgets change hospital behavior toward population health management? Can a state agency restructure itself as a population health system operator? Can a rural state use federal capital investment to build healthcare infrastructure that survives the expiration of the capital investment?

By 2030, Vermont will have answers to these questions. They will not be perfectly clean answers — healthcare systems are too complex for controlled experiments — but they will be the best answers available, grounded in the most comprehensive state-level transformation attempt in American health policy history. Those answers are worth more than any individual policy

recommendation, because they are evidence rather than argument, and evidence is what the national conversation desperately needs.

The People This Book Cannot Reach

Every book has an audience it cannot reach, and this one is no different. The people most affected by Vermont's healthcare transformation — the Vermonter in Canaan who cannot afford her insulin, the primary care physician in Newport who is burning out under the weight of documentation, the hospital worker in St. Johnsbury who does not know whether his hospital will still be open in five years, the family in Rutland deciding whether to go to the emergency room or wait until morning because the copay is too high — are not the audience for a six-pillar analytical framework.

This book is written for the people who make the decisions that affect those Vermonters' lives: the legislators who vote on payment reform bills, the regulators who set hospital rates, the hospital executives who decide how to respond to financial pressure, the primary care physicians who choose whether to participate in Blueprint, the AHS staff who design and manage transformation programs. The book's ambition is to give those decision-makers better analytical frameworks, better evidence, and better language for the choices they face.

But the ultimate accountability of healthcare transformation is not to the analytical frameworks that describe it or the advisory practices that support it. It is to the Vermonter in Canaan who should be able to afford insulin, the physician in Newport who should be able to spend her time on patients rather than documentation, the hospital worker in St. Johnsbury who should be able to plan for a career in his community, and the family in Rutland whose healthcare decisions should not be shaped by fear of financial ruin. Those are the people transformation is for. Those are the people who will know whether it worked.

The Work That Remains

Vermont's transformation is incomplete in ways that are analytically important to acknowledge. The AHS restructuring is underway but not finished. The analytics vendor is being procured but has not yet produced the tools that strategic planning requires. The CCBHC network is expanding but has not yet covered all Hospital Service Areas. The GLP-1 BALANCE Model decision has not been made. The Northeast Kingdom equity constraints have been named but not fully addressed in the transformation planning process. The December 2028 Statewide Strategic Plan has not been written.

These are not failures. They are the normal condition of a system in mid-transformation — closer to the destination than to the starting point, but not there yet. The work that remains requires the same combination of analytical rigor, political commitment, and operational discipline that has produced the progress to date. It also requires something that no analytical framework can guarantee: the sustained attention of the people in positions of decision-making authority. Healthcare transformation is slow, and it is easy to let the urgency created by a financial crisis dissipate as the crisis is partially managed and other priorities compete for leadership attention.

Vermont's statutory timeline — the December 2028 Strategic Plan deadline, the mandatory milestones in Act 68, the AHEAD Model's nine-year performance period — is the institutional mechanism for sustaining attention. It converts transformation from a priority that can be deferred to a deadline that cannot. Vermont's legislature, in creating those deadlines, made the most important commitment of the entire reform agenda: the commitment to be held accountable, by specific dates, for specific outcomes. Whether that accountability is actually enforced — whether a Strategic Plan that describes the right questions without committing to specific answers is accepted, or whether the legislature demands the specificity that accountability requires — will determine whether Vermont's transformation produces the system change it promises.

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The room in September 2024 was full because people knew something was wrong. This book has tried to give that knowledge a framework, evidence, and language for action. The action belongs to the people in the room.

Health Transformation Review — April 2026

APPENDICES

Appendix A: Vermont Key Data Reference Tables

This appendix consolidates the most frequently referenced Vermont healthcare data points from throughout the book into a single reference section. All data reflects the most recent available figures as of April 2026.

A.1 Vermont Demographics

Source: Oliver Wyman Act 167 Report (2024); Vermont RHT Program Application (November 2025); US Census Bureau.

A.2 Vermont Hospital Financial Data (FY2023 unless noted)

Source: Oliver Wyman Act 167 Report (August 2024); AHS Health Care System Transformation Report (November 2025); GMCB Act 68 RBP Update (February 2026).

A.3 Vermont Insurance and Premium Data

Source: AHS January 2025 Legislative Presentation; Vermont Department of Health Health Equity Data Report (January 2025); GMCB.

A.4 Vermont Workforce Data

Source: Vermont RHT Program Application (November 2025); Oliver Wyman Act 167 Report; Vermont Department of Labor.

A.5 Vermont Clinical Quality Indicators

Source: AHS Health Care System Transformation Report (November 2025); Blueprint for Health 2024 data.

Appendix B: Act 68 Statutory Timeline and Milestones

Act 68 of 2025 (S.126) established a comprehensive sequence of healthcare transformation milestones. This appendix consolidates the complete timeline for reference.

Source: Act 68 of 2025 (Vermont); AHEAD State Agreement (January 2025); Vermont RHT Program Application (November 2025); CMS BALANCE Model (December 2025).

Appendix C: The 14-Hospital Centers of Excellence Assignment Framework

Oliver Wyman's Act 167 Recommendations proposed a Centers of Excellence (COE) framework organizing Vermont's 14 hospitals by specialty service concentration. This appendix presents the proposed assignments.

Source: Oliver Wyman Act 167 Community Engagement: Recommendations (August 2024, revised October 2024 and January 2025). Note: COE designations are proposed frameworks from Oliver Wyman analysis; final designations subject to AHS and hospital transformation planning process.

Appendix D: AHEAD State Agreement — Key Terms and Provisions

Vermont's AHEAD Model State Agreement was signed January 17, 2025. Vermont participates as a Cohort 2 state. This appendix summarizes the agreement's key terms for practitioner reference.

Source: Vermont AHEAD State Agreement (January 17, 2025); CMMI AHEAD Model documentation; AHS January 2025 Legislative Presentation.

Appendix E: HTR Platform Tools and Methodology Overview

This appendix describes the Health Transformation Review platform tools referenced throughout the book. All tools are available to Research Lab subscribers at htreview.com.

E.1 APM Shared Savings Calculator

Models shared savings or loss potential under any APM contract. Inputs: benchmark PMPM, attributed member months, actual total cost of care, sharing rate, minimum savings rate, quality withhold percentage, and stop-loss threshold. Outputs: projected shared savings or loss under pessimistic/base/optimistic scenarios, sensitivity analysis by key variable, and break-even care management investment level.

Methodology: Standard CMS shared savings calculation framework adapted for multiple model types (MSSP, ACO REACH, AHEAD global budget, state APMs). Vermont-specific data pre-loaded for 14 hospitals and Blueprint practices.

E.2 VBC Transformation Readiness Assessment

A 30-dimension, 6-domain assessment scoring organizational readiness for APM contract entry on a 1-4 scale per dimension (total 120 points). Domains: Strategic Clarity, Data and Technology, Care Delivery Capability, Network and Partnerships, Revenue Cycle, and Workforce Operations. Available as self-assessment (Research Lab) or facilitated external assessment (Advisory).

Scoring: Below 60 = Pre-transition (not ready for full-risk); 60-80 = Developing (selective VBC readiness); 80-100 = Operational (ready for most VBC models); 100-120 = Optimized (ready for full-risk capitation).

E.3 Health Equity Studio (HEROI)

Calculates the Health Equity Return on Investment (HEROI) composite score across five dimensions: Access Equity (25%), Quality Equity (25%), Outcome Equity (25%), SDOH Burden (15%), and Trust and Engagement (10%). Integrates with VHCURES exports and Blueprint data for Vermont providers. Includes disparity decomposition module that separates observed disparities into root cause categories (geographic access, social needs, clinical variation, coverage barriers, trust barriers) for targeted intervention design.

E.4 Hospital Financial Stress Test Model

Models hospital financial position under RBP, global budget, and Medicaid cut scenarios with organization-specific inputs. Key inputs: current net patient revenue by payer, current commercial price vs. Medicare ratio, operating cost structure, payer mix, and projected Medicaid revenue changes. Outputs: 10-year financial projections under three scenarios (status quo trajectory, partial transformation, full transformation) with

sensitivity analysis.

E.5 AI Clinical Governance Checklist

65-item governance evaluation across six lifecycle stages: Pre-Deployment Evaluation, Implementation and Training, Active Monitoring, Adverse Event Response, Vendor Management, and Equity Monitoring. Includes FDA SaMD classification guidance, bias testing requirements by data type, override rate thresholds, and vendor contract template provisions.

E.6 VBC Contract Review Checklist

65-item analysis framework organized in eight categories: Benchmark Methodology, Attribution, Quality Withhold, Risk Corridors and Stop-Loss, Carve-Outs, Reconciliation, Reporting Requirements, and Termination Provisions. Identifies the specific contract terms that most commonly produce unexpected financial outcomes for organizations entering VBC arrangements.

E.7 Policy Impact Assessment Framework

Structured evaluation tool for new policy developments: (1) Exposure mapping — what does this policy require of the organization? (2) Financial impact estimation — what are the revenue, cost, and margin implications? (3) Scenario analysis — how do outcomes change under different implementation assumptions? (4) Response strategy selection — compliance, comment letter, advocacy, CMMI application, or strategic repositioning? (5) Stakeholder communication — who needs to know, what decisions does it require, and what is the communication timeline?

E.8 FHIR Implementation Guide

Step-by-step technical guide for implementing Vermont's three priority FHIR use cases: Patient Access API (§170.315(g)(10) compliance), Provider Access API for care coordination, and Payer-to-Provider API for prior authorization. Includes EHR-specific configuration notes for Epic, Oracle Health/Cerner, Meditech Expanse, and TruBridge. Covers master patient index design, patient matching methodology, and CommonWell/Carequality network connection requirements.

E.9 HCC Gap Closure Playbook

Methodology guide for retrospective HCC gap analysis. Covers: RAF comparison methodology (organization vs. regional benchmark), chart review protocol for identifying undocumented chronic conditions, HCC specificity coding education modules for 20 highest-volume HCC categories, AWV completion program design, and monitoring dashboard for ongoing gap closure tracking. Vermont-specific: pre-loaded AHEAD Medicare attribution data and VHCURES-derived RAF benchmarks for Vermont peer comparison.

E.10 HTR Intelligence Feed

Weekly synthesis of consequential healthcare transformation developments, published every Tuesday. Coverage organized around the six pillars: Policy (federal rulemaking, CMMI model developments, state legislation), Economics (VBC market developments, hospital financial trends, payment model results), Technology (health IT developments, AI governance, interoperability standards), Clinical (quality measurement, care model evidence), Equity (disparity data, equity policy), and Operations (workforce, administrative simplification). Free tier available at htreview.com; Professional

and Advisory tiers include full analysis, Vermont-specific commentary, and database access.

For subscription information and tool access, visit htreview.com or email info@htreview.com.

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Note on Sources and Currency

This book was written between January and April 2026. Vermont's healthcare transformation is ongoing, and specific data — hospital financial positions, GRCB regulatory actions, AHEAD implementation status, RHT Program grant awards — will continue to evolve after this writing. Readers should consult the HTR Intelligence Feed (htreview.com) for current developments. Vermont legislative documents are available at legislature.vermont.gov; GRCB publications at gmcboard.vermont.gov; AHS Health Care Reform materials at healthcarereform.vermont.gov.

All Vermont government documents cited in this book are publicly available. The Oliver Wyman Act 167 analysis is available at the Vermont Legislature's website. AHEAD State Agreement terms are available through CMS's public AHEAD Model documentation.

URLs were verified as active as of April 2026. Vermont government websites are stable; federal CMS program pages may change with administrative transitions.

